



PARLIAMENT OF TASMANIA

TRANSCRIPT

HOUSE OF ASSEMBLY

ESTIMATES COMMITTEE A

Hon. Jeremy Rockliff MP

Monday 6 June 2022

MEMBERS

The Chair of Committees (Chair);
Ms Alexander (Deputy-Chair);
Ms White
Ms O'Connor

OTHER PARTICIPATING MEMBERS

Dr Broad
Ms Dow
Mr O'Byrne
Dr Woodruff

IN ATTENDANCE

Hon. Jeremy Rockliff MP, Premier, Minister for Health, Minister for Mental Health, Minister for Tourism, Minister for Trade

Ministerial Office

Vanessa Field	Chief of Staff
Sandy Wittison	Principal Adviser
Tony Mayell	Principal Adviser
Sophie Fitzgerald	Senior Adviser
Lucy Gregg	Senior Adviser
Laura Eaton	Senior Adviser
Andrew Johnson	Adviser
Duncan McKenzie	Clinical Adviser
Megan O'Brien	Adviser
Rosita Gallasch	Adviser

PREMIER

Department of Premier and Cabinet

Jenny Gale	Secretary
Craig Limkin	Deputy Secretary, Policy and Delivery
Noelene Kelly	A/Deputy Secretary, Government Services
Rod Nockles	Deputy Secretary People, Performance and Governance
Rob Williams	Project Director, Department of Communities Transition Project
Jane Hanna	Director, State Service Management Office

Department of Communities

Mel Gray	Deputy Secretary Wellbeing, Strategy and Engagement
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HEALTH PORTFOLIO

Department of Health representatives

Kathrine Morgan-Wicks	Secretary and State Health Commander
Tony Lawler	Deputy Secretary Clinical Quality, Regulation and Accreditation/Chief Medical Officer
Mark Veitch	Director of Public Health
Dale Webster	Deputy Secretary Community, Mental Health and Wellbeing, Commander Health Emergency Coordination Centre
Craig Jeffery	Chief Financial Officer
Sonj Hall	Deputy Secretary Policy, Purchasing, Performance and Reform
Shane Gregory	Deputy Secretary Infrastructure
Francine Douce	Chief Nurse and Midwifery Officer
Joe Acker	Chief Executive Ambulance Tasmania
Michelle Searle	Chief People Officer
Lisa Howes	Director Office of the Secretary
Tom Gunner	Assistant Manager, Office of the Secretary

MENTAL HEALTH PORTFOLIO

Department of Health representatives

Kathrine Morgan-Wicks	Secretary and State Health Commander
Dale Webster	Deputy Secretary Community, Mental Health and Wellbeing, Commander Health Emergency Coordination Centre
Craig Jeffery	Chief Financial Officer
Shane Gregory	Deputy Secretary Infrastructure
George Clarke	General Manager, Mental Health, Alcohol and Drug Directorate
Cat Schofield	Director of Services Statewide Mental Health Services
Lisa Howes	Director Office of the Secretary
Tom Gunner	Assistant Manager, Office of the Secretary

TOURISM PORTFOLIO

Tourism Tasmania representatives

John Fitzgerald	Chief Executive Officer
Mark Jones	Chief Operating Officer
Emma Terry	Chief Marketing Officer
Edwina Morris	Director, Office of the CEO

Department of State Growth representatives

Kim Evans	Secretary
Angela Conway	A/Deputy Secretary Cultural and Tourism Development
Claire Fitzgerald	Director, Tourism and Hospitality Support
Amanda Russell	Deputy Secretary, Business Services
Glen Dean	Director Finance

TRADE PORTFOLIO

Department of State Growth

Kim Evans	Secretary
Mark Bowles	Deputy Secretary Business and Jobs
Lara Hendriks	Executive Director Trade
Louise Osborne	Senior Director Trade and International Relations
Amanda Russell	Deputy Secretary, Business Services
Glen Dean	Director Finance

PUBLIC

The Committee commenced at 9.00 a.m.

CHAIR - Thank you all for being here today. Before we begin, I remind all persons attending Estimates to adhere to COVID-19 safe practices. In line with the parliament's COVID-19 Safety Plan and advice from the Presiding Officers, masks are recommended to be worn in committee rooms, given the significant period of time we are in the room and the close proximity of people.

The Chair welcomes the Premier to the committee and I invite the Premier to introduce persons at the tables, names and positions for the benefit of Hansard.

Mr ROCKLIFF - Thank you, Chair. At the table I have with me the Secretary of the Department of Premier and Cabinet, Ms Jenny Gale; and the Deputy Secretary of Policy and Delivery, Craig Limkin.

CHAIR - Thank you, Premier. The time scheduled for the Estimates of the Premier is three hours. We will take a five minute break for morning tea at 11 a.m.. Members will be familiar with the practice of seeking additional information, which must be agreed to be taken by the Premier, and the questions handed in writing to the Secretary.

I remind you that the microphones are sensitive, so please be mindful of Hansard and be careful when moving your folders, documents and water glasses around the table.

Also, it is difficult for Hansard to differentiate when people are talking over each other. I ask that members speak one at a time to assist with this.

Premier, would you like to make an opening statement?

Mr ROCKLIFF - I thank members of the committee. I am pleased to participate in my first Estimates as Premier. Of course, our Government's vision is for Tasmania to be a place where everyone is encouraged and supported to be the best they can be. I promise to lead a Government with heart, integrity and one which is accountable.

We want safe and inclusive communities and, as I have said on many occasions, communities where children and young people are kept safe. This is one of my personal priorities as Premier. We haven't waited for the commission of inquiry's recommendations to act, and have already announced a range of measures that are funded in this State Budget, as well as a number of additional actions we will take before the commission hands down its report.

The Department of Premier and Cabinet will lead the implementation of and reporting on a suite of measures which include: building shared capability for serious investigations across the State Service, that will put a focus on trauma informed practice; improving the right to information process across the State Service so that victim survivors who are going through the RTI process, will have a more transparent and supportive experience that causes no further distress to them; and consideration of legislative solutions and other initiatives that will make it easier to share information about risks to children, including looking at whether issues of concern, practice and culture are creating unnecessary barriers;

PUBLIC

Putting all our major child-related services and skills into one department will go a long way - practically and culturally - to break down these barriers, with the establishment of the new Department of Education, Children and Young People.

As members know, my Department of Premier and Cabinet (DPAC) is the lead or a key support agency for major reform projects and reviews that contemporise our legislation, and systems that will best position Tasmania to meet the state's current and future needs. One of these is putting a whole of Government lens over literacy, with more funding in this Budget for the work of the Literacy Advisory Panel, to create a community wide framework to improve literacy in Tasmania.

We have also provided funds to support the Department to lead our Government's next steps in Pathway to Truth Telling and Treaty. The Budget provides funding to support a project team for this work.

The Department of Premier and Cabinet will also lead the implementation of recommendations from the Royal Commission into International Natural Disaster arrangements which will be actioned across Government agencies.

The final report of the Independent Review of the Tasmanian State Service, undertaken by Dr Ian Watt, was completed in July last year. The review identified the changes we can pursue to ensure that our Tasmanian State Service is fit for purpose for Tasmania, through transforming current structures, services and practices to deliver a more efficient and effective public service. Work has already commenced, and a dedicated project team will lead the implementation of the recommendations with stage 1 to commence in 2022-2023.

The Department will also be leading the coordination of the implementation of our \$100 million Child and Youth and Wellbeing Strategy, the first comprehensive whole-of-government strategy for children and young people 0-25 years of age.

Over the last six months, Tasmania has commenced its transition to living with COVID-19. I acknowledge the sustained efforts since 2020 of our hard-working state servants, those in our department but also across the state sector; as well as our health services, our borders, our teachers, our education service providers and first responders. There is ongoing work by DPAC to deliver the key COVID-19 recovery initiatives, including implementation of the Premier's Economic and Social Recovery Advisory Committee (PESRAC) recommendations and the coordination of monitoring and reporting across agencies.

DPAC is accountable, under the Emergency Management Act and Tasmania's emergency management arrangements, for emergency management policy, the public information unit, public health hotline and recovery, and I am happy to take COVID-19 questions related to these areas. However, all other questions in relation to COVID-19 responses will be dealt with by the Department of Health's hearing, early this afternoon, when the State Health Commander and the Director of Public Health are present. I hope that is acceptable to members of the committee.

I welcome questions, thank you, Chair.

Ms WHITE - Thank you, Chair, thank you Premier. I want to ask you about an element of the Treasurer's speech on the Budget, where he said and I quote:

I have also tasked Treasury with providing advice to me on strategies to ensure our debt levels remain within manageable limits into the future, so we can again use our balance sheet to shield Tasmanian jobs and families should external shocks to our economy occur in the future.

Premier, there is no more detail about what this means. What is your understanding of what this means?

Mr ROCKLIFF - The Treasurer has spoken at various Budget presentations and, as I recall, reiterated a number of those matters in terms of debt and our response to debt. You mentioned debt and the reason why the debt has increased is increasing across the forward Estimates is the investment we're making in intergenerational infrastructure but also the level of funding that has been needed to support our COVID-19 response. We have spent a lot of funds to support the sustainability of businesses and to support the health and welfare of Tasmanians. We don't shy away from going into debt for the good reasons of supporting better intergenerational infrastructure but also keeping Tasmanians safe during a considerable period of disruption.

Ms WHITE - Premier, the reason I ask the question is I'm interested to know what will Treasury be tasked to do? The Treasurer said he's tasked Treasury to do something about the state of the Budget, what's your understanding of that? What does that mean for the operation of Government? What is Treasury tasked to do?

Mr ROCKLIFF - Well just as the Treasurer has outlined, Ms White -

Ms WHITE - That's what he said, 'there was no more detail'.

Mr ROCKLIFF - As the Treasurer has outlined, he has tasked Treasury and that will ensure that not only managing our debt moving forward but also sustaining our Budget into the future. That may well include taxation reform, which the Treasurer has also outlined in his presentations on the day after the Budget, and at the media conference as well as, if I recall correctly, Ms White, on the day of the Budget.

Ms WHITE - Premier, what you are considering as a part of the taxation reform? Are you looking at changes to tax rates, different taxes? Is there anything else you're considering? Are you going to be looking at an efficiency dividend across departments to manage debt levels, for instance?

Mr ROCKLIFF - We'll be open to various options. We're committed to no new taxes, but you would understand, Ms White, the importance of a reform in this area, the importance of sustaining our Budget moving forward, and ensuring that we manage our debt levels appropriately. Per capita our debt levels are far less than, say, Victoria and other states but we do need to be mindful and manage not only our revenue base but also continue the growth of the economy as well, which also can sustain our Budget. I think you are asking me to rule something in or out, like various taxes and all those sorts of things. I have been at budget Estimates for 20 years and can understand where the question is coming from, but if you are

PUBLIC

asking me about our taxation arrangements we will always ensure we have an efficient, fair, simple, stable and effective taxation base. We have already reformed payroll tax and have recently implemented important changes to land tax. We have also made changes to a number of housing-related duty measures as well to further assist with housing.

I guess what is pressing upon the Treasurer is the GST and the ongoing challenges. The Treasurer and I will be asking the Prime Minister when it comes to our fair share from the existing GST revenue pool. We will always fight for a fair and equitable share of the GST and we are looking forward to ensuring that Tasmania is no worse off following 2027.

Ms WHITE - I am trying to understand. Treasury was asked to look at the growing debt position of government and come back with strategies. Do you have any input into that? Is there a budget subcommittee and do you sit on it?

Mr ROCKLIFF - There is a budget committee and I have sat on it. The budget subcommittee will be working very hard over the course of the next 12 months in developing our budget.

Ms WHITE - Who else sits on that committee?

Mr ROCKLIFF - We are about to add members and constitute that committee. We are going to renew the membership of the committee.

Ms O'CONNOR - Your predecessor wouldn't tell us who was on it.

Ms WHITE - Did that committee meet before the Budget was delivered?

Mr ROCKLIFF - We met as Cabinet before the Budget was signed off by the Cabinet.

Ms WHITE - Who will be on the subcommittee?

Mr ROCKLIFF - I am unable to inform the committee at this stage but I am happy to let you know when the new members are on there.

Ms O'CONNOR - I want to talk about the commission of inquiry. Are you able to tell the committee how many State Service staff in total and from which agencies have been stood down under the ED5 process as a result of the investigation that you initiated into historical abuse in the Department of Education, evidence of historical abuse at Ashley Youth Detention Centre and as a result of evidence presented to the commission of inquiry to date?

Mr ROCKLIFF - I can get that information for you, Ms O'Connor. I reiterate there is no more important task for any government than to ensure the safety and wellbeing of our children. Our Government has taken a zero-tolerance approach to allegations of child sexual abuse in the Tasmanian State Service. Since October 2020 there have been 31 suspensions of state servants as a result of allegations of child sexual abuse, 21 related to historical allegations and 10 related to contemporary allegations. I am advised that 20 of the 31 cases are located in the north, with 11 in the south. Of the 31 cases, all have been referred or are known to Tasmania Police. I am advised that 30 of the 31 cases were reported to Tasmania

Police by the relevant agency, with a further case being a matter whereby Tasmania Police charged an employee and the agency subsequently advised of the charge.

Further, I am advised that in 30 of the 31 cases notifications have been made to the regulatory body. A further case was a matter involving an employee whose employment was not subject to any regulatory body. Of the 31 employees, six investigation determinations have found no breach in relation to child sexual abuse, and 6 have returned to duty as the findings of the relevant investigations found it was appropriate for this to happen. I think that was part of your question. Do you want a breakdown of agencies?

Ms O'CONNOR - Can I put that on notice?

Mr ROCKLIFF - Yes, thank you.

Ms O'CONNOR - Premier, what departmental processes have changed to prevent predators from working with children and young people?

Mr ROCKLIFF - The State Service review is across all agencies. With the Department of Education report recommendations are being implemented across the department but also across the whole of government.

Ms GALE - We have the normal checks and balances that have been undertaken for quite some time. We have Working with Vulnerable People checks for the employment of people according to the act. The Department of Education is putting in place additional actions as a result of the report into Department of Education practices in relation to this but that would be a matter for the Department of Education to detail those.

Ms O'CONNOR - Thank you, Ms Gale. What I took from your answer is that if you set aside how the Department of Education has responded to the inquiry into historical abuse, the processes in government remain largely unchanged. You talked about the checks and balances that have been in place for some time, you talked about Working with Vulnerable People registration which is a system that has been in place for more than a decade and obviously has its deficiencies. Is it the case that nothing substantive has changed in terms of government itself becoming a child-safe organisation as a result of information that has been provided to government through these processes I outlined before?

Mr ROCKLIFF - I can outline a number of measures that have been taken by the Department of Health as an example. I have mentioned the Department of Education and the recommendations they are implementing, but several measures are specifically aimed at ensuring the safety of children within our care, and I am referring to the Department of Health. These include ensuring all staff are aware of their mandatory reporting obligations and the available avenues to report if they suspect a child is being abused.

Ms O'CONNOR - Is that on employment with the State Service in the Department of Health, or is that a message that is reinforced with Health department employees on a regular basis?

Mr ROCKLIFF - We are implementing changes which I am about to talk about. We have reviewed internal processes in terms of Health, my responsibility, we are dealing with

allegations of child sexual abuse and implementing a framework within Human Resources to ensure that all necessary steps and key notifications are made within appropriate time frames with the appropriate support services provided to all parties. We are looking to make it easier to report suspected abuse by streamlining and centralising the reporting process.

The department is also progressing an extension of the requirement to hold registration to work with vulnerable people to all staff, contractors and volunteers within the agency. In addition, discussions have been held with the Australian Health Practitioner Regulation Agency regarding improving information-sharing and communication to ensure that both organisations can make appropriate steps to protect the community where there are allegations of misconduct against a health practitioner. The Child-Safe Organisations Report is part of the Tasmanian Government response to the national principles for child-safe organisations and associated child-safe standards. The Government recognises we need to do better and, of course, we must learn from the past. The national principles reflect 10 child-safe standards recommended by the Royal Commission into Institutional Responses to Child Sexual Abuse. The national principles give effect to those recommendations relating to the standards and the national principles have a broader scope that goes beyond sexual abuse to cover other forms of potential harm to children and young people.

I've got some other information here. Work is now well advanced on a revised approach to developing a comprehensive child and youth-safe organisations framework for Tasmania, to be overseen by an independent oversight and regulation body.

Ms O'CONNOR - Do you have a time frame for that Premier, because the commission of inquiry was told it would take three more years and this is a recommendation to the royal commission that was made four years ago?

Mr ROCKLIFF - When it comes to the development of the child-safe organisations framework, including comprehensive legislated child-safe standards and the establishment of a reportable conduct scheme, we're expecting to introduce a bill by November this year; establishment of a new statutory authority by 1 July next year; and completion by 1 July 2024. That is the current time line.

Ms O'CONNOR - Thank you.

Mrs ALEXANDER - Premier, can you please outline to the committee the supports provided to the Tasmanian community, those Tasmanians in need, during the COVID-19 pandemic?

Mr ROCKLIFF - Thank you, Mrs Alexander. We have been supporting many people and families impacted by COVID-19 since early 2020, in partnership with the Salvation Army, Red Cross and many other service providers, including family violence, homelessness and mental health supports. Our COVID-19 care package acknowledges the current increase in the need of Tasmanians who need to isolate or are close contacts, so we have extended the funding available through the Salvation Army service with an additional \$100 000 to ensure Tasmanians can continue accessing these day-to-day essentials.

As of 24 April, over \$2 million in direct financial assistance has been provided to over 100 000 Tasmanians through a range of assisted measures through the pandemic isolation

assistance grants. Of course, we not only support Tasmanians dealing with the impacts of the pandemic but also our vital community sector organisations which are delivering help to those in need. In fact, our fast response to the community sector has been described as 'nation-leading' by TasCOSS CEO Adrienne Picone.

This has been backed by other community organisations, including the CEO of Neighbourhood Houses Tasmania. These initiatives include our initial allocation of \$100 000 in funding for TasCOSS to distribute grants for PPE, rapid antigen tests and masks, which was quickly exhausted. We responded to this strong demand by providing just over \$253 000 in additional funding. We're also continuing to fund TasCOSS to continue running their fortnightly forums for the sector, which are helping organisations to understand current public health requirements and supports available. The COVID-19 care package responds to our community service sector's request to ensure their workers and those they help remain safe, with funding now available for PPE and rapid antigen tests through TasCOSS.

This is on top of direct access of rapid antigen tests and PPE for residents of disability group homes and homeless shelters, as well as the availability of RATs for essential workers through WorkSafe Tasmania.

Seasonal workers who have come to Tasmania to support the agricultural industry have also been provided with over 14 000 payments of \$250 each while in isolation and unable to work. These individualised supports are on top of existing employment and income support funding for those who have lost work through COVID-19.

Communities Tasmania continues to monitor the demand for all funded services to identify emerging issues and ensure there is capacity to meet current demand.

Mr O'BYRNE - Thank you, Chair. Premier, my question is around wages policy. We currently have probably the lowest paid nurses in the country. lowest paid firefighters in the country, some of the lowest paid teachers in the country. Given all of the work that those workers did for the Tasmanian people over the last couple of years during COVID-19, how is that a way to reward them - with a wage offer which is effectively below the cost of living? So, essentially a wage cut. It is not a great legacy for you.

Mr ROCKLIFF - We work with our employees well and I thank all of our State Service employees for the work they have done over the course of the last couple of years, in particular, during the huge disruption the pandemic has created. I met regularly with various advocates of employees, including HACSU, the ANMF and others, the AMA included in that, in terms of the pandemic. We have had very regular meetings with our health commander and our Public Health director, Mark Veitch, over the course of the last year or so.

More broadly, we are committed to negotiating with unions on wage agreements in good faith. I cannot pre-empt any outcomes, of course, Mr O'Byrne. We recognise the increased cost-of-living pressures on Tasmanian households and we are committed to delivering a fair, reasonable and affordable pay rise for our hardworking State Service employees. We are also committed to delivering a sustainable Budget and we cannot agree to a wage increase that we cannot afford, or cannot offset with savings, as I am sure you would appreciate, having been in government before.

There are 16 wage agreements due for negotiation in 2022. This includes the PSUWA agreement which covers all employees by the Tasmanian State Service Award and the Health and Human Services Award. Each agreement is negotiated for the specific group of employees to deliver fair and reasonable terms and conditions, including affordable wage increases.

The 2.5 per cent level of wage indexation is not an allocation for the purpose of developing a budget. It is not a wages policy. We will negotiate all new agreements in good faith while being mindful of the impact that wage outcomes will have on the budget position.

Mr O'BYRNE - Premier -

CHAIR - Mr O'Byrne, you have had your question. I will now pass to Ms White.

Mr O'BYRNE - Just as a follow-up, you have nurses taking industrial action. You have firefighters taking industrial action. You say the 2.5 per cent doesn't indicate your wages policy, yet that is what you have budgeted for. Is it just a matter of time before you walk away? Why don't you just walk away now and say that that is not the wage offer? Why don't you just say that now?

Mr ROCKLIFF - Of course, Mr O'Byrne, we are going to sit down with our unions and negotiate in good faith.

Mr O'BYRNE - Are you going to force them to take industrial action?

CHAIR - Mr O'Byrne, I am passing to Ms White.

Mr ROCKLIFF - No need for industrial action, Mr O'Byrne.

Mr O'BYRNE - But they are already taking it. Nurses are already taking industrial action and we will have firefighters out on the streets within days.

Mr ROCKLIFF - And we will work with our unions. I value State Service employees. Sitting down and negotiating in good faith is exactly what we intend to do.

Ms WHITE - Premier, I wanted to go back to the line of questioning I started around which was the work Treasury has been tasked to do. Will they be looking at efficiency dividends across agency to manage the Budget?

Mr ROCKLIFF - That is not on our agenda at this stage. We recognise the enormous amount of supports we have provided across agencies and the enormous amount of effort that agencies have made, especially in working together and delivering services during a very disruptive time. Ms White, we will be developing next year's Budget pretty soon after this one is officially passed.

Ms WHITE - Can I ask you about one of the commitments you outlined in your opening statement and that is the safeguarding officers. Can you confirm that the money is to be found from within the department's existing resources for those announcements you made?

Mr ROCKLIFF - We are providing additional resourcing to schools to support specific safeguarding actions which were recommended by the Department of Education Inquiry into Child Sexual Abuse. We were discussing that before, with Ms O'Connor. This initiative is supported by our continued growth in funding for Tasmanian education. In this Budget and across the forward Estimates we are investing \$8.5 billion. Funding provided to schools increases year on year, with the Government meeting the commitment of an additional \$340 million to state schools over the next 10 years - that is 2018 to 2027 - and that is part of the bilateral and the National School Reform Agreement.

The agreement will deliver an additional \$490 million of state and Australian Government funding to Tasmanian Government schools over these next 10 years. Keeping children safe is our highest priority and it starts with safeguarding the rights of all our children and young people to be kept safe from harm. I will highlight some of the initiatives. We have \$26.1 million over four years from 2022-23, and \$9.7 million ongoing, to appoint school safeguarding officers in every Government school as part of a support and wellbeing team lead role that will plan and implement school strategies to support the wellbeing of all students.

There is \$2.6 million over four years from 2022-23, and \$600 000 ongoing, for mandatory professional development for all departmental staff in understanding, preventing and responding to child sexual abuse in schools, thereby building a culture of putting the child and their safety at the centre of all decisions and actions that affect them. There is 1.27 million over two years from 2022-23 to provide more support for children and young people affected by harmful, sexual behaviours, including: four full-time equivalent senior support staff with specialist expertise; \$3.8 million over four years from 2022-23; and \$1.68 million to ongoing employment of additional psychologists and social workers to directly support schools.

Ms WHITE - My question was about whether this was new money, because the Budget papers clearly state - in relation to the safeguarding of children - that this is a new initiative funded from within a department's existing resources. Can you understand, therefore, why there is concern that the department has not been provided with additional money to fund these new initiatives but will have to find that money from within their existing allocation? How can you explain that footnote which clearly says that it is to be funded from within existing resources, and claim that it is new money?

Mr ROCKLIFF - Because we have growth funding. I recall signing the bilateral agreement - I believe it was in November 2017 - as Minister for Education, which provided for that growth funding across the 10 years, 2018 to 2027. The growth funding we included then was to support students with disability and a trauma informed practice as well, as an example of that. We have growth funding over the course of the next 10 years, of which we can support increased services and services delivered within the Department of Education.

Ms WHITE - Premier, you have just confirmed there is no new money. It is to be funded out of the Budget allocation that would have been forecast last year, because there was the growth funding that you had already signed up to as part of the agreement.

PUBLIC

Mr ROCKLIFF - Yes; but there continues to be growth funding across the forward Estimates over the course of the next 10 years.

Ms WHITE - It is not new money or initiatives.

Mr ROCKLIFF - They are new initiatives and they will be delivered.

Ms WHITE - Do you understand how concerning this is, and that this is essentially an efficiency dividend by stealth because you are requiring the department to start delivering new initiatives within their existing budget? This will impact on their ability to do their core functions especially at a time when our NAPLAN results are the worst in the country and they are rightly concerned about teaching the basics of the curriculum?

Mr ROCKLIFF - Ms White, we've got growth funding moving forward as part of the bilateral agreement, and everything that we have invested in and that we have said we are going to invest in will be delivered.

Ms WHITE - But it's not new money, you've just confirmed, thank you.

Mr ROCKLIFF - Well, increasing more money across the next forward Estimates, additional funding.

Ms WHITE - But it's not more than what was in the Budget last year.

Mr ROCKLIFF - What we've committed to, we'll be delivering.

Ms O'CONNOR - Thank you, Chair. Premier, earlier you were talking about the delivery of the Child Safe Standards Framework. In evidence before the Commission of Inquiry, the Secretary of Communities Tasmania, Ms Webster, responded to a question and said that the 'Child Safe Standards Framework would take three years to deliver.' Are you saying that work has been brought forward and why will it take, according to your previous answer, two years to have a completed Child Safe Standards Framework and reportable conduct scheme in place, given that the Royal Commission made this recommendation four years ago and Victoria has had this scheme in place since 2017?

Mr ROCKLIFF - Ms O'Connor, I can only reiterate the indicative time line that I've just outlined: introducing the bill by the end of this year - November - and the new statutory authority by 1 July 2023 - in just over 12 months - and completion by 1 July 2024.

Ms O'CONNOR - Right, okay.

Mr ROCKLIFF - If I can just reiterate that.

Ms O'CONNOR - Can I ask whether, as a result of evidence given to the Commission of Inquiry, the Royal Commission Response Unit in the Department of Justice has been asked to expedite, to accelerate its work to implement the recommendations of the royal commission?

Mr ROCKLIFF - That may well be a question directly for the Attorney-General, M O'Connor. What I can say though in response to the commission of inquiry to date, is that the president of the commission clearly said in her opening statement that 'Government should not wait before delivering on much-needed reforms in this area,' or words to that effect. I outlined exactly that, in a ministerial statement just a few weeks ago, in terms of what we are enacting. I will cover some of those today and there may well be other measures over the course of the next 12 months that we might implement as well when gaps come to light. We will be looking forward to the recommendations of the commission of inquiry in about 12 months.

Ms O'CONNOR - Premier, evidence provided to the commission paints a picture of a deeply unhealthy relationship between Government here and the media. A journalist who gave evidence noted that, 'the former premier, Peter Gutwein, would often accuse journalists of talking down Tasmania when they asked basic questions' - something we are all familiar with in his responses in parliament. She went on to say:

Every time you make a request for information here, and again I'm talking about from the most basic requests to ones that I would acknowledge are more potentially hairy for a government, it's 'why do you need the information, what are you going to do with it, what are you writing, what's your angle,' argue, argue, argue - 'it's 5 o'clock, it's too late, sorry, everyone's gone home.'

She noted that the attitude in the ACT was markedly more helpful. Premier, will you commit to establishing a more positive and reasonable culture of being open with information in your Government? Do you accept that it's not good enough, or it hasn't been to date?

Mr ROCKLIFF - I can't speak for previous premiers, but of course the short answer to the first part of your question is 'yes.' I'm very keen to increase transparency and a commitment to transparency agenda and leading an open and accountable Government. I have demonstrated that with my various ministerial responsibilities over the course of the last eight years, in terms of data being presented. Through education it's very clear that an increasing amount of data was released from me, as Minister for Education, so all that was out there, 'warts and all', effectively. As Minister for Health, more transparency in terms of our Health Dashboard. I know there is a lot of public interest when that is released. It used to be released quarterly and is now released monthly, so the answer to your question is yes.

Ms O'CONNOR - Can I just ask quickly how you are giving effect to that? Is there a message going out to agencies, where a lot of this problem has been identified as beginning, that there is no need now to be so difficult about right to information requests or other requests for information? Has the message gone out to departments that things are going to change?

Mr ROCKLIFF - I have made it very clear in my opening statement as Premier around accountability and integrity and I would expect the culture that I am creating in the Premier's office extends right throughout the public service.

PUBLIC

Mrs ALEXANDER - Premier, the hotel quarantine program was widely used across Australia and in Tasmania during the COVID-19 situation. Could you please give an overview to the committee of how successful this particular measure was in Tasmania?

Mr ROCKLIFF - Thanks, Mrs Alexander, for your question. We are continually monitoring, assessing and adjusting Tasmania's COVID-19 response to the consequences and impact of the pandemic on our community. The Department of Communities has had responsibility for the administration of the Government's hotel quarantine program and has done a tremendous job. Since its inception around 17 500 people have been accommodated through the hotel quarantine program. The program was designed to be robust in terms of public safety while also being mindful of individual needs.

Notably, Tasmania's program was the only one in all Australian jurisdictions that did not result in the transfer of infection from a hotel quarantine setting to the broader community. Hotel quarantine was a significant component of our overall response to the pandemic but as all state border restrictions have now been removed, the emergency management arrangements have now been removed and hotel quarantine settings are no longer required.

There were significant challenges involved in running a quarantine program, including using hotels which were not specifically designed for quarantine and compassionately balancing the full range of guests' needs with the requirement to quarantine. Whilst the majority of guests in hotel quarantine were domestic guests, the quarantine program also included repatriated Australians, Antarctic expeditioners and seasonal fruit workers. Five of the eight facilities engaged by Communities Tas were released back to the private sector in March this year, whilst the remaining three hotels were handed over to the Department of Health.

I would like to take this opportunity, as I have done in various forums, to express my thanks to all the hotel quarantine staff who have had a very difficult time. Seven days a week, 24 hours a day they worked tirelessly to ensure our quarantine hotels ran smoothly and effectively. I expressed my thanks to them in a video presentation, if I recall. When I compare our hotel quarantine system to that of the challenges they had in Victoria, for example, our team did a stellar performance.

Ms WHITE - Will the safeguarding children officers be new staff recruited specifically for those roles?

Mr ROCKLIFF - I will have to refer that to the Department of Education and the Minister for Education to give you detail in respect to those matters.

Ms WHITE - Okay. I want talk about the abolition of the Department of Communities. Stakeholders have criticised the complete lack of consultation by the Government on the abolition of the Department of Communities. Do you agree that this requires further consultation? You may have noted this morning's article where the CPSU has called for a halt for at least 12 months and we support that. Given the work of the commission of inquiry is still underway and the experience in South Australia that prompted a royal commission when they merged their Department of Communities with the Department of Education because

children were falling through the cracks, do you agree that it is time to pause this and properly consult about what it means for children and their welfare?

Mr ROCKLIFF - Thank you for the question but I do not agree. I also do not agree that you can compare us with South Australia either, because from my understanding of South Australia, the key differences are the resourcing. South Australia went into that reform aiming to have fewer resources, if I am correct, whereas we are not wanting to reduce resources as a result of our reform. It is important when it comes to the commission of inquiry and the safety of our children that we make every opportunity to break down silos between agencies. That is incredibly important.

The creation of the new Department of Education, Children and Young People provides such an opportunity to increase the sharing of information and build a stronger and better coordinated system to improve outcomes for children and young people. It is very different from the South Australian experience which I am advised involved rationalisation of resources. From my own personal experience through the pandemic I saw that there were siloed arrangements between the Department of Education and the Department of Communities which I became aware of when we were keeping school sites open but encouraging our young people to learn from home, and particularly maintaining sight of our young people in very vulnerable settings. That pandemic response was a very good opportunity for the Department of Communities and their representatives and the Department of Education to work very closely together so we could have the full sight of the needs of our children across Tasmania.

Unions and staff are being consulted on how the transition will take place and I believe the project team met with the unions as recently as Friday 27 May in relation to the transition with feedback used to update the consultation and change plan, which is now with relevant employees for comment. The industrial relations working group will meet again with unions at the end of the feedback period which closes on 10 June. Since the announcement more than 16 formal communiqués have also been issued to affected staff in relevant agencies to ensure they are kept up to date.

I do not share your view around comparing South Australia with Tasmania, Ms White. We are not about taking away resources, we are ensuring we are breaking down silos when it comes to the Department of Education, Children and Young People.

Ms WHITE - Are you aware there is no consultation to occur with the staff about the changes? I understand, as it has been put to the unions through that working group, that they have been told there is essentially no change except who the department secretary reports to, so there is no consultation required with the workforce. Do you understand how problematic that is, particularly because you are talking about changing the awards some workers are employed under? Will you agree to proper consultation with the workforce, please?

Mr ROCKLIFF - I have just outlined that we are consulting the workforce.

Ms WHITE - You are not. You are consulting with the unions, not with your workforce.

PUBLIC

Mr ROCKLIFF - I have mentioned staff. Ms Gale, do you have anything further to comment?

Ms GALE - We are consulting with staff and, as you would be aware, Ms White, this is a staged transition. At the moment we have a consultation period with employees who are involved in the first tranche of moves which will be from the Department of Communities Tasmania to the Department of State Growth and also with those staff who are in the corporate area of DCT. There is a consultation period open at the moment with those employees.

Ms WHITE - Premier, do you commit to consult with the entire workforce? You are talking about an agency that was only set up in the last term of Government that you are now choosing to abolish, and they are on different awards. Bringing them into the Department of Education is very different. I think it would be fair, in terms of what you have stated your Government needs to be - which is accountable and acts with integrity and is consultative - that you do speak to them.

Mr ROCKLIFF - It has been my expectation. Of course, we will speak to them.

Ms GALE - Ms White, as I said, the transition from DCT into the receiving agencies is being undertaken in a staged way. And yes, there will be consultation with all of the employees as we reach the appropriate time in each of the stages of the transition across.

Ms WHITE - Premier, who's idea was it to abolish the Department of Communities?

Mr ROCKLIFF - Cabinet makes these decisions.

Ms WHITE - Based on what information or evidence to support that decision, when you only established the department in the last term of Government?

Mr ROCKLIFF - I have just outlined to you the key reasons, in this very important environment, where we are learning more as a result of the Commission of Inquiry and where we have failed our children and young people over successive decades. No doubt, there still are gaps. But, breaking down silos between departments, particularly when it comes to children and young people, is incredibly important. I have said that - on 24 May and in parliament. Breaking down silos and information sharing, it could prevent us from protecting a child. Anything we can do to break down silos and increase the sharing of information in the interests of the safety of our young people, we will do it.

Ms O'CONNOR - Premier, I don't know if you have read any of the transcripts from the commission of inquiry, but it struck me when I was reading the testimony of journalists from 5 May, just how poorly the Government that you have now part of has treated journalists reporting on the Griffin cases or abuse at Ashley. The inquiry heard journalists have been told by public servants and, in at least one case, the Commissioner for Children, that the reporting was going to harm children. We would argue this reporting was central to community understanding of the extent of the abuse and the establishment of the commission of inquiry.

Premier, is it not the case that the very agencies making these claims have routinely failed to protect children, as evidenced by the matters being reported on; and, we wouldn't have the commission of inquiry or the closure of Ashley without the work of journalists? Don't you think your Government owes journalists, and particular the journalists like Camille Bianchi, an apology?

Mr ROCKLIFF - Of course our journalists play a very important role in our community in terms of shining the light on matters where Governments have failed and where Governments need to be more accountable. I have always respected the role of our journalists and reporters, in that sense. As a member of parliament for 20 years and certainly as a minister for eight years and now as Premier, my view hasn't changed in terms of the important role that journalists have. I respect the role. I may not always agree with the opinions of various commentators and the like, but I certainly respect the investigative nature of journalism in keeping Governments to account and effecting change.

Ms O'CONNOR - That is very good to hear. Premier, Camille Bianchi's podcast *The Nurse* exposed the extent of Griffin's abuse at the LGH. She gives testimony to the commission of inquiry that she lodged her right to information request with the Department of Health in early April 2020. The department effectively blocked the request and then after a long delay, didn't provide the information. Then there was an Ombudsman's decision which should have put the Department of Health on notice, but the information was still not provided. The Right to Information material was provided 22 months after Camille Bianchi first asked for it, after a program went to air on *Seven News*; within about half an hour of that program going to air that mentioned the RTI. Do you agree that is just not good enough, and when you talk about improving the right to information process, what does that mean in the context of this sort of conduct on the part of the Department of Health?

Mr ROCKLIFF - More broadly, in late March 2021, the then premier, Mr Gutwein, discussed with the Ombudsman improvements that we are making to address the Ombudsman's concerns, including: provision of more in-depth RTI training to delegates; a new online portal to make the routine disclosure of Government information easier to find; and development of enhanced processes and systems for RTI requests. Additionally, the Government has provided increased funding to the Tasmanian Ombudsman to support the functions of its office. As you would appreciate, in terms of Health, we have Health Estimates coming up, Ms O'Connor, which I am sure you can drill down on the RTI processes. Of course they are at arms-length from me, as minister, and from all ministers, in fact. If you are asking me is 22 months too long - yes.

Ms O'CONNOR - Thank you. A final question on this line of questioning. Premier, are you foreshadowing changes to the Right to Information Act so that the capacity for agencies to actively disclose information is reinforced and strengthened, and do you understand why journalists are giving that sort of testimony to the commission of inquiry? It is not that long ago that this blocking, if you like, from the Department of Health happened. Do you understand the frustration and concern about the way the Government that you are a part of has dealt with Right to Information requests - and it is not just in the Health Department?

Mr ROCKLIFF - We are open to improvement in all these areas of transparency, Ms O'Connor. I accept what you say about the Health Right to Information, of course, which we

can explore further in terms of our RTI agenda. What we have done in terms of our transparency agenda includes committing \$500 000 over two years to support the significant uplift of Right to Information capability and practice in the Tasmanian State Service. The funding will facilitate the provision of centralised training, building skilled RTI practitioners and will reduce key person dependencies in agencies. We will promote supported and consistent practice across the RTI space, which will deliver enhanced processes and systems for RTI.

Ms O'CONNOR - Do you agree that active disclosure should be really the default position of Government agencies, rather than attempting to conceal information, which has been the evidence and the culture at least for the past eight years? That agencies should furnish the information actively, rather than trying to block RTI requests like Camille Bianchi experienced?

Mr ROCKLIFF - Ms O'Connor, I have said I would lead a Government that is accountable. There has been, as I understand it, a significant increase in the number of routine disclosures of information to date. Since 2014 - you mentioned eight years - we have committed to improve openness and accountability of Government decision making through what has become known as the Government's Transparency Agenda. We have delivered reforms under this agenda, including publishing Right to Information responses, online within 48 hours of release to applicants, for example. I can go into other matters outside of the RTI but, of course, delegating ministerial responsibilities under the Right to Information Act 2009 to department officers would be relevant to your RTI question.

Mrs ALEXANDER - Premier, the Public Health Hotline has been a good instrument in the aftermath of COVID-19, as we transition to live with COVID-19. Would you please present a bit of information around the services and activities of the Public Health Hotline and how this is working?

Mr ROCKLIFF - Thanks, Mrs Alexander. It is the primary contact point for community inquiries relating to the COVID-19 disease and has played a critical role in supporting the community's response to the pandemic. It links our community with crucial and critical information, support and services, including COVID-19 testing, accommodation, emergency relief and financial support. The hotline employs about 150 part-time and casual call-takers, maintaining services seven days a week. It consistently provides timely advice and information to the Tasmanian public in support of the current public health messaging.

Calls to the hotline peaked on Tuesday 4 January, with 14 035 calls offered during business hours. In the two weeks ending 13 May, call volumes averaged around 1100 calls per weekday, with an average speed of answer of 10 seconds. I am advised that the call volumes on the weekends are typically significantly less than those received during the week. Since March 2020 the Public Health Hotline has received more than 1 300 000 calls.

Recent technology upgrades have increased the capacity of the call centre to scale up or down quickly, with the ability for operators to work remotely now embedded within the centre.

The critical role of the hotline has included referring callers to Public Health for COVID-19 testing or completing a request for a RAT kit; linking people in isolation at home

or in government-run facilities with support, such as accommodation, food and financial support; registering and providing financial assistance to temporary visa holders; providing factual information about the response, including social distancing and public gathering requirements; providing information in relation to the G2G application process; processing registrations for the Check In Tas app, including SITA (?) cards; and providing information regarding COVID-19 vaccination eligibility and booking of appointments.

Thank you very much for the question. I would also particularly like to thank all the staff who have operated the hotline, which at times was enormously busy and a hectic working environment. I was pleased to meet with them on a number of occasions throughout the course of the pandemic.

Ms WHITE - Premier, do you believe that Jane Howlett had a significant undisclosed potential conflict of interest when she was the minister for Sport?

Mr ROCKLIFF - No. I haven't seen or I am not aware of any evidence to support that and you have made no case for a conflict of interest against the former minister for Sport and Recreation. Therefore, there is no case for an inquiry. If you have evidence, then please bring it forward, Ms White.

Ms WHITE - So, you have never received any information, including verbally, that would lead you to believe that Jane Howlett had a significant conflict of interest when she was the minister for Sport? Nothing at all?

Mr ROCKLIFF - My understanding is that the premier of the day signed off on matters relating to the JackJumpers and I haven't - if you've got information -

Ms WHITE - Never heard a thing?

Mr ROCKLIFF - I have heard questions in parliament, of course, around these matters.

Ms WHITE - Were you involved in any discussions with the former premier, Peter Gutwein, regarding the Cabinet reshuffle that occurred on 7 February following Sarah Courtney's resignation?

Mr ROCKLIFF - I would have been, yes.

Ms WHITE - The reason I ask is that Ms Howlett was replaced as the minister for Sport during that reshuffle but she retained her other portfolios. What was your understanding about why this change occurred?

Mr ROCKLIFF - The decisions of Cabinet make-up are the purview of the premier of the day. I recently provided a change of Cabinet positions as well and I am very proud of our team.

Ms WHITE - Have you directly asked Jane Howlett about these allegations?

Mr ROCKLIFF - No, I haven't.

Ms WHITE - Why didn't you return Jane Howlett to the ministry?

Mr ROCKLIFF - It is my decision to choose members of Cabinet as I see fit and I have chosen a very good team with the right mix of lived experience. I am very proud of all our members of Cabinet.

Ms WHITE - Do you believe that Jane Howlett could be returned to the Cabinet in the future under your leadership?

Mr ROCKLIFF - I am not anticipating any Cabinet reshuffles any time soon at all, Ms White.

Ms O'CONNOR - I want to finish a line of questioning on the commission of inquiry, Premier. Evidence that came before the commission on 6 May, where the head of the State Service, Ms Gale, was interviewed by the commission as well as the head of Communities Tasmania, Gina Webster, revealed that there is no oversight of the Royal Commission recommendation implementations at the head-of-state-service level. Are you able to tell the committee whether there has been any change since that evidence was presented to the inquiry?

And has the implementation of the recommendations of the Royal Commission into Institutional Responses to Child Sexual Abuse been elevated to the Cabinet level, where there are Cabinet briefings of progress on that implementation process?

Mr ROCKLIFF - I am advised that Justice is leading the work with respect to the national commission of inquiry. What I have requested Ms Gale to do is to implement and lead the work when it comes to the measures that I outlined in parliament through a ministerial statement recently.

Ms O'CONNOR - Okay. I am not sure if you have read the transcript but it was a bit jarring to see that the head of the State Service does not apparently have any oversight or responsibility for the recommendations of the Royal Commission. It does not sound like that has changed.

Premier, during evidence to the commission it has become clear that a number of statutes are likely to need amendment to provide better protection for children and young people. This includes the Commissioner for Children and Young People Act to clarify the powers to advocate on an individual level; the Public Interest Disclosure Act in order to protect the suppliers who come forward about abuse, whether or not that conflicts with the State Service Act; potentially the Ombudsman Act to strengthen the Ombudsman's power; the Criminal Code Act; the Integrity Commission Act; the Right to Information Act; the Children, Young Persons and their Families Act; the State Service Act; Working with Vulnerable People registration. Has any work begun on the legislative reforms that have already been highlighted as necessary by evidence that has come to the commission?

Mr ROCKLIFF - As I have said previously, there are a number of areas in which successive governments have failed our children and young people. There are quite possibly gaps still. We will, of course, accept the recommendations of the commission of inquiry but

we are not waiting for those recommendations, as I have outlined, Ms O'Connor. We are considering legislative solutions and other initiatives that will make it easier to share information about risks to children, including looking at whether issues of custom, practice and culture are creating unnecessary barriers. DPAC are leading that work.

Ms O'CONNOR - Can you just confirm, and this is my final question on this line of questioning. Can you confirm that legislative changes to strengthen the statutory protections of children and young people is already underway and that DPAC, your advisers, are listening to the evidence that has come before the commission of inquiry which reveal the necessity for statutory change and that work is being done already and you are not waiting for the recommendations from the inquiry, which are about a year away?

Mr ROCKLIFF - I have said in my ministerial statement that legislation will be drafted this year to create a new crime of failing to protect children or a young person for people in authority within an organisation who fail to safeguard a child from substantial risk of sexual abuse by an adult associated with the organisation. Of course the Attorney-General has taken the lead on this work and I understand progress is already being made. There will be consultation on this legislation and I hope it will be passed by the end of the year. We also plan to amend the Criminal Code to introduce a presumption that children under the age of 17 cannot consent to sexual intercourse when a person is in a position of authority over them. These legislative changes will bring our criminal justice system into alignment with community expectations -

Ms O'CONNOR - Is that happening this year?

Mr ROCKLIFF - If possible. We will be exploring options to expand the scope of regulated activities under the registration to work with vulnerable people legislation to ensure the screening scheme for people who work or volunteer with vulnerable people, including children, is the best it can be.

Mr O'BYRNE - Premier, I am a bit confused by your previous answer around wages policy. The Budget talks about 2.5 per cent and you said that does not reflect the Government's wages policy. The forward Estimates predict overall expenses for government will actually go down in the next couple of years. I suppose it begs the question, what is the Government's wages policy and do you agree that workers should not receive a pay cut and should receive a pay increase at least to keep pace with the cost of living?

Mr ROCKLIFF - The 2.5 per cent level of wage indexation is an assumption for the purposes of developing the Budget and I am sure you are well aware of that.

Mr O'BYRNE - You have developed the Budget - the Budget has landed.

Mr ROCKLIFF - Mr O'Byrne, the Government will negotiate all new agreements in good faith, while being mindful of the Government's budget position. For example, a 1 per cent increase over and above the level of indexation provided is forecast to cost approximately \$397 million over the Budget and forward Estimates period, so it is important all wage negotiation outcomes are fiscally sustainable and within the capacity of the Budget and forward Estimates. I have just provided you with an example of the cost and 1 per cent

PUBLIC

above that 2.5 per cent is significant, so there is a balancing act here but we are committed to negotiating with unions in good faith.

CHAIR - I call Mrs Alexander.

Mr O'BYRNE - Sorry, you didn't answer my question.

CHAIR - Mr O'Byrne, you know the rules. You are an independent. Mrs Alexander.

Mrs ALEXANDER - Premier, in 2016 former premier Will Hodgman announced the new family violence leave provisions for State Service employees. Could you please update or provide an overview to this committee on how this is making a change or a benefit for the employees, as well as the number of days accessed by employees?

Mr ROCKLIFF - Thank you, Mrs Alexander. I state at the outset that violence against anyone in any form is simply utterly unacceptable. The harm caused by family and sexual violence is utterly devastating and I would like to first acknowledge and pay tribute to the courage of all victims/survivors of family and sexual violence.

I would also like to thank all those in our Government and our NGOs who assist victims/survivors of family and sexual violence and help them in their most vulnerable time of need, as well as for their strong dedication and ongoing efforts to support Tasmanians impacted by family and sexual violence. Family violence does not discriminate. It is up to all of us to do our bit to eliminate family violence and provide support to those who need it.

Our Government has continued to take a leadership role in tackling the scourge of family violence through a number of measures. One of these is the provision of an additional 10 days leave per year for State Service employees on top of current leave arrangements. Previously leave for family violence had to be taken as personal leave. This leave can be accessed for attending to matters such as health, legal, financial, housing, childcare and other matters. For the 2021-22 year, as of 31 March 2022 employees have accessed a total number of 18 days for circumstances involving family violence. I note the Fair Work Commission recently issued a provisional decision to include paid family and domestic violence leave in awards for permanent employees. I understand the decision is not finalised yet, with the commission seeking input from interested parties before issuing a final decision.

Another important piece of work underway to address family violence is the third Family and Sexual Violence Action Plan, with \$12.5 million provided for in the 2022-23 State Budget. One of the key priority actions in the new action plan will be a commitment of increased recurrent core funding for Tasmania's nine specialist family and sexual violence services, with five-year contracts, so as to provide greater certainty and increased operational capacity to respond to demand over the longer term. It is important for the Government to lead by example and that is exactly what we are doing in this important area.

Ms WHITE - Premier, you said earlier that all agreements were between Peter Gutwein and the JackJumpers but that is not true. Did you deliberately mislead the committee or have you been misled?

Mr ROCKLIFF - That is the information I have.

PUBLIC

Ms WHITE - Through RTI we received a copy of the contract that was a grant of \$250 000 between the JackJumpers and Ms Howlett's department as Minister for Sport and Recreation, signed by the deputy secretary of that agency. That is clearly one example where there has been a direct agreement that does not include Peter Gutwein's signature at all.

Mr ROCKLIFF - I think that has been well explained. The funds provided to Basketball Tasmania trading as the Tasmanian JackJumpers for an upgrade to facilities at the Kingborough Sports Centre was a commitment negotiated directly through the former premier's office and the former premier approved the funding. I am advised that Ms Howlett had no involvement in those negotiations and her department was responsible for the payment process, not the approval process, as we have outlined publicly before. What the RTI provided was simply a procedural letter which was necessary to enable the department to release funds and therefore entirely appropriate. The project being funded at Kingborough includes a coach's office, physiotherapy and recovery room, a kitchenette, players' lounge and a new courtside storage unit to house all JackJumper-specific equipment and is due for completion by 31 October 2022.

Ms WHITE - Are you trying to tell me that a minister of an agency that has allocated funds from that agency has nothing to do with the funds allocated from that agency?

Mr ROCKLIFF - I can only reiterate what I am advised, Ms White. The former premier approved the funding and Ms Howlett had no involvement in negotiations. Ms Howlett's department was responsible for the payment process, not the approval process.

Ms WHITE - Premier, can you confirm whether or not this was an election commitment?

Mr ROCKLIFF - I am advised this was not an election commitment and that is why it is not in last year's Budget.

Ms WHITE - That is contrary to what Ms Howlett said. She described it as an election commitment. The Right to Information documents show that when Ms Howlett wrote to the premier in late October, nearly six months after the election, requesting funding for this \$250 000 grant, the letter refers to it as an election commitment.

Mr ROCKLIFF - Okay. I can only tell you what I am advised - that it was not an election commitment. That is why it was not in last year's Budget. I am advised it was negotiated by the former premier's office following the 2020-21 state election and funding was provided in the supplementary appropriation bill in parliament.

Ms WHITE - If it was not an election commitment, why did Ms Howlett's letter outline that it was?

Mr ROCKLIFF - That is something for Ms Howlett.

Ms WHITE - You are the Premier; you need to be able to answer. This is why an investigation needs to be called, Premier, because there are too many things like this that get fobbed off. There was a \$250 000 allocation made to the JackJumpers, signed off by Ms

PUBLIC

Howlett's department, that you are claiming was not an election commitment. She claimed it was; the former premier said it was all to do with him. His name is not on this anywhere. It is signed off by Mr Brookhouse and the deputy secretary of Ms Howlett's department. It is very murky, don't you admit?

Mr ROCKLIFF - DPAC follows all the election commitments to ensure that they are delivered, number one, and I am advised that DPAC is unable to find that as an election commitment. As I say, the former premier approved the funding. Ms Howlett had no involvement in those negotiations and her department was responsible for the payment process, not the approval.

Ms WHITE - Then what is going on here? It indicates potentially, that Ms Howlett made a secret election promise, that she then couldn't fulfil.

CHAIR - Ms White, you have had your allocation of questions. Order, Ms White. Do you want me to name you?

Ms WHITE - Not really.

CHAIR - Thank you.

Ms O'CONNOR - Premier, I want to talk about the state's COVID-19 response. We are now at 79 Tasmanian lives cut short; 168 000 reported infections to date; 50 000 young people under the age of 19 infected. Has the Government adopted a herd immunity strategy in response to this virus? Is there a belief in Government that immunity can develop within the population?

Mr ROCKLIFF - I have outlined where the Department of Premier and Cabinet is responsible for the pandemic response, Ms O'Connor. Dr Veitch and our Health Commander will be available this afternoon at 2 o'clock, if I recall. I just want to reiterate that we do take Public Health advice on these matters. We have been, as a Government, as close to Public Health as possible when it comes to our pandemic response, unlike other states. I am sure Dr Veitch will be very open to discussing a number of matters with you when it comes to the pandemic and our response.

Ms O'CONNOR - Dr Veitch will have the pleasure of our epidemiologist, Dr Woodruff's company, at the table today. I note that you haven't commented on whether the state is adopting a herd immunity strategy -

Mr ROCKLIFF - Not to my knowledge, no.

Ms O'CONNOR - When you talk about 'living with COVID-19', is it the plan for Tasmanians to face repeat infection with a novel coronavirus that causes long-term health consequences for those infected, even those who are vaccinated?

Mr ROCKLIFF - I want to assure you, Ms O'Connor, and all Tasmanians, that we are removing restrictions safely, sensibly and in line with national and Tasmanian Public Health advice. We are taking sensible steps to remove restrictions, including -

Ms O'CONNOR - They are protections.

Mr ROCKLIFF - density limits, as an important step in Tasmania's ongoing transition to living with COVID-19. As I've said before, every death is a tragedy, and I feel every death as I see the information come through. I extend my condolences to all the loved ones, the families and friends of those affected by such tragedy. I could point to statistics to say that we have a relatively low death rate and hospitalisations compared to other states of Australia; but I know that would give no comfort to families that have lost loved ones.

Ms O'CONNOR - The Kirby Institute modelling which was commissioned by the Tasmanian Government, forecast that about this period after the border was opened, there would be about 70 thousand reported infections. We're now at more than double that and, as I said earlier, 79 people have died as result of contracting COVID-19. Has the state done any revised modelling on infections and death rates? I simply note to you, that now the national forecast for deaths to COVID-19 this year, is 20 000 Australians.

Mr ROCKLIFF - That Institute reporting was done with the Delta modelling in mind.

Ms O'CONNOR - Have you updated it?

Mr ROCKLIFF - Not to my knowledge. We have a very different variant. The Omicron variant is a lot more transmissible but, as you would appreciate, far less severe.

Ms O'CONNOR - As a result of vaccination only, according to the research.

Mr ROCKLIFF - Far less severe. You mentioned vaccination, and the importance of it. Other states opened up earlier than Tasmania, when it came to the percentage of vaccinated population. I stand corrected, but other states, opened up at 80 per cent. We waited until 90 per cent until we opened up our borders. I can only reinforce that message of the importance of being up to date with your vaccinations, which is the terminology.

Ms O'CONNOR - The *Scientific American* magazine has just released a paper, which is a study of 13 million people. It found that vaccination lowers the risk of long COVID-19 by only 15 per cent, which points to a flawed policy on the part of governments to rely on vaccinations only. You didn't answer my question earlier. Is it the plan for Tasmanians to face repeat infection with a novel coronavirus that causes long-term health consequences for those infected, even those who are vaccinated? What is the plan here?

Mr ROCKLIFF - We will continue to work alongside, and be guided by, not only our state advice; but public health gets regular reporting through the Australian Health Protection Principal Committee (AHPPC) and we're using that national advice. When I say 'we', public health is using national advice through AHPPC. We will maintain our policy of being guided by Dr Veitch and his team, and public health advice.

Ms O'CONNOR - Public Health has refused to confirm it is an airborne virus.

CHAIR - Ms O'Connor, do you want me to name you too?

Ms O'CONNOR - You need to warn people before you name them.

CHAIR - I am talking, you have had your quota of questions.

Ms O'CONNOR - I didn't get an answer.

CHAIR - You said before, that was going to be your last question on that line.

Ms O'CONNOR - I just wanted an answer.

CHAIR - I'm going to hand to Mr O'Byrne.

Ms O'CONNOR - Well, an answer would be good.

Mr O'BYRNE - Premier, cost of living is a major issue for many Tasmanians. The cost of energy is a major issue. Can you enlighten the committee on some of your Government thought processes? In 2017, your Government announced a winter energy supplement, which was inexplicably delivered in the middle of summer of 2018. You then announced a winter energy supplement in 2021. Could you enlighten the committee as to why those two years were picked, particularly given your Minister was saying that the regulated price of energy reduced in the 2021 year apparently on the basis of reduced bills. Why did you pick those two years to have a winter energy supplement, and why are ruling one out this year?

Mr ROCKLIFF - Energy concessions have increased by some \$32.8 million -

Mr O'BYRNE - The question is about the winter energy supplement.

Mr ROCKLIFF - I know it is - good question. Since 2012-13, while in real terms regulated energy prices have decreased by some 18 per cent over the past seven years for residential customers, we will continue to put downward pressure on power prices because we understand the cost-of-living challenges being faced by Tasmanians. Our energy policy since 2018 has been very clear. Following our election in 2018 and during a period of volatile mainland energy prices we fulfilled our promise in terms of legislated capped regulated power prices and a commercial industrial rebate scheme, measures that ensured that wholesale impacts being experienced by mainland customers did not immediately impact on Tasmania.

This occurred over three years from 2017-18 through to 2019-20 and following that period wholesale electricity prices stabilised. The Tasmanian Energy Regulator has set the prices for the past two years. Last year Tasmanian households received a 7.1 per cent reduction in regulated energy prices and small business received an 11 per cent drop. In fact over the past seven years, in real terms regulated energy prices have decreased by 18 per cent for residential customers and by 27 per cent, as I outlined in parliament last week if my memory serves me correctly, for small business customers.

We are now in a historic period for the National Electricity Market with the retirement of coal and the disruption from the war in the Ukraine. This is being seen across the country. The Tasmanian Minister for Energy will be meeting with his fellow energy ministers shortly to discuss these challenges. Without being political, there is a large piece of work for the new

PUBLIC

federal Labor government in this space and we will work with and alongside them, as we will do in a number of matters. However, we have said quite clearly that we remain steady to assess further concessions and supports that may be needed.

Ms O'BYRNE - Chair -

CHAIR - Mr O'Byrne, you have had your question. You know the rules, you are an independent member and I would appreciate it if you could follow the rules. Mrs Alexander.

Mrs ALEXANDER - What new initiatives are included in this year's DPAC budget and how are these additional initiatives going to benefit Tasmania?

Mr ROCKLIFF - To Mr O'Byrne's question, we are taking action on the cost of living. We are keeping Tasmanians safe, investing more into health, mental health, education and housing like never before. The Budget provides additional funding through the key deliverables, totally \$18.5 million across the forward Estimates, with a total \$9.9 million to be spent in 2022-23.

Some of these initiatives include \$3 million in 2022-23 to continue the operations of the COVID-19 Public Health Hotline; \$360 000 in 2022-23 for the continuation of the 2020-21 election commitment for a literacy advisory panel to support the development of a community-wide literacy framework; \$500 000 to support the coordination and implementation of the recommendations provided by Professor Kate Warner AC and Professor Tim McCormick in the Pathway to Truth-Telling Treaty report; \$2 million over four years to continue the Government's 2020-21 election commitment to reform Tasmania's planning system to ensure that it is fit for purpose, contemporary and efficient, with a total of \$710 000 to be spent in 2022-23; \$750 000 over two years, with \$250 000 to be spent in 2022-23 and \$500 000 to expand funding for the Premier's Fund for Children and Young People for a further two years, providing children and young people to more recreational and social opportunities in areas where they live; and \$4.96 million over three years to deliver on Tasmania's commitment to implement recommendations of the Royal Commission into National Disaster Arrangements.

There is \$3.2 million over four years to uplift information technology support for Service Tasmania, providing that organisation with the best capability to provide digital services to the Tasmanian community into the future. Members would appreciate that digital technology is a big focus of our Government, particularly with the recent announcement of some \$150 million over four years for our health digital initiatives. There is also \$1.6 million over two years to support a review of local government that will create a more robust and capable system of local government and is ready for challenges and opportunities for the future; and \$2 million over three years towards supporting the coordinating of the implementation of recommendations provided by Dr Ian Watt AC. This funding will support the development of changes to the State Service Act 2000 and employment directions, build shared capability in the Tasmanian State Service for dealing with serious investigations and improving the right to information processes, which has been a subject of discussion today. These are just some of the initiatives in the Budget that will be our focus.

PUBLIC

Ms WHITE - Premier, it appears that there was a secret election promise made by Jane Howlett as minister for sport to the JackJumpers basketball team that not even DPAC knew about. Doesn't this alone give reason to commence an investigation into what happened here?

Mr ROCKLIFF - I am advised that it did not form part of our election commitment but was negotiated after the election by the Premier's office. It was negotiated and agreed to by the then Premier, not Ms Howlett, and the letter of 23 April confirms that.

Ms WHITE - There are also letters from Jane Howlett as minister confirming it was an election commitment, which makes it more murky. It seems like it was a secret deal she had made and it had to be honoured. The RTI also shows that the department originally believed the funding was to go the Kingborough Council, the owner of that facility in Kingston, but the money ended up going to the JackJumpers. Can you explain that discrepancy?

Mr ROCKLIFF - All I can reiterate is what I have said before, Ms White. I have answered it -

Ms WHITE - Can you find out why there was a discrepancy?

Mr ROCKLIFF - I have answered it as succinctly as I have. The former Premier approved the funding. My advice is Ms Howlett had no involvement in those negotiations and her department was responsible for the payment process, not the approval process.

Ms WHITE - There have been lots of things said that sound like there is a bit of a cover-up here, including another RTI we have received where, again, the former Premier maintained that Jane Howlett had no involvement with the DEC redevelopment. Premier, how do you explain correspondence we have received through RTI between Mr Brookhouse and Jane Howlett, an email from him to her where he thanks her for her support of the project? I can read it for you if you would like. It says:

Hello Minister

On behalf of all of us involved with the MyState Bank Arena we just want to thank you for your support of this amazing project and more importantly for taking the time to attend Wednesday's launch. To have the ongoing support of the current Government is the foundation for the success of the arena's development and subsequent operations and your presence never goes unnoticed. Thank you.

Kind regards,

Simon

Why would he be corresponding directly with the minister about the DEC and thanking her for her support of this project if she had nothing to do with it?

Mr ROCKLIFF - Well, the former premier was the one who made the funding decision. He approved the funding. That is my answer; it is clear.

PUBLIC

Ms WHITE - You would have been in Cabinet when I presume Cabinet discussed the DEC. Can you confirm whether or not Jane Howlett was involved in those discussions?

Mr ROCKLIFF - I am not going into Cabinet matters, Ms White.

Ms WHITE - Can you not understand the concern about the potential conflict of interest here? There are mountains of correspondence we have only got through RTI, where there are conflicting statements between what you are saying, what the former premier said, what is in written correspondence between the former Sports minister about whether something is an election commitment, whether Mr Brookhouse was thanking her for her involvement, with her participation in projects and what you have said. There are other concerns we have too.

There was an allocation of an extra \$1 million of support for the relocation of the New Zealand Breakers. Do you know whether Ms Howlett was involved in the work that was undertaken to get that \$1 million allocated to the New Zealand Breakers?

Mr ROCKLIFF - There is a conflict of interest process for Cabinet that is followed -

Ms WHITE - Was it followed?

CHAIR - Ms White, Ms O'Connor's turn.

Mr ROCKLIFF - followed at each meeting. I don't go into matters of detail of Cabinet. These matters have been well canvassed with the former premier for a long time. You haven't spoken about much else, to be honest.

Ms WHITE - You want to lead with integrity.

CHAIR - Ms White, please let the Premier answer then we are moving to Ms O'Connor.

Ms WHITE - Premier, if you want to lead a government with integrity, do you not believe this needs to be cleared up?

CHAIR - Order, Ms White.

Mr ROCKLIFF - I do and I am leading a government with integrity and accountability, as I have demonstrated already.

Ms O'CONNOR - Premier, we didn't get any clarity on what the plan actually is in relation to COVID-19, other than rolling waves of infection in the community. Given that we are at hundreds of daily cases reported each day, 168 000 reported infections in Tasmania, getting close to one in three Tasmanians infected with a novel coronavirus that has long-term health consequences, do you believe the state is doing the best it can to protect the health of Tasmanians?

Mr ROCKLIFF - Yes I do, Ms O'Connor.

Ms O'CONNOR - Really?

Mr ROCKLIFF - We've done the best we can -

Ms O'CONNOR - You removed indoor mask mandates.

Mr ROCKLIFF - since the start of the pandemic and we were the first state to close our borders, way back in early 2020. Since that time, we have implemented a number of restrictions, which we are lifting in a sensible and measured way.

When it comes to the health and safety of Tasmanian people, it has always been the number one priority. Of course, vaccination is a key part of that. We have consistently led the nation, or almost led the nation, when it comes to our vaccination rates. That is still the point. Our planning, our preparation, including boosting the number of health workers and ensuring our surge capacity, means our health system is operating well with COVID-19. Yes, there are some challenges but the current impact on hospitals is significantly less than what we had prepared for, albeit we did do a lot of preparation. COVID@home would be a very good example of our preparedness.

Ms O'CONNOR - Are you done?

Mr ROCKLIFF - Yes.

Ms O'CONNOR - Okay. What you have just confirmed by not answering my previous three questions is that there is no plan on the state's part to do anything other than allow continual rolling infection. You would be aware in your capacity as Health minister that we have now got subvariants of Omicron - BA.4 and BA.5 -which are on the rise in other countries. What you call 'restrictions' independent epidemiologists and immunologists call protections.

Do you think it is good enough to have a public policy which doesn't let Tasmanians know COVID-19 is airborne, removes protections like masks from indoor settings and allows for hundreds of new infections every day? That, to me and Dr Woodruff, looks like a Public Health failure, as it does to Dr Raina MacIntyre from the Kirby Institute, who did the original modelling.

Mr ROCKLIFF - I am sure we will be able to talk further with respect to these matters in Health Estimates but our Public Health measures such as vaccination, our booster program, safeguards in high-risk settings, isolation requirements for positive cases, the continued physical distancing and good hand hygiene have helped us to avoid a very challenging scenario.

Ms O'CONNOR - You don't think 168 000 infections is a challenging scenario, given that the *Scientific American* paper shows, in a study of 13 million people, that the risk of long COVID is only reduced by about 15 per cent as a result of vaccinations?

Mr ROCKLIFF - I am advised that the state-specific modelling showed that if we were to implement no public health and social measures at the peak of the outbreak, we

PUBLIC

would see more than 630 people in hospital, almost 170 people in the ICU and, there would be more than 200 deaths -

Ms O'CONNOR - Is this based on the modelling that -

CHAIR - Ms O'Connor, please let the Premier finish without talking over the top of him.

Ms O'CONNOR - Thank you. I would like to understand what modelling the Premier is referring to, given that he has confirmed there is no updated modelling after the Kirby Institute's modelling, which projected 70 000 infections at this point after the border reopening?

Mr ROCKLIFF - Removing our border restrictions and reducing our quarantine and isolation requirements was not a decision that was made lightly. We knew that once they were eased, it was inevitable that COVID-19 would enter our community. Such was the case for other Australian jurisdictions when they opened their borders.

Ms O'CONNOR - Are you citing the old modelling -

CHAIR - Ms O'Connor, please let the Premier answer.

Ms O'CONNOR - because you don't have any new modelling?

Mr ROCKLIFF - I am advised that there is regular national modelling for AHPPC. The Public Health Emergency Declaration is being extended to 30 June 2022, as outlined. The Director of Public Health has determined that the public health threat posed by COVID-19 will soon be manageable -

Ms O'CONNOR - What does that mean?

Mr ROCKLIFF - with workplace health and safety practices and community continuing with COVID-19-safe behaviours. Positive cases and contacts will be managed like other notifiable communicable diseases. From 21 May 2022, density and capacity limits for all events and premises are no longer required. However, owners and operators are free to voluntarily use capacity limits as part of their ongoing management of COVID-19. In addition, only events with more than 5000 visitors and music festivals with more than 2000 patrons will need to have a COVID-19 event safety plan approved.

Our focus now is on our winter strategy as we prepare to manage both the COVID-19 and our influenza peaks should they occur separately in tandem.

I will go back to early May, with asymptomatic close contacts no longer having to quarantine. Instead, they will need to follow a set of rules for seven days to reduce the risk of potential spread, which includes testing, when they plan to leave home wearing a mask in all indoor settings outside of the home, avoid high-risk settings such as hospitals and residential aged care, and informing their workplace that they are a close contact.

Ms O'CONNOR - Still not getting any answers, Chair.

Mr ROCKLIFF - We are providing answers, Ms O'Connor, respectfully, and we have Health Estimates this afternoon where, of course, you are welcome to prosecute your arguments.

Ms O'CONNOR - I'll leave that to Dr Woodruff who knows her stuff here.

Mr O'BYRNE - Premier, again on the Winter Energy Supplement, thank you for your response. It didn't really answer the question. So, maybe you can answer this: can you confirm that in 2018 and 2021, the Winter Energy Supplement coincided with election years?

Mr ROCKLIFF - I can confirm that we had an election in 2018 and I can confirm we had an election in 2021.

Mr O'BYRNE - And those are the only two winter energy supplements that the Government have offered to people?

Mr ROCKLIFF - Of course, we are open to looking at further -

CHAIR - Mr O'Byrne, you have had your quota of questions. Passing the call to Mrs Alexander.

Mr ROCKLIFF - Very amusing.

Mr O'BYRNE - It's the reality though, isn't it?

Mrs ALEXANDER - Premier, how important is it that we invest in the wellbeing of children and young people? And what is the Government doing to deliver services that actually focus on that wellbeing for children and young people.

Ms O'CONNOR - Fifty thousand of whom have been infected with COVID-19.

Mr ROCKLIFF - Mental health and wellbeing is something I've personally championed for as long as I have been in this place. Knowing it leads to a happy and fulfilling life, it is something that I want for all Tasmanians. I know the importance of ensuring that this starts with our children and our young people. Last year we launched the state's first comprehensive, whole-of-Government Child and Youth Wellbeing Strategy, It Takes a Tasmanian Village. This \$100 million, four-year strategy provides a long-term direction for Government to improve the wellbeing outcomes for Tasmanian children and young people, aged zero to 25 years, with the focus on the first 1 000 days of life - ensuring they have the best start in life.

I am excited, as Premier, to lead the implementation of this strategy and today I want to outline the progress we have already made in its delivery including: a \$6.5 million expansion of the Bringing Baby Home service, in the north and south of the state, which provides pre and post birth support for parents at risk of having their infant removed. To date, 14 referrals have been received statewide for 2021-22, including both residential support and in-home support; establishment of \$1.2 million support in an Expecting and Parenting teams' program in the north-west which has included recruitment of a mentor, and 10 new clients

commencing the program in the north-west over the last three months; project and clinical planning are underway to establish the \$6.1 million Child, Health and Parenting Services Sustained Nurse Home Visiting Program, with the planned commencement of the service in August this year; and a new, two-page spread in the Child, Health and Parenting Services blue book, providing parents with some practical tips on supporting their baby's development. I have a copy with me here today.

The Sure Start project has provided two UTAS scholarships with 124 laptop services to high school students in the south and north who are in out of home care. Arrangements for tutoring provision in primary schools and driving lessons for young people over 16 are also being progressed, with the permission of a scholarship recipient under the Sure Start initiative. I will quote a letter which says:

Because of your generosity I am closer to my dream of becoming a nurse. A scholarship is especially helpful for me as I have recently moved out of my foster home. This means that alongside the costs of university I have taken on expenses, such as groceries and rent. Thank you for your support, it has brought me one step closer to my dream career. [TBC 10:53:14]

Stories like this are very heartening, and I am so pleased that the programs have been put in place are truly making a difference. We are delivering - including: a comprehensive professional development program for teachers, teacher assistants, wellbeing leads and principals on trauma informed practice will be undertaken by the Australian Childhood Foundation in partnership with the University of Tasmania, commencing in term 2 this year; additional support for families within child and family learning centres, with the filling of five professional support positions across the state;. completion of Youth Mental Health training for 75 school nurses across the state; and the establishment of a technical working group that will develop a nation leading approach to key indicators data and evaluation.

Other initiatives, such as a \$6 million Kid's Care Clinic for vulnerable children, are due to commence this coming financial year. I look forward to providing more detailed updates on some of our key projects under this strategy throughout the year. Thank you for your question, Mrs Alexander.

Ms WHITE - There is no doubt that the rising cost of living, shortage of housing, access to health care are the priorities for Tasmanians. And yet, your Government has committed to a \$750 million floating stadium in Hobart. These RTI documents we secured show that the costings were essentially done on the back of an envelope. How could you sign your Government up to something like this, when the priorities of Tasmanians are very different?

Ms O'CONNOR - He did not, his predecessor did.

Ms WHITE - He agreed to it when he came into the role.

Mr ROCKLIFF - I recognise the priorities for Tasmanians are health, education, child safety, and the health and wellbeing and safety of our children and young people. I'm also well aware of the cost of living challenges that all Australians are experiencing, as well as Tasmanians, and so the way you are politicising the stadium, in many respects, is not

particularly accurate either. Our Budget has committed \$1.25 million to the feasibility study of a new stadium, and I look forward to that body of work that is being done.

Ms WHITE - Premier, are you committed to build a new stadium?

Mr ROCKLIFF - I believe that a new stadium will be of benefit. I want to separate an AFL licence from this discussion as well. It's not just about an AFL team, it's also about events. Southern Stadium supports our aspiration for securing our own AFL team. I've mentioned before that it is more than just a stadium; it is about urban transformation and renewal and about delivering a world-class, multi-purpose venue that maximises local economic and social benefits.

The stadium will put Tasmania on the national and international stage for both sporting and entertainment events, on a scale that hasn't been possible in the past. It will be an important part of the state's sporting and entertainment evolution, and growing and supporting Tasmania's visitor economy. It will also inspire infrastructure and transport systems to support industry business and community growth, filling the city and the state with accommodation, hospitality, and retail activity as well.

Our goal is to develop a multi-purpose stadium that takes the best of today's stadium design standards and trends and has a flexible design that allows for future evolution of the facilities and infrastructure. Importantly, we can in Tasmania - just as in other states and territories - invest in transformational infrastructure and deliver investments in critical areas such as health, which I know Tasmanians care deeply about.

Ms WHITE - Premier, can you confirm that the stadium is not part of the bid for a Tasmanian AFL team?

Mr ROCKLIFF - In terms of the funding arrangements as well that, from day one, we have said the stadium will be funded through a multi-stream funding arrangement, including state, federal and private investment. No-one is expecting Tasmania will be investing that sum of money and, Ms White it's disingenuous of you to state that.

It does, however, need the sort of preparatory work that any major infrastructure project must follow. That is why we are funding the feasibility study of some \$1.25 million to determine the feasibility of a major sporting and entertainment facility in Tasmania, and to unlock those substantial economic and social benefits as have been seen in others states.

There was a recommendation of the AFL Taskforce, and will put Tasmania on the national and international stage for sporting, entertainment, and cultural events on a scale that has not been possible in the past.

Ms WHITE - Can you confirm it is not part of the bid for a team, though, Premier? That was the question.

Mr ROCKLIFF - A stadium is not contingent on a licence...

Ms WHITE - Is the licence contingent on the stadium?

Mr ROCKLIFF - While it is not contingent on the licence, which will be a known fact this year before the feasibility study is even completed, it is necessary to have the appropriate world-class standard infrastructure to support the success of an AFL team into the future.

Ms WHITE - Is the licence contingent on the stadium? Does the bid include the stadium?

Mr ROCKLIFF - We've been very open about the bid, in terms of the \$50 million upfront cost. We need to do the feasibility study, which we have committed to at this Budget of some \$1.25 million, as I have said. While it is not contingent on the licence, it is necessary to have the appropriate world-class standard infrastructure to support the success of an AFL team into the future.

CHAIR - As the time is now 11 o'clock, we will break for five minutes and return at 11.05 for people to have a rest break.

The committee suspended from 11 a.m. to 11.08 a.m.

CHAIR - We will now restart with a question from Ms O'Connor.

Ms O'CONNOR - Premier, do you understand that when governments say we have to live with COVID-19 that makes people who are disabled and immunocompromised afraid? Many people I speak to who are disabled or immunocompromised this sort of half-life where they rarely go out in Tasmania, where most people now don't wear masks in indoor or public settings. Do you understand that for the disabled and immunocompromised living with COVID means a very difficult life?

Mr ROCKLIFF - I would understand that, yes, and that's why we have been very measured in our approach to lifting restrictions and protections and have followed Public Health advice. The health, safety and wellbeing of Tasmanians has always been our priority throughout the pandemic, Ms O'Connor. We have a disability emergency operations centre which supports people living with disability and their support providers to respond to COVID challenges, including outbreaks in residential settings. Some people are more vulnerable to serious impacts from COVID-19 and we have prioritised these groups for vaccination and this includes those with disability. The Tasmania vaccination team has been working with disability peak bodies, offering tailored and information sessions to people with disability, their families and carers. Additional support such as quiet day clinics has been offered and additional support is available at clinics if needed.

A Public Health direction was signed on 29 March requiring that a person employed or engaged in providing high-intensity support to an NDIS participant must have received the booster dose of the COVID-19 vaccine by 23 April, or if not yet eligible then receive the booster within four weeks of becoming eligible.

As at 26 May 2022, NDIS registered providers reported that 96.8 per cent of disability workers required to be vaccinated had received two doses and a booster, while as of 19 May 2022, 88.6 per cent of NDIS participants aged 16 and over had received one dose and 84.1 per cent of participants aged 12 to 15 had received their first dose. The DOC works directly with disability support providers and the people with disability they support through outbreak

preparedness and response in management. Support may include support from exposure or outbreak declaration through to stand-down and beyond if necessary; linkages and liaison with Public Health in relation to infection, prevention and control measures; PCR testing and Public Health advice regarding contact definitions and isolation requirements; a liaison with Health ECC logistics to ensure residential disability settings have sufficient PPE and rapid antigen tests during exposures and outbreaks; liaison with COVID@home to ensure that people with disability are being appropriately prioritised and supported; and advice regarding maintaining social supports, safe and effective isolation in quarantine and previous learnings from other outbreaks. I could continue -

Ms O'CONNOR - Well don't, because it didn't actually answer the question. What you call a 'measured' response is refuted by independent experts like Dr Raina MacIntyre from the Kirby Institute who talks about the wearing of masks in indoor settings as a critical prevention measure. I've seen the information that is provided to people with disabilities or service providers and first of all it doesn't mention that COVID is an airborne infection, and secondly, it talks about what to do in the event of infection, it doesn't advise people on how to prevent infection. Why not?

Mr ROCKLIFF - Again, we can take these matters up with Public Health, as we said we would, Ms O'Connor, or Dr Woodruff will.

Ms O'CONNOR - This is a whole-of-government thing.

Mr ROCKLIFF - We have always been mindful of people in high-risk settings and that is why there are still restrictions in place for people -

Ms O'CONNOR - They're protections.

Mr ROCKLIFF - Protections, restrictions, whatever terminology you prefer - they are being maintained.

Ms O'CONNOR - Not for long; you've already foreshadowed that they will be removed.

I have one more question. Premier, have you been advised by Public Health that the risk of reinfection with COVID, and including the Omicron variant, is high? That means that people can be infected even though they're vaccinated and then be reinfected, and in some cases, according to the evidence, within a matter of weeks. Have you been advised that the risk of reinfection within the Tasmanian population is high?

Mr ROCKLIFF - Of course we have.

Ms O'CONNOR - Have you?

Mr ROCKLIFF - These are questions for Dr Veitch.

Ms O'CONNOR - Not entirely, because it is a whole-of-government response, and that's why you're getting these questions.

PUBLIC

Mr ROCKLIFF - This is a Public Health question and Dr Veitch will be able to provide you with the scientific analysis and evidence with respect to the pandemic and COVID-19. We have always worked very hard alongside Dr Veitch and those within Public Health.

Ms O'CONNOR - I didn't think that was a hard question. Have you been advised that the risk of reinfection is high, which it is?

Mr ROCKLIFF - I am not aware of this advice coming to MCEM.

Ms O'CONNOR - To who?

Mr ROCKLIFF - The Ministerial Committee for Emergency Management.

Ms O'CONNOR - So you haven't been advised that the risk of reinfection is high? That is confronting, given that the evidence is that it is.

Mr ROCKLIFF - You're putting words into my mouth -

Ms O'CONNOR - No, I'm not.

Mr ROCKLIFF - It was a detailed question that you had. These matters can be taken up with Dr Veitch and he can provide all the scientific information that you require with respect to these matters.

Ms O'CONNOR - But we're just confirming that you are setting public policy without the evidence. That's what's happening here.

CHAIR - Order, Ms O'Connor.

Mr ROCKLIFF - No, Ms O'Connor, we listen to the Public Health advice and set public health -

Ms O'CONNOR - They haven't given you good advice.

Ms WHITE - Have you asked for advice?

Ms O'CONNOR - No, they just don't want to know.

Mr ROCKLIFF - We do want to know, Ms O'Connor. We have worked very closely with Public Health over the course of the last couple of years.

Ms O'CONNOR - The evidence is incontrovertible that the reinfection risk is high.

Mr ROCKLIFF - We are always working alongside Public Health advice to ensure that we keep our Tasmanian community as safe as possible throughout the pandemic. We have done that.

Ms O'CONNOR - There are 170 000 infections.

Mr ROCKLIFF - I accept the infections. There are daily statistics when it comes to the matters pertaining to Public Health.

Ms O'CONNOR - Epic fail.

Mr O'BYRNE - Premier, my question is around housing. We know this is one of the major issues confronting Tasmanians and it is not new, it has been building for quite a while. Your Government announced an establishment of a housing authority at arm's length from government. A number of people in the community are wondering whether that is an opportunity for you to minimise political risk to your Government, because to push it to arm's length means you've got someone to blame. There is no money in the Budget for the housing authority that I can see but perhaps you can enlighten us on that. What is the scope of the housing authority? What will it actually do?

Mr ROCKLIFF - I reject the premise of your question in terms of us escaping accountability or scrutiny. Of course the Premier and Minister for Housing will always be held accountable to matters when it comes to housing and any other matter across government. We are pleased with the very strong policy we have that is reflected in the Budget for a 10-year \$1.5 billion housing package to build on our existing reforms. In the forward Estimates, there are some \$538 million, as you would be aware, in terms of investment into housing, and an additional 10 000 social and affordable homes by 2032. A new statutory authority will be charged with delivering these new homes through more affordable houses and units and ensuring we leave no stone unturned to deliver the stock of houses and services required to cater for growing demand.

The legislation to set up the new authority will be brought before the House for consideration, ready for the authority to begin operating on 1 October 2022. The minister will have more to say about the details of this legislation in coming days. Creating an authority with complete responsibility for housing in Tasmanian will help us to better address the housing challenges. It will create the most cohesive and integrated housing and homelessness service in the country, from housing supply all the way through to services like health support for our homeless. Corporate bodies are also used in other states to manage public housing, whether they are supported by a government agency or not. Our plan to create a housing authority has received strong support from across the sector and industry, showing we are on the right track.

CHAIR - I call Mrs Alexander.

Mr O'BYRNE - Just a point of clarification, Chair. You are establishing it in October of this year but there is no budget for it?

Mr ROCKLIFF - As I say, the minister will have more to say. I have provided an answer. The initial legislation creates the authority. It does not change any significant policy areas such as planning approvals. We are aware there are issues that are currently frustrating the delivery of our housing program and we are actively working through what changes may need to be made in that space. We intend to address these issues in separate legislation, if required, following the establishment of the authority planned for October 2022.

PUBLIC

The consultation on the draft legislation will also provide an opportunity for feedback on other issues, powers or functions that the new authority requires to address housing challenges in Tasmania. Funding will transfer from the current Housing Tasmania.

Mrs ALEXANDER - Premier, one of the five key domains of the social determinants of health is education. I am particularly interested in literacy. I would like to get a better understanding on how the Government is improving literacy for Tasmanians. I also understand there is the Literacy Advisory Panel. How is that working together to improve literacy for Tasmanians?

Mr ROCKLIFF - Thank you for your questions, Mrs Alexander. Improving our literacy levels is critical to ensure Tasmanians engage fully with their family, community and work. That is why the former premier established the Literacy Advisory Panel to oversee the development of a community-wide framework to achieve a literate Tasmania.

The Literacy Advisory Panel is co-chaired by the secretary of the Department of Premier and Cabinet, Jenny Gale, and the director of the Peter Underwood Centre for Educational Attainment, Professor Natalie Brown. The panel is reviewing current literacy programs and supports across the community, and will make recommendations for improvement of our approach to working towards achieving our aspirational goal of 100 per cent functional literacy in Tasmania.

In February this year, the panel released a consultation paper to gather information on current activities to improve literacy in Tasmania and to hear views on gaps, opportunities and supports needed to improve literacy in Tasmania. Consultation closed on 25 March this year.

There were 352 responses, including submissions, social media responses and responses from those with lived experience. I understand several recurring themes emerged from the community submissions analysis. These included the importance of the role of families and carers in promoting and supporting learners' literacy development; supports for children and young people and their families; teacher training and education, initial and ongoing professional development; timely and affordable access and interventions to specialist services, including allied health; lack of general awareness and understanding of the breadth of services available; a gap in explicit literacy supports for adults; and increased funding and access for literacy programs and services.

The panel will be releasing a draft of the community-wide framework for public consultation later in the year. It is my expectation that the panel will be delivering the community-wide framework to me by March next year.

Ms WHITE - That got blown out. Why is it delayed?

Mr ROCKLIFF - The framework was to be delivered in June 2022 but the panel requested more time to ensure comprehensive consultation. The new completion is 31 March 2023. Thank you for your question. Is it included as a question?

Ms WHITE - Definitely not.

PUBLIC

Mr ROCKLIFF - Developing the community-wide framework is complex and needs to be right to ensure it will support and improve literacy levels. Importantly, the panel is ensuring it engages meaningfully with the Tasmanian community, as I say, including those with lived experience of literacy challenges. I look forward to seeing the draft community-wide framework released later this year.

Ms WHITE - Is the stadium part of the bid for a Tassie AFL team? Yes or no?

Mr ROCKLIFF - I emailed you the bid.

Ms WHITE - Yes. I want you to publicly say that because this is important -

Mr ROCKLIFF - So you know it.

Ms WHITE - We've provided our support for this and if it's changed, then that's a problem.

Mr ROCKLIFF - No, it hasn't changed from the bid. I'm confused as to why you kept asking me the question -

Ms WHITE - It's a very straightforward question to answer then. So, you just confirmed that the stadium is not part of the bid for a Tassie AFL team?

Mr ROCKLIFF - It's part of the recommendation of the taskforce, of course, as I understand it and appreciate. But it's not part of the bid. The bid, as you know, is \$50 million upfront, it's \$10 million a year, \$100 million annually and \$150 million in total cost.

Ms WHITE - Can you confirm reports that were in the *Herald Sun* last week that just three games will be played in the north if we get an AFL team?

Mr ROCKLIFF - I'm unable to confirm that. There's a lot of work that needs to be done between the AFL, the AFL taskforce and others. I stand to be corrected. It doesn't sound correct to me but I can take that on notice if you'd like.

Ms WHITE - I can put that on notice, thank you. Premier, I want to ask you about cost of living. You've mentioned it a couple of times but do you believe there's a cost-of-living crisis in Tasmania currently?

Mr ROCKLIFF - I believe that many Tasmanians are experiencing cost-of-living challenges at this point in time.

Ms WHITE - Where in your Budget shows that you recognise that these challenges are growing?

Mr ROCKLIFF - There's \$305 million -

Ms WHITE - That's a cut from last year.

Mr ROCKLIFF - I've explained that it isn't. We've got \$305 million in concessions to help vulnerable Tasmanians with day-to-day living expenses such as electricity costs, water and sewerage, and council rates. The concessions provided by the Government make a real difference to those in need. An eligible electricity concession cardholder can receive \$514 in annual assistance with electricity costs, for example. From July 2022, water and sewerage concessions of up to \$211.50 will be available to eligible concessions cardholders - \$105.75 for water services and \$105.75 for water sewerage services. An eligible concession cardholder can receive council rates remission assistance of \$326 per annum for a TasWater customer or up to \$479 for ratepayers who are not TasWater customers. I will break that down further in terms of the \$305 million: \$79.2 million for council rates remissions, \$39.1 million for water and sewerage concessions -

Ms WHITE - Sorry, I didn't ask for the details.

Mr ROCKLIFF - Well, it is Budget Estimates - \$185.8 million for electricity concessions.

Ms WHITE - Premier, you'd understand that groceries and the cost of food is becoming incredibly difficult for some families to manage to put food on the table for their families. I understand that you had a request for additional food relief support that wasn't funded in the Budget. As a consequence, Loaves and Fishes has had to reduce its services. They're now only delivering down the west coast fortnightly, not weekly, as they did in the past. And they've identified particular challenges on King Island.

I'm not going to do what was done to members through the federal election campaign and ask you what the price of bread is but, for your information, the price of bread for Wonderloaf here in Tasmania is about \$3.70 a loaf. On King Island it's \$6.55 a loaf. The cost for a litre of milk on King Island is \$4.85, compared to about \$2.60 on mainland Tasmania. Unfortunately, because there was no additional relief in the Budget for food relief services, they are reducing their outreach to these communities. How are you going to ensure Tasmanians can afford to eat?

Mr ROCKLIFF - I'll come to Loaves and Fishes in a moment but I want to reiterate our support for Tasmanians. We were the Government that froze water prices for two years and capped them going forwards. We were the Government that waived school levies at a cost of \$14 million in relief to families and we also waived school debts.

Ms WHITE - That was a COVID-19 measure which we supported.

Mr ROCKLIFF - It is an example of how we are willing work with Tasmanians to support their needs. This Budget continues the Government's expanded Student Assistance Scheme which waives the cost of school levies. Now one in two students are no longer paying school levies. We have provided free sanitary items in all Tasmanian government schools. We are expanding the school lunch pilot to an additional 30 schools so that all the students have access to nutritious food and that is a \$1.4 million investment. We have increased funding over several years for emergency food relief and this Budget provides \$300 000 to support additional place-based food relief initiatives.

Ms WHITE - What does that mean? What is that for? Can you outline the details?

Mr ROCKLIFF - I will keep going. We are also continuing our \$5 million program for additional laptops and iPads. We are providing ongoing support to NILS Tasmania to help Tasmanians access financial support. We have provided funding through the family assistance program and we have introduced the FuelCheck app to increase fuel price transparency. I know the federal government's initiative on fuel excise has been welcomed. We have doubled the value of Ticket to Play in terms of sports vouchers.

We are delivering the lowest third-party car insurance premiums in the nation. We have delivered land tax relief as well and delivered the largest per capita social economic support package per capita in the country. I have worked with Loaves and Fishes a lot over the years. Nic Street is now our Minister for Community Services and Development. When I was Minister for Community Services and Development we worked on, consulted with and released Tasmania's first food security strategy. I welcome the commitments of both the major parties of the last federal election to support Loaves and Fishes in terms of their distribution centre. I believe - I stand to be corrected - that was a \$1.5 million commitment by both major parties and that will support Loaves and Fishes.

Ms WHITE - That's not for operational expenses, though, that is to purchase the building.

Mr ROCKLIFF - But it will also relieve, as I understand it, potentially their operational expenses as well. We will always work with our federal colleagues for organisations such as Loaves and Fishes and other organisations providing food security and food relief. We will go in to bat for them, as we have done, and support organisations such as Loaves and Fishes.

Ms WHITE - Why was there no money in the Budget for them?

CHAIR - Order, Ms White.

Mr ROCKLIFF - We have provided increased funding for food security services and released Tasmania's first food security strategy.

CHAIR - I am going to hand the call to Ms O'Connor.

Ms WHITE - I will be asking the minister where the action plan is.

Ms O'CONNOR - Premier, I know you take a close personal interest in the issue of climate change and how we mitigate it. I also know you are aware of a paper that was recently released by researchers at Australian National University and Griffith University because we've brought it on for debate in the parliament. That paper shows that Tasmania is one of the first places on the planet that has been able to not just reach carbon neutrality but negative carbon status; that is, we sequester more than we emit. Do you agree with the researchers' contention that it is as a result of the forests that have been protected and that they found that the mitigation benefit from forest protection is about 22 million tonnes of CO₂ a year? Do you agree that protecting our native forests is critical to our fight to bring down global emissions?

Mr ROCKLIFF - We already have the lowest emissions profile. Tasmania's emissions profile is the envy of the nation and indeed -

Ms O'CONNOR - Thank you, Tasmanian Forest Agreement.

Mr ROCKLIFF - the world. We have achieved our target of net zero emissions in six of the past seven years. We have achieved our target for Tasmania to be 100 per cent self-sufficient in renewable electricity generation. In 2019 our net emissions were 108 per cent lower than 1990, while our economy has doubled and over 60 000 jobs have been created. Our Government will legislate a target of net zero emissions or lower from 2030, the most ambitious target in the country and one of the most ambitious in the world. The Climate Change (State Action) Amendment Bill 2021 will put in place a nation-leading framework on climate change.

Tasmania's enviable emissions status and our ambitious targets will deliver brand benefits and bring investment and jobs while strengthening everything we offer and produce here, including advanced manufacturing, hospitality and tourism, science, irrigation, agriculture, aquaculture and resources. Economic analysis demonstrates that actions to achieve our 2030 target will both grow our economy and create more jobs. Our Budget allocates significant funding towards action on climate change, which I am sure you would welcome.

There is almost \$10 million over four years to deliver our next climate change action plan and \$10 million over four years to replace the Government's ageing fleet of fossil fuel boilers. We have also increased our investment to transition the government fleet to 100 per cent electric vehicles by 2030 to \$4.6 million. We are acting across our economy to reduce emissions including in our energy, transport, agriculture, industrial and waste sectors and we will continue to support industry businesses and our community to reduce emissions, adapt and become more resilient to the changing climate.

I have some information around timber -

Ms O'CONNOR - Do you want to call them forests or do you just want to call them timber like your colleague, Mr Barnett, does?

Mr ROCKLIFF - Our Government is a strong supporter of our renewable and sustainable native forest industry. Other governments in Victoria and Western Australia pursue their agendas. I read an article on ABC online in the last 24 hours of the impact in Western Australia, if my memory serves me correctly, of closing down the native forest industry there and the impact on jobs and families' livelihoods. Wood and wood fibre products are used for a myriad of products including house construction and are an effective solution to carbon storage and reducing emissions. When timber is harvested from our native forests it is done so in a sustainable way, Ms O'Connor, in accordance with our world-class forest practices system.

Ms O'CONNOR - You are joking!

Mr ROCKLIFF - I believe that. It is about managing your resources sensibly. I know your objective in closing down our native forest industry.

Ms O'CONNOR - I would like to get to my second question because you haven't answered my first one.

Mr ROCKLIFF - I thought I was actually doing that.

Ms O'CONNOR - First of all, I note that in your answer you didn't once refer to 'our forests', which was the basis of the question and the scientific paper I have cited. I just point out to you that it is a matter of public record that we export about 1 million tonnes of native forest timber as woodchips to China every year, which has a very low carbon life because it ends up in landfill. Do you agree that the continuing and increased protection of native forests is critical to your Government reaching, or maintaining, a net zero profile by 2030 off the back of forests that were set aside during the Tasmanian Forest Agreement as the research confirms?

Mr ROCKLIFF - I believe in sustainable management of our resources.

Ms O'CONNOR - So you believe a million tonnes of woodchips going to China every year and ending up in landfill is sustainable management? That's what happens.

CHAIR - Ms O'Connor, please don't put words in the Premier's mouth. Please let him answer.

Mr ROCKLIFF - Timber sourced from plantations makes up the majority of our total wood supply - approximately 75 per cent, I am advised. However, there is still demand, as you would appreciate, Ms O'Connor, for high-value appearance-grade forest products that can only be sourced from native forests and cannot be substituted by plantation wood. Half of Tasmania's forests, or approximately 1.79 million hectares, are protected in reserves. This includes more than one million hectares, or well over 80 per cent, of Tasmania's old-growth forests. I am advised -

Ms O'CONNOR - Remaining old-growth forests.

Mr ROCKLIFF - Less than one per cent of native forest on the permanent timber reduction zone land is harvested in a given year. Our renewable and sustainable native forest industry supports employment across the state, and we are committed to the long-term sustainable management of our forests to the benefit of all Tasmanians.

Ms O'CONNOR - Can I confirm what your Government's intention is, with the remaining 300 000 plus hectares of forest that your Government has removed from future reserve scheduling and called future potential production forests, and which your colleague calls a wood bank? Is it your intention to leave those forests unlogged, or will they be logged; and if they are logged, how do we meet and maintain our 2030 target? Also, will you build a bigger gaol to put protestors in, after your odious legislation passes through both Houses of parliament?

Mr ROCKLIFF - To your last question, it's about supporting people's right to free speech, which is -

Ms O'CONNOR - No, it's not.

Mr ROCKLIFF - It is.

Ms O'CONNOR - It is about backing in the corporates.

CHAIR - Ms O'Connor, please let the Premier answer.

Mr ROCKLIFF - Supporting Tasmanian's right to free speech, while ensuring the safety of, not only people that want to express their view - and they have every right to - but also ensuring the safety of the workplace is maintained and -

Ms O'CONNOR - You are not going to answer the question about the FPPF forests?

CHAIR - Ms O'Connor, please let the Premier answer.

Mr ROCKLIFF - I am advised 812 000 hectares of permanent timber production zoned land in Tasmania. Of this less than half, or 45 per cent, contains native forests that can be harvested. I want to give you an -

Ms O'CONNOR - Do you want to tell us what your plan is for the FPPF forests?

Mr ROCKLIFF - Perhaps if I can take that on notice, Ms O'Connor, and put the question to our minister for Forestry.

CHAIR - All right. We will move on. Mrs Alexander first, please.

Ms O'CONNOR - I can't get a straight answer out of him, so I thought I would try you.

Mr ROCKLIFF - All right. I will endeavour to give you a straight answer but -

Ms O'CONNOR - Thank you.

Mr ROCKLIFF - I'm sure our minister for Forestry will provide the answer for you.

Ms O'CONNOR - No, he doesn't. He just provides rhetoric and clichés.

Mrs ALEXANDER - Premier, could you update to the committee in relation to how the development of Tasmania's first wellbeing framework is progressing?

Mr ROCKLIFF - Thank you, Mrs Alexander, for your question. By any of the standard economic measures, Tasmania is doing very well. This is, of course, fundamentally important to wellbeing because it allows for security and certainty and an opportunity to invest more into services that matter to Tasmanians. However, economic conditions are not the only factors that contribute to the quality of life of Tasmanians right now. Governments around the world are recognising that economic growth alone does not account for a community's success or sustainable and inclusive growth, with health and happiness key success factors. Wellbeing is important to all Tasmanians, which is why former premier Mr Gutwein released the state's first Child and Youth Wellbeing Strategy to give our young

people the best chance in their early years. That is why I also announced that we will take the next step to developing Tasmania's first Wellbeing Framework, which I announced on the first day of parliament that I was Premier.

Wellbeing can mean different things to different people, but it includes: economy; health; education; safety; housing; living standards; environment and climate; social inclusion and connection; identity and belonging; good governance; and access to services. Having a set of wellbeing indicators will help prioritise where we need to invest more of our time, energy and creativity to make a real difference to Tasmanians who currently aren't sharing in the benefits of our prosperity in a way that they should. Putting wellbeing at the heart of our approach will also lead to improved whole-of-Government consideration of complex issues, and ensure a cross-agency approach is taken when developing policy and service delivery proposals.

The indicators in the framework will be informed by a series of measures that will use both people's subjective interpretations of quality of life and data that charts objective progress. Some indicators will draw on measures that are established, with a long history of information and data collection behind them. Others will require further exploration to effectively allow measurement and reporting due to a current lack of information, or data, but have been chosen because they have been identified as important by our community. Measuring these factors that drive the wellbeing of Tasmanians will help us evaluate policy and programs and guide future policy design and decision-making, and ensure these are being given the best chance of providing outcomes.

To do this, we will engage deeply with Tasmanians and ensure their wellbeing priorities are Government priorities, supported by the right targets, measures, policies and services. The framework will be developed over an extensive process of community consultation. It will commence in July this year with the release of a discussion paper that is being prepared with input from UTAS, which is undertaking inter-jurisdiction and international analysis. A series of community round tables will occur in various parts of the state, and Tasmanians will be able to register their interest to attend these later today. Should anyone want to attend, I encourage them to register with the Department of Premier and Cabinet website. I also intend to seek input from peak bodies of associations, various government advisory councils - particularly my Health and Wellbeing Advisory Council - and regional recovery committees. Thank you for your question, and you'll see some action on this matter in July this year.

Ms WHITE - Premier, you recently announced you'll be tabling legislation to increase the size of parliament to 35. Do you have an open mind about whether that's the existing five electorates of seven members, or will you create seven electorates of five?

Mr ROCKLIFF - Thank you for the question. I've outlined the reasons why we're increasing the size of parliament, should that be accepted by both houses of parliament, of course. There are two key principles when it comes to this matter. Firstly, it's 35 seats and secondly, it will be the Hare-Clark system. Clearly, we need - and I would expect - from Tasmanians that they expect us to put the priorities that they care about first, naturally, in terms of health, education -

Ms White - Cost of living.

Ms O'Connor - Housing.

Mr ROCKLIFF - cost-of-living, housing, all very important measures. My view, and I believe it is the view of the parliament as expressed the other day, is that we do need a functional parliament and importantly a functional committee system. I accept that it might not be popular, and I appreciate the tri-partisan support for the restoration, in terms of at least 2025, and we'll be consulting with the community. A bill will be put out for consultation. We'll be working through the matters that you've raised, in terms of five electorates of seven, or seven electorates of five. I've noticed some public commentary in relation to these matters, for various reasons. Our commitment at this stage is to increase the House of Assembly from 25 to 35 seats and we're committed to retaining the Hare-Clark system. We have heard feedback around electoral boundaries to ensure that the electorates provide a community of interest approach. I will be seeking further advice on the matter and, of course, any draft bill will be coming out for public consultation.

I have written to the Electoral Commissioner, Mr Andrew Hawkey, with respect to our recent announcement around 25 to 35 seats to ensure the parliament is best positioned to deliver outcomes for Tasmanians. I have indicated to Mr Hawkey that later this year it is my intention to introduce legislation into the parliament to restore the size of the House of Assembly to pre-1998 levels, which is 35 seats. This would come into effect at the next election, due in 2025. I reiterated to Mr Hawkey the key guiding principles of our approach: 35 seats and maintaining the Hare-Clark system.

But I did want to explore, given the recent community discussion around if there is a benefit in reforming electoral boundaries. I said that I would appreciate his advice, or the Electoral Commission's advice, on the consequences of revising Tasmania's existing electoral boundaries from five electorates to seven electorates, including but not limited to the impact on quotas and any costs of administering such change.

I note some of the arguments around the sharp geographic division of the electorate of Franklin, for example. I note the large electorate of Lyons and the electorates, using those examples, reflecting community interests. I stated that I am mindful of community discussion generated by the announcement in parliament and open to information and advice concerning such considerations.

I reiterated to Mr Hawkey that restoring the numbers in the House of Assembly is a necessary step to ensure we have a functional parliament and committee system that best represents and serves the people of Tasmania.

Ms O'CONNOR - Where are the purifiers?

Mr ROCKLIFF - Sorry?

Ms O'CONNOR - I am just asking the Chair where the air purifiers are if we are closing windows.

CHAIR - We are shutting the windows because of the noise that was occurring outside.

Ms O'CONNOR - Okay. I would rather not catch COVID-19, just to be clear.

CHAIR - Thank you for your opinion, Ms O'Connor.

Ms O'CONNOR - It is not just an opinion. It is a matter of my health and the health of the people in this room. Where are the purifiers?

Ms WHITE - When are you expecting to receive advice back from Mr Hawkey?

Mr ROCKLIFF - I understand that Mr Hawkey has acknowledged my letter and will come back with advice to the Department of Premier and Cabinet later this week, I am advised. It may take a bit longer around the boundary matters but I thought I would just seek some advice, Ms White. The Department of Premier and Cabinet has organised to meet with Mr Hawkey later on this week and I look forward to the receipt of his advice. But I want to be fully informed. It is important to take the community with us in terms of any change, as perhaps challenging as that might be in terms of increasing the numbers of politicians and the like. But we want a functioning democracy, a functioning parliament that ensures a robust committee system as well. That will be delivered by an increased parliament.

Ms DOW - Premier, can you rule out moving the Burnie Court complex to the Mooreville Rd site?

Mr ROCKLIFF - I thank you very much for your question, Ms Dow. I know that it is your local patch. You were the mayor of Burnie for a number of years and -

Ms DOW - It is your electorate too.

Mr ROCKLIFF - We share the electorate of Braddon, correct, and I am well versed on some of the matters and concerns of the community with respect to those matters. We have had an expression of interest process that, if my memory serves me correctly, closed on 18 May.

Ms DOW - When can we expect to understand the outcome of that expression of interest process, Premier?

Mr ROCKLIFF - It closed on 18 May. We recognise the need to provide some clarity as soon as possible. The expression of interest process resulted in a strong response, I am advised, a number of applications being submitted. For probity reasons it is not appropriate to comment on the number or the nature of events until after they have been assessed by Treasury. Following an evaluation of all the expression of interest submissions, our Government will be presented with a report that details the current availability of potentially suitable alternative CBD sites which will enable it to make a decision on the next steps for the project, anticipating that this will occur in June or July.

We have listened closely to the Burnie community and there is a firm agreement from all stakeholders that the current facilities are no longer fit for purpose and must be replaced, and also that the Burnie CBD is in need of revitalisation. We are committed to delivering a replacement court. I make that very clear. But it is important that all possible options are thoroughly explored before the decision is made.

Ms DOW - So, you won't rule out the Mooreville Rd site?

Mr ROCKLIFF - The former UTAS campus Mooreville Rd site was previously identified as a suitable Crown land site for a new court complex following a significant process to review and consider various sites. At the time it was selected, the site was determined to be suitable and appropriate due to some key factors. The Government already owns it. It is soon to become vacant. It can accommodate existing court functions and planned future expansion. At the time, there were no suitable sites in the CBD that were either owned by the Crown or for sale or available on the open market. But we are firmly committed to working with the local community with respect to these matters.

You may want to pursue greater detail from the Attorney-General in her Estimates and I am sure you will. But we do acknowledge that the Burnie community has a strong interest in the issue. I thank them for their feedback and engagement to date, and we will continue to work with them.

Ms O'CONNOR - Premier, this year the Tasmanian Law Reform Institute released their report on sexual orientation and gender identity conversion practices. I know this is in area in which you have significant interest and concern. Do you support the recommendations in the report? When will you respond to the report? Will you initiate the reform process to prohibit the unscientific and dangerous practice of conversion therapy?

Mr ROCKLIFF - Thank you for the question. I have read the report. I have met with people who have been through conversion practices, albeit through a Zoom meeting. I understand how much it has affected individuals to the detriment of their wellbeing, to put it mildly. I am interested in the recommendations of the report. There are some 16 recommendations, as you would be aware. Around half of those recommendations pertain to my responsibility as Minister for Health and Minister for Mental Health and Wellbeing. A number of other matters are the responsibility of the Attorney-General. The Attorney-General and I are working together.

Ms O'CONNOR - Do you support a ban on conversion therapy?

Mr ROCKLIFF - I support acting on the recommendations of the Law Reform Institute report. As I was about to explain, the Attorney-General and I are working together on these matters. I will be leading the change.

Ms O'CONNOR - There will be a change?

Mr ROCKLIFF - Yes.

Ms O'CONNOR - There will? That's what I wanted to hear.

Mr ROCKLIFF - I'm not going to have 16 recommendations and do nothing - I can tell you that right now. I'll be introducing legislation to the Parliament that gives effect to the recommendations.

Ms O'CONNOR - Thank you.

Mr ROCKLIFF - Naturally, there will be a consultation process. It is clear to me, as Minister for Mental Health and Wellbeing, that there needs to be change. I do take this very seriously. For example, a person shouldn't be considered to have a mental illness by reason only of their gender identity. I agree with that.

Ms O'CONNOR - That is right.

Mr ROCKLIFF - I've always stood strongly for equality and I will continue to do so; but, as you'd appreciate, it's a comprehensive report. Various agencies have been charged with considering the advice from agencies as soon as possible. The usual practice for receiving such a report is to obtain departmental advice, and at the present time the Attorney-General and I are seeking that advice.

Ms O'CONNOR - Can I ask also, on the matter of people born with a variation of sex characteristics, there was also a Tasmanian Law Reform Institute (TLRI) report which recommended law change, to prevent children who can't give consent, from being mutilated, effectively, at birth. I call on Government to act to outlaw these practices as well. Do you have a response to that issue?

Mr ROCKLIFF - Yes, I do. Again, this is really my responsibility as Minister for Health. I have met with advocates, as you will have done; Simone-Lisa Anderson is an example of that. As a Government, we value the research and analysis undertaken by the Law Reform Institute and I thank them for their continued work. Given the complexity of matters addressed in the 2020 Report on the Recognition of Sex and Gender, at my request, the Department of Health is undertaking detailed analysis of the report's recommendations regarding surgical intervention to alter the sex characteristics of intersex children. I understand that the sex and gender report along with TLRI's 2022 Final Report on Sexual Orientation and Gender Identity Conversion Practices, addresses interlinked issues surrounding sex and gender.

I have asked my department to work collaboratively with the Department of Justice, to examine these matters and to provide comprehensive advice on action required to respond. The work will consider policy perspectives and the legislative framework from the whole-of-Government perspective, to respond to the recommendations of the TLRI's recent reports.

I hope that provides a succinct enough answer for you.

Ms WHITE - I wanted to ask about the cessation of tenancies for people who are part of the National Rental Affordability Scheme (NRAS). It appears that your government did extend eligibility for the Private Rental Incentive Scheme (PRIS) to tenants in Redwood village, for which I thank you. Are going to extend the same support to all tenants, to ensure they were automatically eligible for the PRIS; or what else you can do to support those tenants as their tenancies come to an end?

Mr ROCKLIFF - I acknowledge the distress and concern this matter has given to a number of residents involved. I assure them that of course we will do everything we can to ensure they are assisted, and not forced into homelessness.

PUBLIC

I outlined in parliament last week, that our Minister for State Development, Construction and Housing, Mr Barnett, visited Redwood village last Monday night. The department has been working, as a priority, to look for solutions. The Department of Communities has been asked to work closely with residents leaving the NRAS to ensure they are offered whatever support they are eligible for under our Tasmanian programs. This may include the Private Rental Incentive Program, and the Government has made changes to that program to enable tenanted properties exiting from NRAS to be considered eligible for assessment under the state provided program. Indeed, the Budget includes that \$9.25 million to expand the program. The Government offers assistance under the program for those who are eligible, according to income thresholds, and the department will be working with residents through this process. We are doing everything within our power to try and keep these residents in their homes, noting that their properties are owned by private landlords and it is up to the landlord to agree to have their property in our private rental incentive program.

I do note the NRAS program had different eligibility than housing programs in Tasmania but where possible, of course, we will provide assistance to those tenants. We will also have to work with - and willingly - the Australian Government and the new Minister for Housing and Homelessness, Julie Collins. One of the most pressing issues is the winding up of the NRAS scheme and I know it causes a great deal of anxiety for many Tasmanians. It is a federal scheme. We will play our part and I hope and expect that the federal Government also plays its part.

Ms WHITE - You must have given an automatic eligibility for those tenants at Redwood Village to the Private Rental Incentives Scheme (PRIS) because some of them had previously been told that they weren't eligible. Will you grant automatic eligibility for all tenants in NRAS? I note you still use the caveat, 'if they are eligible', but we know some at Redwood Village weren't; but you obviously then fixed that for them?

Mr ROCKLIFF - As I say, Ms White, we will be doing everything in our power to try to keep residents in their homes, and we have demonstrated that in terms of the Redwood Village example. We will use that same effort and willingness to support other residents as well. I am sure the Minister for State Development, Construction and Housing will be more than willing and able to take similar questions and outline in greater detail not only his work with the federal Government, given it's a dual responsibility, but action to support our tenants as well.

Ms WHITE - Premier, I have one final question, which is on the Department of Communities and your resistance to halting that. You said that you will commit to implement all of the commission of inquiry recommendations. What if one of the recommendations is to keep children safe by keeping them separate from the Department of Education - keeping the Department of Community as it is, and separate from the Department of Education? Have you considered that, particularly given the commissioner's experience with the South Australian model?

Mr ROCKLIFF - Again, the South Australian model is different. The South Australian model -

PUBLIC

CHAIR - Premier, the time has expired for this portfolio, and we will now transfer across to the portfolio of Tourism, please. I will give you a couple of minutes to swap your staff across.

Mr ROCKLIFF - Thank you, and I thank Ms Gale and Mr Limkin and the Department of Premier and Cabinet for their Estimates preparation.

The Committee suspended from 12.08 a.m. to 12.11 p.m .

CHAIR - Premier, please introduce the people at the table, their names and positions for the benefit of Hansard.

Mr ROCKLIFF - Thank you, Chair. To my left the is the secretary of the Department of State Growth, Kim Evans, and to my right is Tourism Tasmania's representative, CEO John Fitzgerald.

CHAIR - The time scheduled for the Estimates of the Minister for Tourism is half an hour. Would the Premier like to make a short opening statement?

Mr ROCKLIFF - I will make it short, which means I might have to paraphrase.

Tourism employs more Tasmanians per capita than any other state or territory, equating to 14.9 per cent of the Tasmanian workforce. It contributes directly and indirectly \$3 billion per annum or 9 per cent of the gross state product, which is the highest contribution by tourism than any other state or territory. The key motivators for travel remain our natural landscapes and wildlife, with our world-class food and beverage products becoming increasingly powerful.

Research shows clearly that we are in an incredibly powerful position, with a nationally unmatched level of blatant demand for what we have to offer. The competition will be fierce for the travel dollar as the world continues to reopen and it is critical that we do not stand still and expect travellers will naturally find their way to our wonderful island. That is why we cannot take our current and recent successes for granted and why our Government will proactively encourage and support innovation and investment in and via our industry.

Product stagnation and lack of awareness of the motivators and priorities of today's travellers are the most critical dangers to the health and sustainability of our visitor economy. More immediate threats to our industry and economy in general is the challenge we face along with the rest of the nation in recruiting the suitably skilled workforce that is obviously required to deliver the products that visitors travel here to experience. The issue is being considered as an absolute priority by both the T21 committee and the Premier's Visitor Economy Advisory Council which I chair.

The reality is that this will require a concerted effort and strategy in partnership with the Australian Government and those conversations will be had and I have reached out to Don Farrell, our new Trade and Tourism minister, and congratulated him on his success. Our focus will remain firmly on yield and not numbers or volumes of visitors. There are plenty of others that play in that space and, to be frank, we are not interested in competing for that type of traveller.

Since the onset of COVID-19 we have delivered more than seven financial support programs and provided financial support to our peak tourism bodies including the Tasmanian Hospitality Association, the Tourism Industry Council -

Ms O'CONNOR - Can I just check you're not going to read that whole extra page.

Mr ROCKLIFF - and Business Events. I said I am paraphrasing. We will have a genuine focus on bringing our state's Aboriginal story as well to the floor and to taking a unique convict heritage story to another level. It is also very much about acknowledging the critical importance of finding a balance between tourism and our environment which is obviously a primary motivator, with both being intriguingly linked on many levels and another reason for us to invest in world-class infrastructure in order to host and stage events as well as elite sports fixtures which contribute most massively towards our better economy, all of which provides us with resources to invest more in critical areas of support our community.

In winding up, a joint commitment with industry for it to become carbon-neutral by 2025 is an exciting opportunity and I can advise that a pathway has been identified and that industry has responded with great enthusiasm. The future looks incredibly positive but we must continue to work together with industry and make every effort to stay ahead of what is an aggressive and well-resourced competition. I cut that opening statement in half in the interests of time.

Ms O'CONNOR - With half an hour for these Estimates and a four-minute opening statement, not good.

CHAIR - Thank you, Ms O'Connor.

Ms WHITE - Premier, I wanted to ask you about cruise ships. On 17 April this year the Australian Government lifted the biosecurity determination that prohibited international cruise ships. When will your Government will permit cruise ships to return to Tasmania?

Ms O'CONNOR - Do you want them here, Ms White?

Ms WHITE - I am asking the question because on their website they have some information that it says work is underway on a policy. I would like to know the details of that please, Premier.

Ms O'CONNOR - Shipping in COVID by the shipload - let's do that.

Mr ROCKLIFF - We recognise the importance and contribution of cruise shipping to the state's visitor economy. There are approximately 100 tourism operators across Tasmania that have been directly contracted in the cruise line shore tour programs that are also independent to operators, retail centres advising businesses that benefit from cruise shipping, in addition to direct expenditure from cruise ship passengers. In 2019-20, direct expenditure in Tasmania from cruise ship passengers and crew was estimated to be some \$48.7 million from 124 port calls.

PUBLIC

To your question, in April the Australian Government lifted the biosecurity determination that prohibits international cruise ships from arriving and departing from Australian ports. We want to ensure that cruise ships return to Tasmania in a way that protects the health and safety of our communities as well as the crew and passengers on board the vessels. With this in mind, our public health authorities are finalising a set of protocols for the cruise ship industry that will be in place prior to the return of cruise ships to Tasmania, likely to be in October this year.

In developing these, we have been carefully considering recent national progress including the Eastern Seaboard Cruise Protocols and the Communicable Diseases Network's Australian guidelines for cruising to ensure we have something place that aligns with these and works in the Tasmanian context.

Ms WHITE - You have outlined that Public Health is developing some protocols ahead of the likely return in October this year. In those conversations in developing policy, what sort of protocols are you talking about?

Ms O'CONNOR - Anything goes, Ms White.

Mr ROCKLIFF - We have always followed Public Health advice with a range of matters including our borders opening and this will continue when it comes to the Public Health advice we will seek with respect to the return of our cruise ships.

Ms WHITE - Thank you, Premier, but that is not really an answer. On the website it says:

Work is underway on a policy to support the safe resumption of cruising in Tasmania, with guidance to be provided for industry ahead of the next cruise season.

You have outlined that it could be as soon as this October that cruise ships return. How are you going to consult with the industry about the return of cruise ships? It is only a few short months away. How are we going to prepare the community? You seem very vague about what public health protocols might be in place. Do you have any advice that you could be sharing with us, please?

Mr ROCKLIFF - In terms of consultation of the industry, perhaps Mr Fitzgerald can respond?

Mr FITZGERALD - I think the plan is, once Public Health has considered how they believe cruise shipping's going to resume, my advice is that they intend to consult with the industry. My understanding is that that is going to be relatively soon, but I don't have an absolute timeline on that.

Ms WHITE - So you are still waiting for Public Health.

Mr FITZGERALD - Correct.

PUBLIC

Ms WHITE - Premier, I wanted to ask about the media you did in the lead-up to the Budget, where you announced an additional \$10 million for tourism. Can you confirm that was money you had already announced a year earlier at the election? Because you would know as well as I that the election was held a year early, which meant that there was still an allocation of funding in the Budget for the financial year that we are currently in, that then fell off a cliff.

Mr ROCKLIFF - I have: in 2022-23, a marketing expenditure of \$6 million; 2023-24, \$6 million; and 2024-25, \$6 million. I have added securing, recovering, and maximising Tasmania's visitor economy future 3, 3, 2, and 2, totalling 10 over the forward Estimates to 2025-26.

Ms WHITE - That is additional from the election last year when you made that announcement?

Mr ROCKLIFF - Well, it is additional funding in this Budget, yes.

Ms WHITE - No. It is additional from the announcement that was made last election?

Mr ROCKLIFF - Well, I think we have just answered the question, Ms White, in terms of the 2021-22 election commitments, marketing expenditure. 6, 6, 6, and we have since added that, I am advised.

Mr FITZGERALD - That was your election commitment.

Mr ROCKLIFF - Including the visitor information technology of \$500 000?

Mr FITZGERALD - Correct.

Ms O'CONNOR - Premier, the Tourism Industry Council of Tasmania states publicly, and to inquiries from the public, that they receive no Government funding, and that they are not for profit; and so have no requirement to produce an annual financial report for members. The TICT denies its own members that report, when it is requested. As you know, Tourism Tasmania is a state authority run by Government, and Tourism Tasmania's annual report shows that it contributes \$200 000 each year to the Tourism Industry Council. Are you able to tell the committee what accountability there is for the TICT to report on this expenditure of public funds?

Mr ROCKLIFF - I am advised that the TICT is an important part in the development and implementation of the T21 strategy, which was spoken about before, to support the sustainable growth of Tasmania's visitor economy. We have provided \$600 000 funding -

Ms O'CONNOR - Over what period?

Mr ROCKLIFF - Support for the three years to 30 June, 2022, to assist the TICT to deliver three specific programs for the benefit of Tasmania's tourism industry. That is administered via a grant deed by Tourism Tasmania. Funding has been paid in annual instalments of \$200 000, which I think is where you are getting your \$200 000 from, is it?

Ms O'CONNOR - Yes.

Mr ROCKLIFF - Supporter programs include: administration of the National Quality Tourism Accreditation Program in Tasmania; sponsorship of the state's major tourism event, the Tasmanian tourism conference held annually in Launceston; and sponsorship of the Tasmanian tourism awards, with winners progressing to the national titles. Quality tourism accreditation provides a practical program for tourism operators to develop business and operational plans aimed at raising professionalism and improved business operations to deliver quality service.

Now, the funding contribution for the accreditation program has been \$120 000 annually, which enables the TICT to deliver the program at an affordable rate for tourism operators. We have 1 050 businesses that are currently involved in the accreditation program and its various specialised modules, representing 1 150 tourism products and experiences as businesses may have multiple products under one accreditation. Given the important support and advocacy role TICT provides for the industry, especially at this critical time given the disruption that we have had with respect to the pandemic, I am advised that a similar level of funding will be negotiated each year.

Ms O'CONNOR - We have just established that TICT CEO, Luke Martin, has never confirmed, and that is that there is a significant level of Government funding that goes towards that industry body. This year it will be \$420 000 so, over the next three years it will be more than a \$1 million of public money that goes towards the TICT which does not produce an annual report or any reporting information to its members. Do you think that is satisfactory in terms of an entity that is an industry body - in significant part, funded by Government and member fees - that there is no transparency about their finances or how they spend public money?

Mr ROCKLIFF - We are all about transparency. They do have to acquit their grant deed.

Mr FITZGERALD - They have to acquit their grant deed. It is a formal grant deed. Certainly, for the \$200 000 we provide them, they have to acquit that annually and indicate quite formally as to how that has gone, and have that approved by us for release.

Ms O'CONNOR - Thank you. We get feedback from tourism industry participants about their dissatisfaction with the representation they receive from the TICT; the way it backs in the Government's native forest logging; and the way it takes public funds and is not accountable for those funds or transparent about its operations to its own members.

Finally, on this line of questioning, do you agree that there is a disconnect between the TICT CEO, Luke Martin, when he states they get zero government funding, but actually receive more than \$320 000 a year, at least for the next three years, from Tourism Tasmania? Do you think that it would do the TICT well, in terms of member faith in the way it operates, to be more transparent about how it spends its public money as well as its member fees?

Mr ROCKLIFF - Thank you for the question. Mr Fitzgerald has outlined matters regarding the deed and the transparency.

Mr FITZGERALD - That is the government's funds.

Mr ROCKLIFF - I understand from your line of questioning that you are saying that some people are not happy with the TICT.

Ms O'CONNOR - They do not get an annual report and do not get accounting for the TICT's operational expenditure.

Mr ROCKLIFF - Any further information on that, Kim?

Ms O'CONNOR - Their own members, who pay fees to this organisation, get treated like mushrooms.

Mr EVANS - I just checked on the State Growth side and we have one funding deed with the TICT, which is \$60 000.

Ms O'CONNOR - Annual?

Mr EVANS - No, it is a one-off with the TICT, to conduct a pilot ambassador program to develop ambassadors for the tourism industry. As Mr Fitzgerald said, that is subject to a funding deed with strict milestones and reporting requirements. For the sake of completeness, I thought I should add that.

Mr ROCKLIFF - You mentioned \$320 000, Ms O'Connor. As I said, administered via grant deed by Tourism Tasmania, funding has been paid in annual installments of \$200 000. When I spoke of the \$120 000 funding contribution for the accreditation program, we spent \$120 000 annually, enabling the TICT to deliver the program at an affordable rate for tourism operators. My advice is that that is within the \$200 000.

Mr FITZGERALD - That's part of it.

Mr ROCKLIFF - The total is not \$320 000; the \$120 00 is within the \$200 000.

Ms O'CONNOR - For clarity then, initially you said there was \$200 000 per annum for three years for a range of activities. There was, then, this \$120 000 which is the accreditation money, which we were told was annual. But what you are saying is the annual accreditation money is absorbed into this three-year grant of \$200 000 a year?

Mr FITZGERALD - That is correct.

Mr ROCKLIFF - That is right.

Ms O'CONNOR - Are you able to make public those deeds with Tourism Industry Council, because members are really dissatisfied with the lack of transparency on the organisation's part.

Mr ROCKLIFF - Firstly, if members are dissatisfied, I would encourage them to make their feelings known to the TICT.

Ms O'CONNOR - They have, and they get fobbed off.

Ms WHITE - Premier, do you accept that the short-stay accommodation sector has an impact on traditional BnB and tourism accommodation providers, and that it also has an impact on the availability of properties for a workforce servicing the tourism industry, particularly in our regions because they cannot find anywhere to live? Why hasn't your Government done more to regulate the sharing economy to limit the listing of whole homes?

Ms O'CONNOR - They represent the propertied class, Ms White.

Mr ROCKLIFF - Thank you for the question. I will just seek some answers for you.

Ms WHITE - Surely you don't need advice to answer those questions and accept there is an impact on workforce shortage because they cannot find a house because the sharing economy is taking up a lot of those properties.

Mr FITZGERALD - Ms White, I was just looking at some data recently. Whilst forward bookings are strong for the Airbnb sector, my recollection - I would need to just confirm - is that there are 1500 fewer Airbnb properties in Tasmania now than there were pre-COVID. That is the only data I have to try to give you some advice on your query.

Ms WHITE - Where do you get that data from?

Mr FITZGERALD - We have an arrangement with a company called AirDNA. It looks at all the Airbnb forward data. It tells us how many properties there are in the market compared to when they were previously. I am recollecting my view of one of my slides. I am sure there are 1500 less Airbnb properties currently in circulation than there was in 2019.

Ms WHITE - Premier, would it be possible for me to put a question on notice, perhaps to clarify that is an accurate recollection.

Mr FITZGERALD - Happy to provide that. It is data we share with the industry so it is public data.

Ms WHITE - I am guessing that is on the TBS stuff as well. I am curious because the Government started collecting their own data a couple of years ago. How have you used that data, Premier, to inform the decisions of government, particularly around the regulation of the sharing economy? It is not just about the competitor and the accommodation info, but it is about the availability of housing for our regional tourism workforce and broader workforce.

You have been collecting data for years now. What are you doing with that and how are you using it?

Mr ROCKLIFF - I would be happy to take that on notice along with the clarification of the question asked of Mr Fitzgerald, if you like, Ms White, in terms of the data that is presented and the policies we have formulated around it.

PUBLIC

Ms WHITE - Okay. Premier, I wanted to ask you about VXT. There is no additional funding for VXT in the budget, which is the RTO that the Tourism Industry Council and THA were provided support to establish. You seem a bit unsure about what I was meaning.

Is it your intention to cease funding the VXT?

Mr ROCKLIFF - The Tasmanian Tourism and Hospitality Training Organisation was established with a five-member board appointed in December 2020. That included two representatives from the Tasmanian Hospitality Association and the Tourism Industry Council of Tasmania. On 10 June 2021 THTT launched a new brand and name and the organisation, as you say, is now known as the experienced training VXT. To date, VXT has run 40 courses with a total of 262 participants. The 2020-21 Budget included \$1 million to fund the establishment of the tourism and hospitality-led training organisation to deliver quality training and courses in areas where gaps currently exist within the sector. Funding for the organisation's establishment and initial operation is provided through a grant deed administered by the Tourism and Hospitality Support Unit of the Department of State Growth, with \$500 000 in funding paid in the 2020-21 financial year and the remaining \$500 000 paid in the 2021-22 financial year, on 10 June last year. I've mentioned the change of name.

In the first 12 months of operation, accreditation and non-accredited training was to be brokered, coordinated on behalf of industry from other providers, with VXT delivering some non-accredited priority training as well. It complements private training providers and TasTAFE Drysdale to expand the market and offer industry-informed training programs.

This funding demonstrates our Government's clear commitment to improve training options for these important sectors and fulfils our long-term aspiration to grow our skilled Tasmanian hospitality and tourism workforce. Got any further information on funding?

Mr EVANS - No. The only thing I might add is that this is obviously a critical issue within the tourism industry and is probably the highest priority on the agenda of T21 steering committee, which John chairs. We are leading some work with the tourism and hospitality workforce advisory committee and my agency, including the various partners in T21, to develop a new workforce development plan. We are talking to VXT and we will be looking at their place in any workforce solutions and the funding required from that. Obviously, it wouldn't come through these budgets. It would come through Skills Tasmania budgets moving forward.

Ms O'CONNOR - Minister, the Tasmanian Walking Company is the only commercial operator allowed to operate on the Three Capes Track. They have two private luxury lodges to accommodate their guests and they pay a token fee for the lease on public land. This is a great deal for them, obviously. In a secret deal, it was revealed this year via a right to information request, that the company has received hundreds of thousands of dollars in marketing grants - public money, a luxury not afforded to other tourism operators to our knowledge. Will this largesse continue? Do any other tourism companies receive similar marketing grants?

Mr ROCKLIFF - I am advised that it's perhaps a question you could put to Mrs Petrusma as the Parks -

PUBLIC

Ms O'CONNOR - But Tourism would know about these sorts of arrangements, I think, where the taxpayer funds the marketing of a private company.

Mr ROCKLIFF - more specifically for Parks' responsibility.

Ms O'CONNOR - So, you don't know of any other situations where private companies operating inside public protected areas receive public money in order to market their business? That is a matter for the Tourism minister, isn't it?

Mr ROCKLIFF - You are not going to put words in my mouth.

Ms O'CONNOR - I am just asking.

Mr ROCKLIFF - Your original question is in the responsibility of the Minister for Parks unless, John, you can provide any further information?

Mr FITZGERALD - No, it is not in our bailiwick, Premier. It is very much a Parks marketing initiative.

Ms O'CONNOR - Does Tourism Tasmania provide marketing grants to other companies that operate either on public or private land?

Mr FITZGERALD - No, we don't.

Ms O'CONNOR - No. So, it is unique to Parks to hand out public money to a private operator to profit from public land. Interesting. Thanks.

Mrs ALEXANDER - Premier, my question is around the situation we currently have post-recovery, which is a very highly competitive market in terms of tourism, and it would be interesting for me to understand how Tourism Tasmania has adapted its strategy and its marketing efforts in order to ensure that we still remain quite a valuable and sought-after tourism destination for travellers.

Mr ROCKLIFF - The importance of targeted and effective marketing cannot be overstated in what is a highly competitive environment, both from a competing destination perspective but also in a climate that has seen travellers and consumers behaving quite differently to what was experienced prior to the pandemic. This is particularly true for Tasmania, a small state that requires every effort to create awareness and overcome barriers to travel. In marketing parlance, Tasmania is a challenger brand which requires destination marketing to break through all the competing noise and clutter in the marketplace, particularly in the sea of sameness that is apparent to the travel pedigree.

CHAIR - Thank you. The time has expired for the session. If we can swap over as quickly as we can to the Trade portfolio, please.

Ms WHITE - For what it is worth, we did want more time in Tourism.

Mr ROCKLIFF - I accept that. I have to apologise - 10 hours today.

PUBLIC

Ms WHITE - That's why we wanted you to come back for another day, Jeremy.

Mr ROCKLIFF - Ms Giddings had 7.5 hours when she was premier, Ms White.

MINISTER FOR TRADE

CHAIR - I invite the Premier to introduce the persons at the table, their names and positions for the benefit of Hansard, please.

Mr ROCKLIFF - To my left is the Department of State Growth secretary, Kim Evans, Lara Hendriks, the executive director of Trade, and to my right is Mark Bowles, deputy secretary, Business and Jobs.

I have an opening statement and I will be very brief. I am pleased to be back as Minister for Trade. The time is right to refocus our trade efforts after the disruption to markets we have seen worldwide since early 2020. While Tasmania has weathered the pandemic well, it has not been without its difficulties, coupled with the region's geopolitical tensions and the Russian-Ukraine war. It has been and will continue to be a challenging period for exporters in the foreseeable future.

To the credit of our businesses, through their resilience, perseverance and innovation, and our strong economic response, we have seen Tasmania's export trade defy the odds. We continue to deliver record-breaking trade levels, with ABS data confirming that Tasmania has experienced, month on month, annual growth reaching a good export value of \$4.67 billion annually as of April 2022.

The report card is also good news. It shows that our state's goods exports managed 5.2 per cent growth across that year. Much of it is due to our vital minerals industry and high commodity prices. Our services exports contributed \$646 million despite international borders remaining closed, which is an extremely strong result given almost 90 per cent of our service exports are either international education or tourism.

Notably, the report card showed our combined goods and services exports reached \$5 billion during 2020-21, keeping us well on track to achieve our ambitious target of \$15 billion in export trade by 2050.

We released the trade strategy in 2019. It was purposely designed at the time with the intent to support Tasmanian businesses to remain resilient and competitive, and respond to global market adversity and uncertainty such as events like we have experienced with the pandemic. With an event of this scale, the strategy was certainly put to the ultimate test and it demonstrated its effectiveness. A consistent theme in our trade response and actions is to continue to encourage good business practice, including risk reduction through market product and service diversification, with a focus on developing opportunities for new jobs, investment and growth for Tasmania. As Premier, one of my highest priorities will continue to be to ensure Tasmanian businesses continue to access the support they need to realise the opportunities offered by the global economy.

Dr BROAD - What is the total allocation to the Trade portfolio this year and across the forward Estimates?

Mr ROCKLIFF - There is \$1.8 million for the trade strategy but in the State Growth portfolio there is industry and business development of trade. Total funding in 2022-23 is \$5.982 million, in 2023-24 it is \$9.61 million, in 2024-25 it is \$3.957 million and in 2025-26 it is \$2.012 million.

Dr BROAD - Why isn't that spelled out in the Budget? Why is it allocated into output group 1.2? Why isn't that separated out?

Mr EVANS - In terms of the specific funds allocated to Trade, the Trade budget is embedded in the broader business and jobs Tasmania budget. The reconciliation the Premier just gave you is the actual situation. The reason it drops away in the out-years is that in this current Budget we have received additional funding of \$1.8 million over two years to keep funding the trade and investment mission plan through to 2024. In future years those decisions will be considered by the Government as part of future budget processes.

Dr BROAD - How many FTEs are directly attached to the Trade portfolio?

Mr ROCKLIFF - Would it help if I gave some salary costs? I am always trying to be helpful. In 2022-23 it is \$2.13 million, in 2023-24 it is \$2.056 million, in 2024-25 it is \$1.96 million and in 2025-26 it is \$1.872 million in terms of salary costs, with operational costs of \$39 million, \$40 million, \$40 million and \$40 million respectively across the forward Estimates.

Mr EVANS - The actual number of FTEs is currently 12 but we are in the market for two additional staff as we speak, and that is not accounting for our in-market presence in terms of trade advocates and representatives that we have bolstered in the last 12 months or so.

Dr BROAD - How many of those FTEs are based outside Tasmania and how many are overseas?

Mr ROCKLIFF - We have three trade advocates based in the US, Singapore and Japan.

Ms HENDRIKS - Through the Premier, there is Vivian in Shanghai as well. She is an industry business development manager.

Mr ROCKLIFF - So that is four FTEs then?

Ms HENDRIKS - Yes, in support of the trade advocates on top of the 12.

Mr EVANS - We've got 12 staff and then we have the in-market presence.

Dr BROAD - Which is an extra four.

Ms HENDRIKS - That is correct.

Mr ROCKLIFF - So that is 16 in total, I am advised.

Ms HENDRIKS - Through you, Premier, the ministerial portfolio that you would see in your output papers also includes science and technology, which also sit under 'mean trade,' which equates to 21 FTEs in total.

Ms O'CONNOR - Minister, under the Australian constitution, as you know, the federal government has responsibility for trade. In this portfolio you have no legislation to administer, attend no cross-jurisdictional meetings as minister and administer no outputs in the Budget. Although you're attending the upcoming trade delegation to New Zealand your media release says you're attending as Premier. All previous trade missions have been led by the Premier, including in 2016 before the Trade portfolio was established. We regard the portfolio of Trade really as redundant in terms of trade delegations. Minister, what do you do in this portfolio? Is your portfolio just a waste of taxpayer money for the purpose of political messaging?

Mr ROCKLIFF - No, it's not.

Ms O'CONNOR - What do you do in this portfolio that you don't do as Premier?

Mr ROCKLIFF - I recall as Trade minister previously, if my memory serves me correctly, we launched the trade plan in around 2019, which I think I've already explained. As well, our trade action plans and trade strategy 2019-25 will expand and diversify Tasmanian export interests and ensure the Government's support and initiatives align with market-driven growth opportunities across all sectors.

Ms O'CONNOR - This used to be done by the minister for economic development or State Growth.

Mr ROCKLIFF - We have a federal minister for trade in Don Farrell.

Ms O'CONNOR - Because it's a federal responsibility -

Mr ROCKLIFF - Yes, but I am sure there have been trade ministers in various other states, so it's a whole-of-government approach in many respects because trade -

Ms O'CONNOR - Like the Premier.

Mr ROCKLIFF - covers primary industries and the resources sectors and it is important for any trade minister to be on the front foot and engage with international markets and the like and we've got some key markets to engage with over the course of the next little while. The first is New Zealand, as you've said, Ms O'Connor, and I am Premier as well as Trade minister. When I was previously Trade minister I was Deputy Premier, but having a premier who is also Trade minister I think will be beneficial in terms of selling Tasmania in international markets.

Ms O'CONNOR - Okay, thank you. Premier, over the 2018-19 financial year Australia issued 45 weapons export permits to the United Arab Emirates, 23 to Saudi Arabia, 14 to Sri

Lanka and four to the Democratic Republic of Congo. Militaries and militias in all of those countries have been consistently accused of human rights violations, including war crimes and crimes against humanity. As Minister for Trade, can you account for all Tasmanian arms exports or the technology that contributes towards arms? Can you claim with confidence that none of our arms exports, which is an export market you're actively trying to grow, either go directly or indirectly to militaries and militias linked with war crimes and human rights violations or end up in supply chains that do?

Mr ROCKLIFF - Just for clarification, Ms O'Connor, when you say, 'arms' you mean with respect to -

Ms O'CONNOR - The advanced manufacturing technologies under the defence section of the Trade portfolio.

Mr ROCKLIFF - First can I say they are military exports regulated by the Australian Government -

Ms O'CONNOR - But you'd take an interest as Trade minister in making sure we weren't giving weaponry to human rights abusers and militias?

Mr ROCKLIFF - I'm advised we don't have any arms exports. I think we had this conversation a few years ago when I was minister for defence industries and a lot of the technology we're utilising with respect to our innovation and in our defence industry space is really about protecting people.

Ms O'CONNOR - But if you're giving arms to Saudi Arabia, for example; or if you're providing equipment that goes towards arming Saudi Arabia in its war in Yemen, isn't that a concern to Tasmania?

Mr ROCKLIFF - Are you talking about Australian Government, which regulates matters?

Ms O'CONNOR - I would have thought that you, as Tasmania's Minister for Trade, might take an interest in making sure that materials that are produced here by our people, are not going towards genocide or activities or human rights abuses in other countries like Yemen.

Mr ROCKLIFF - I am advised that, in fact, we don't have arms exports. Of course, when it comes to the Minister for Advanced Manufacturing and Defence Industries - and you have also have spoken to Rear Admiral Steven Gilmour, before he retired - they may well be able to explore these matters. But my focus on trade is the key markets that we mentioned on Budget day - New Zealand; Indonesia; Singapore; Japan; and others - with our wonderful produce and the unique offerings we have in our premium product space.

Ms O'CONNOR - That is marvelous; but my question related to supply chain checks on components of weapons that may be produced in Tasmania and are sent to governments whose values don't align with Australia's, actually.

PUBLIC

Mr ROCKLIFF - Of course, you'll put this question to our Minister for Advanced Manufacturing and Defence Industries. As I've said, I'm advised we don't have any arms exports in terms of the niche areas of advanced manufacturing. Looking around, I can confirm that is the case. I'm not aware of any. I stand to be corrected.

Dr BROAD - How much do the trade advocates cost the Tasmanian Budget?

Mr ROCKLIFF - There is three trade advocates, well, three trade advocates and the person based. We have, comes to, 12 months contract. For example, we have a trade advocate in Japan, who is contracted for 12 months at \$125 000. There was a six-month extension on that - that was a \$62 500. I want to make sure I'm detailing this correctly; Ms Hendricks, can you assist here?

Dr BROAD - So the advocates are paid as contractors, short term contracts?

Mr ROCKLIFF - Perhaps if I get Ms Hendricks to explain.

Ms HENDRICKS - Our advocates are all on different contracts. It depends on the size and scale of market and what we assessed as the volume of need. We have our chap in Singapore, who is Udi [TBC], who is on a 12-month contract at \$145 000, they've come up to their 12-month contract. We've got an option to roll for six months, which we've activated. Jo Gayton [TBC] who is our Japanese advocate, is on \$125 000. Emily Minson [TBC], is on \$180 000 in the US. As well, in the original first 12 months, we activated Chris Oldfield for 12 months at \$30 000 to support Emily in that significant market in the US. Vivien is actually paid out of the Office of the Coordinator-General. Our advocates are 85 per cent trade and 15 per cent investment. Vivien's the opposite - she was strongly in investment, and is now in the trade space. I believe, but I would have to take that on notice, that her contract is about \$235 000.

Dr BROAD - Do they receive bonuses? How is their performance measured?

Ms HENDRICKS - Their contracts are not dissimilar to any contract; they have KPIs that they need to meet, milestones over the period of the year and the amount of engagement that they need to undertake - whether that is leads or presented at expositions. While we haven't been able to be there in the last couple of years, they have been really proactive. They do not get any kind of bonus for any kind of additional activity. It is a straight contract arrangement.

Dr BROAD - What is the expected return on investment for the trade and investment mission plan?

Mr ROCKLIFF - In terms of the trade advocates?

Dr BROAD - No, in terms of your -

Mr ROCKLIFF - The \$1.8 million, you mean? More broadly, Dr Broad, trade missions support safeguarding Tasmania against external trade and market shocks by targeting key markets that deliver tangible economic outcomes for job creation, investment and innovation for the state. They open doors, engage in market, showcase capabilities and

connect with international buyers, distributors and trading partners by leveraging off Government endorsement and, of course, our Tasmanian premium brand. There are many global borders now open. Countries worldwide are actively and aggressively seeking to reclaim and grow their market share through in-person trade missions, delegations and business events. To respond we must be swift, we must be proactive to compete in this trading environment.

The Tasmanian Government is developing a two-year trade mission plan, encompassing key emerging and growing markets and new opportunities through free trade agreements. I have some information here - since 2015, for example, missions have been facilitated to China, South Asia, Europe, United Kingdom, Korea and Japan.

Ms O'CONNOR - Not to Taiwan yet?

Mr ROCKLIFF - One hundred and thirty-two organisations have participated on trade and investment missions across the four years. Of the participants surveyed across the missions, 100 per cent said they would participate in a mission again; 86 per cent achieved their objective; 10 per cent saw immediate commercial outcomes; 10 per cent expected commercial outcomes within the next three months; 60 per cent expected commercial outcomes within the next 12 months; and 10 per cent expected commercial outcomes within the next five years; and 80 per cent expected to achieve new agreements with distributors, buyers and partners. Increasing our exports has significant flow-on benefits for the state and, according to Austrade data, exporting businesses compared with non-exporting businesses employ 23.8 per cent more staff, enjoy 13.4 per cent greater labour productivity, offer 11.5 per cent higher average wages and invest some 7.6 per cent more in capital expenditure. That probably goes to your question in terms of return on investment.

Our Budget is \$1.765 million over two years, to fund the Investment Mission plan. The plan will be to enable transparency on market focus, support forward planning to coordinate industry and Government engagement, maximising the outcomes of these international visitation programs. They will be informed by insights from our Tasmanian Trade Advocates Network and our international stakeholders' ongoing industry and Commonwealth engagement, and existing and independently commissioned market research.

Dr BROAD - You talked about it being responsive and on the spot. When rock lobsters were blocked from the Chinese market, how did the members of the trade portfolio react, or is that left to the federal government to sort out?

Mr ROCKLIFF - As I recall, it was a combination of both federal and state government working together.

Ms O'CONNOR - And it wasn't our fault, either.

Mr ROCKLIFF - No. While China's new testing regime on rock lobsters has had a large impact on Tasmania's rock lobster industry, the state has seen other alternative new markets open, including Hong Kong, Vietnam and Singapore.

Ms O'CONNOR - That is why you diversify.

Mr ROCKLIFF - Due to the ongoing trade restrictions with China regarding rock lobster, the Tasmanian Government has facilitated the coordination of a rock lobster cluster to support the sector in reducing single-market dependency and proactively investing in diversification activities such as building brand awareness in markets and developing new value-added products.

The Tasmanian rock lobster export processor cluster comprises of Salco Live Lobster, JSJ Seafood, Red Rock Lobster and the Seafood Gateway, which represents some 90 per cent of the export volume and value across the Tasmanian rock lobster sector. I am not sure if Ms Hendriks wants to add anything further to that, but it was a key area of concern. I remember being in King Island in very early January 2020 and meeting with a rock lobster exporter and I was talking about alternative markets to Sydney and the like, but of course that was China's response and then the pandemic hit not too long after that, so I know it has been some time, but perhaps Ms Hendricks would like to respond.

Ms HENDRIKS - I can quickly add to that. It has been a fabulous cluster arrangement between the industry. The industry has really stepped up with the support of the Tasmanian Government. One of the foundations of the mission plan is to support the diversification of products like lobster and in September and October this year at Food Hotel Asia there will be an official launch of that product into the Singaporean market and we will also be introducing it into Vietnam and Malaysia.

Ms O'CONNOR - Premier, action 9 of the Tasmanian Trade Strategy Annual Action Plan of 2021 reads:

Leverage Tasmania's strengths through the Asian market engagement plan, marketing campaigns and trade advocates to promote Tasmania as an international education destination of choice.

How do you reconcile this with the fact that UTAS's current approach is to wind back its reliance on international students, particularly from mainland China? It looks very much like the government strategy is at complete odds with that of UTAS.

Mr ROCKLIFF - We've actually got a very proud history of attracting international students and supporting them as well. International students help meet demand within the employment market and fill current and future skill shortages. They have a positive impact on Tasmania economically, socially, culturally and of course these have flow-on benefits for tourism, business linkages, jobs, and population growth.

In 2019-20 the Department of Education, Skills, and Employment estimated that international students contributed some \$673 million to the Tasmanian economy. Study Tasmania within the Department of State Growth implements the strategy for the sector and key achievements in the last 12 months include hosting and participating in a range of virtual events to continue to maintain and build Tasmania's profile in key markets.

Ms O'CONNOR - I just wanted your thoughts on the fact that it is in contradiction to UTAS's stated policy that it wants to rely less on the international student market.

PUBLIC

Mr ROCKLIFF - I will allow UTAS to talk for themselves. What we are doing is hosting familiarisation trips for education agents to showcase Tasmania as a study destination and connecting students and communities through the enhanced student experience grants program, which provides grants of up to \$5000 to deliver activities that enhance student experience and help connect students with the Tasmanian community. Various initiatives which aim to support student employability include the launching career coaching modules in collaboration with the Migrant Resources Centre Tasmania, incorporating elements on how to network, write resumes and prepare for interviews, and launching an online jobs portal to connect international students. It also supports schools, vocational education and training and TasTAFE.

When it comes to education providers, including the University of Tasmania, TasTAFE and government independent schools also have accommodation options for international students. I am advised that Study Tasmania within the Department of State Growth has engaged international education consultancy Edified to undertake a survey of international students studying within Tasmanian institutions to be better understand their needs, particularly as it relates to supporting students trying to find accommodation. This work will be completed by September 2022 and will help inform any future approaches in this area.

Ms O'CONNOR - Are you going to have a chat with UTAS about their different strategy?

Mr ROCKLIFF - I will talk to the University of Tasmania and ensure we discuss that.

Ms FINLAY - Going back to the rock lobster cluster, given the size of the market when it went into China and now framing that as a success, I am wondering how much of the market has recovered and the work being done in this cluster, or over what period of time you imaging you're able to go back to 100 per cent?

Ms HENDRIKS - Obviously the launch is only happening in September, so it has taken this long to get the frozen product for market. We hope that through Singapore, Malaysia and Vietnam that we'd be able to recover. It's also worth noting that New Zealand repositioned themselves to put the lobster into China when we had the bans and we were actually exporting our lobster into New Zealand because they had a shortage in their domestic market.

Mr ROCKLIFF - Ms White asked me about AFL games. There has been no decision made on the split of games between the north and south of the state, with our primary focus on securing the AFL and AFLW licences. We know how important league football is for the north of the state and I would not envisage the situation where there would be any less AFL games than are currently played there. It would be our ambition, particularly with a women's team also, to ensure more AFLW content is played across both ends of the state. I would appreciate Ms White working together with me to do what we can to set about achieving what we have wanted for the past 30 years and that is to secure a team of our own once and for all.

The committee suspended from 1.13 p.m. to 2.00 p.m.

HEALTH

PUBLIC

CHAIR - I invite the Premier to introduce persons at the table, names and positions, for the benefit of Hansard.

Mr ROCKLIFF - Thank you, Chair. To my right at the table is Kathrine Morgan-Wicks, secretary of Health and State Health Commander. To Kath's right is Dale Webster, deputy secretary of Community Mental Health and Wellbeing, Commander Health Emergency Coordination Centre. To my left is Tony Lawler, deputy secretary, Clinical Quality, Regulation and Accreditation, and Chief Medical Officer.

It is a privilege to sit here for my second Estimates hearing as Minister for Health. Health is consistently a top priority for Tasmanians. That is why I retained the Health portfolio when I became Premier.

Our budget, in terms of health expenditure, will increase to \$11.2 billion and now accounts for 33.6 per cent of the Budget's total operating expenditure, compared to 28 per cent in 2013-2014. We will spend an average of \$7.3 million every day on delivering health services to the Tasmanian community.

As you are aware, Health is an exceptionally complex portfolio. I want to start by openly acknowledging a couple of the major challenges we currently have before us.

We are still very much managing the transition out of the COVID-19 pandemic and considerable resource and effort is still aimed at achieving this in the safest, most balanced way possible. I think you would agree if you take a moment to look internationally and even domestically, Tasmania has been very fortunate in terms of how our health system has continued to manage pandemic-related challenges. This is largely due to the high number of Tasmanians being vaccinated and, of course, the hard work of health staff, the diligence of our broader community and investment we have made towards the COVID-19 response. Nevertheless, the challenge remains. We have the added challenge of this year's flu season but we continue to work through it and take advice on how best to respond.

As we look to the future of our health system, we are looking at a situation not uncommon for developed jurisdictions where we have an ageing population, increasing levels of chronic disease and increasing demand for services.

Despite these factors, I am very proud of the fact that our Government hasn't shied away from making tough decisions in order to prioritise investment in Tasmania's public health system. Our significant investment in the 2022-2023 Budget reflects our focus on health. We have a vision for a contemporary agile health system and we are appropriately funding initiatives that will facilitate this.

You would have heard of the announcement of \$150 million over the next four years to commence our improving patient care digital strategy, which is a 10-year project that will transform the delivery of health services in Tasmania. This investment will enable public and private healthcare providers across the state to securely view patient records and access up-to-date clinical information. This will improve the quality and timeliness of patient care, create an increase in early interventions and improve patient flow. More importantly, it will deliver better access and equity of access to health care across the state.

Another significant change within the hospital system will be the introduction of new inter-hospital transfer policy to ease the pressure on our emergency departments, as foreshadowed today. Patients transferred between health facilities under this process will be authorised by a practitioner for direct transfer to a destination ward, bypassing the ED when it is clinically appropriate to do so. The policy will help address broad access and flow challenges in our hospitals. It will support an efficient patient handover process between ambulance and hospital staff to enable our ambulances to get back out on the road quicker, ready to respond to other cases. It will also help reduce pressures on our emergency departments.

We are investing in a number of other critical service and infrastructure projects in this year's Budget as well. We know that Ambulance Tasmania continues to experience increasing demand. The Government has committed to ongoing funding for the 48 new paramedics recruited over the last 12 months. Not only are we maintaining this increased workload but we are also providing for an additional 11 paramedics to upgrade the Huonville and Sorell double branch stations to career stations. Funding for new vehicles and equipment required to mobilise this increase in the workforce is also included.

You would have heard me speak about the PACER initiative, the emergency mental health co-response model. Given its immediate success in treating people in the community and avoiding unnecessary ED presentations, we have committed to additional ongoing funding to continue the model in the south. And I am pleased to confirm that a pilot will begin in the north west in early 2023 as the next phase in evolving PACER to a statewide model.

The 2022-23 Budget also includes funding to ensure that our major hospitals can manage adequate resourcing to meet demand, particularly during the COVID-19 pandemic. This will ensure that beds that were opened early to prepare for the state border changes in December 2021 remain open. Continuing recruitment is underway to ensure these beds are properly resourced and supported. Further, our successful private partnership model to purchase beds and services from private hospitals around Tasmania will continue through 2023.

The outpatient transformation program will receive funding of \$1.8 million per year to permanently establish a clinical support team dedicated to service enhancement, including implementing a statewide outpatient admin hub and digital portal to improve the outpatient experience.

A significant capital works program has also been supported. Funding has been allocated to the LGH redevelopment to complete stage 1 projects and continue stage 2 projects to complete a purpose-built ante-natal clinic in the North West Regional Hospital. The Budget includes funding towards stage 2 of the Royal Hobart Hospital redevelopment.

These are just some of the key investments we are making toward our vision of a modern healthcare system that meets the changing needs and expectations of the Tasmanian community. I trust members at the table have had time to read through the relevant Budget chapter and acknowledge the additional commitments which time will not permit me to mention.

PUBLIC

Every Tasmanian has a story about a time that they or a loved one needed treatment and care from our public health system. I have complete faith in our dedicated, capable workforce to shape and maintain the services we all rely on. I look forward to continuing to work with colleagues, the community and the Department of Health to ensure Tasmanians can access the right care in the right place at the right time.

Ms DOW - Thank you, Chair. As of today, if you are wanting to access inpatient mental health services and you live in the north and north west of the state, you would be sent to Hobart. Why wasn't this significant change announced publicly to inform people of arrangements that are now in place and options available for care? How will people be transported? And when can we expect services to be resumed in the north and north west?

Mr ROCKLIFF - Thank you. I believe Ms Morgan-Wicks commented publicly and certainly communicated to staff as well in terms of the challenges.

Ms DOW - There was a memo to staff but nothing conveyed publicly, to my understanding.

Mr ROCKLIFF - I will pass on to Ms Morgan-Wicks to explain that.

Ms MORGAN-WICKS - In terms of the management of outbreaks as they are currently occurring in different ward environments in our hospital and health services, we are communicating with our staff to announce if an outbreak has occurred the utilisation of an outbreak management team in terms of the management of that outbreak. We are also communicating with patients within the ward or discharged patients if they are within a time period of concern in relation to the transmission of COVID-19. So, we've certainly made communications to staff and patients who are potentially impacted. I also publicly commented on that earlier today.

Ms DOW - Are there enough beds in the south to accommodate patients from the north and north west?

Mr ROCKLIFF - Thank you, good question.

Ms MORGAN-WICKS - It is very important that the public understand that if they are suffering from a mental health condition that they should feel free to attend both the North West Regional Hospital and the Launceston General Hospital so that they can be assessed and triaged. It might be that we can provide them with treatment on the spot, or treatment that keeps them within their home through dedicated support for mental health in the home purposes.

If they do require admission, we will be admitting them to the Royal Hobart Hospital. I am informed by my team that we are able to cope in terms of the transfers through to the Royal at the moment.

In relation to the Northside Clinic itself, my understanding is that it is likely the outbreak will be concluded tomorrow, if there are no further positive cases detected. The Spencer Unit will be a few more days, given that the outbreak has only been recently detected.

Ms DOW - Premier, you have budgeted 2.5 per cent for public sector wages, yet the wages comparison being undertaken by Edge Legal to determine how far behind Tasmanian healthcare workers are and what wage increase is required to base salaries is not yet complete. How will this be funded if a significant, for example, 20 per cent, base salary increase is recommended? And when will that report be made available, and will you make it public?

Mr ROCKLIFF - The Government is committed, as I said this morning, in response to a number of questions, to negotiating with all unions on wage agreements in good faith. We won't pre-empt any outcomes but we do recognise the increased cost-of-living pressures on Tasmanian households and we are committed to delivering a fair and reasonable, and affordable pay rise for our hard-working State Service employees.

We are also committed to delivering a sustainable Budget and we cannot agree to a wage increase that we cannot afford, or cannot offset with savings.

There are 16 wage agreements, as I referred to this morning, due for negotiation this year. This includes the PSUWA agreement which covers all employees by the Tasmanian State Service Award and the Health and Human Services Award. Each agreement is negotiated for a specific group of employees to deliver fair and reasonable terms and conditions, including affordable wage increases. The 2.5 per cent level of wage indexation is not an allocation for the purposes of developing the Budget. It is not a wages policy.

We are willing to work with unions. I recognise the cost-of-living pressures, as I say, and we will sit down with the various industrial advocates and unions in a fair and reasonable way. We have been working on the health recruitment taskforce as well. Six meetings of the health recruitment taskforce were held last year and the taskforce formed recommendations to health executive, which included a draft employment direction exclusive to health, a review of national base salaries and amending the timeframe for a review of selection decisions to improve time to fill. There was also discussion in relation to nursing and midwifery projects and recruitment time-frame benchmarks. As for specific wards, you are talking about that benchmark of a level 3 escalation at 30 days and asking what wards -

Ms DOW - What regions of the state?

Mr ROCKLIFF - What regions?

Ms DOW - What hospitals, what wards and what units?

Mr ROCKLIFF - That would have reached that benchmark of the escalation level 3, 30 days. Dale, thank you.

Mr WEBSTER - The draft agreement, because it is not yet signed, commences on 1 April, all escalations after 1 April 2022. The Launceston General Hospital was at escalation on 1 April and ran through until about the third week of May, so the escalation payment will be paid for a number of weeks to staff at the LGH. I will come back to the wards and those sorts of things. The Royal Hobart Hospital was also at escalation level 3 throughout that period, so again the payment will apply to the Royal Hobart Hospital. With the Royal, it will

also apply to some ancillaries because the declaration of level 3 was not just the main campus. The third area that was declared at level 3 was the community sector parts of statewide and mental health services, which include the correctional primary health service and the Wilfred Lopes Centre. That was declared in mid-April and was removed last week so, again, there will be a period of time where the COVID-19 payment is paid there.

In terms of the wards, there is a scale of three levels of payment: \$20, \$30 and \$60 per shift. The \$60 is paid for direct wards where COVID-19 patients are present. That would include the ED, the ICU and the specific COVID-19 wards we have in each of those hospitals. The \$30 amount is in areas where there are no direct COVID-19 patients but they support the COVID-19 effort. The \$20 is the support areas, such as administration, those sorts of things within that particular escalation area.

Ms DOW - Chair, I have just got one more on that, it just feeds into that. Premier, will any nurses or midwives in the north west receive the COVID-19 allowance after their significant efforts during the 2020 COVID-19 outbreak?

Mr ROCKLIFF - You mean, back in 2020?

Ms DOW - That is right.

Mr ROCKLIFF - Backdated? I am advised it is not backdated. It is dated to 1 April this year, once the agreement is signed.

Ms DOW - Why is that?

CHAIR - Dr Woodruff.

Dr WOODRUFF - Thank you. Minister, this time last year there were zero cases of COVID-19 in Tasmania and 13 people had died of COVID disease. Since the borders reopened, there has been 167 774 people infected with Omicron, 66 extra people have died and Tasmania continues to have one of the highest reported rates of infection in the world.

COVID-19 is the highest cause of death in Australia now and we are learning more every day about the disabling and sometimes deadly effects of long COVID. Do you think your response since reopening the borders has done everything possible to keep Tasmanians safe?

Mr ROCKLIFF - We prepared as well as we possibly could, in terms of hospital preparedness. Of course, we based our modelling on the Delta variant. Our opening on 15 December 2021 did present challenges when it comes to the Omicron variant and other variants such as BA.2. You mentioned the number of people who have had COVID-19. I would also point to our relatively low hospitalisations compared to other jurisdictions, as well as relatively low death rates compared to other jurisdictions. As I said this morning, I am sure for the family and friends and loved ones of people who have passed away with or of COVID-19, that provides no comfort. Except I would point to the fact that we have continued to manage as best as possible the pandemic in terms of our measured easing of restrictions - you'll probably recall I named them protections, as Ms O'Conner does - but it has been done in a methodical measured way with the safety of Tasmanians in mind, first and

foremost. We are guided by the best of Public Health advice and these discussions also happened at a national level through AHPPC and we will continue to be guided by Public Health advice.

Dr WOODRUFF - Why have you continued to focus on the short-term deadly effects of the SARS COV-2 virus and refused to include the potential serious risk of long-term complications from COVID infections in the Government's response and the aims for managing coronavirus in Tasmania?

Mr ROCKLIFF - Are you talking about long COVID?

Dr WOODRUFF - I'm talking about long-term complications, long COVID; post-viral illness is a post-viral complication. The long-term complications of COVID are many and we'll get to that in a minute, but your Government's response is focused only on hospitalisations, which is the acute, immediate risk of death or serious illness from COVID. Why have you not focused on long-term complications?

Mr ROCKLIFF - We have been focusing on the immediate needs of Tasmanians and ensuring that we have the health system and services as prepared as possible. That includes not only increasing our capacity in terms of hospital beds and indeed staff and our focus on vaccination, which we've consistently done very well with, but also investing in areas such as COVID@home, which has ensured we've been able to support people in their home and therefore easing the pressures off our hospital system. I am advised that has worked very well to date and will of course continue.

Dr WOODRUFF - The majority of people in Tasmania would be walking around under the misbelief that getting vaccinated against COVID is all they need to do to keep themselves safe. That is because your Government continues to reinforce that vaccines are the best protection against COVID infection. Do you agree that that approach deliberately ignores the reality of the waning effectiveness of vaccines and also of long-term COVID complications from being infected with the virus?

Mr ROCKLIFF - When it comes to people who are hospitalised with a COVID-19 diagnosis in Tasmania, I am advised that those unvaccinated are more likely to die than those who had received two or more doses of a COVID-19 vaccine. That is why getting vaccinated and keeping up to date with a booster continues to be the best defence against COVID-19. The vaccination is available at over 100 pharmacies around Tasmania, 100-plus GPs and state-run clinics across Tasmania as well. I want to reiterate that we are removing restrictions safely, sensibly and in line with national and Tasmanian Public Health advice. While I understand the lifting of restrictions may be concerning for some, it is another important step in our ongoing transition to live with COVID as we continue with our COVID-safe behaviours.

Dr WOODRUFF - What you just said, that vaccines continue to be our best defence against COVID, is exactly my point. When you say that, what you're talking about is the risk of serious illness and hospitalisation. You are not talking about the risk of infection from the virus itself and therefore you are not talking about the risk to people of potentially serious long-term disabling and deadly COVID infection outcomes. Are you ignoring long COVID and long-term complications?

Mr ROCKLIFF - No, I'm not ignoring it at all. The understanding of long COVID internationally of course is still evolving. It is a syndrome that may affect a variety of body systems to varying degrees and symptoms of long COVID are highly variable from individual to individual. As you would appreciate, the dominant strain of COVID-19 in Tasmania is the Omicron variant and the current view is that the presence of long COVID associated with Omicron is less severe, but this is not to say -

Dr WOODRUFF - You're talking about long COVID, not about long-term complications of SARS COV-2 infection. There are many of them and they're potentially very serious and can cause death, disabling, dementia, diabetes, cardiovascular disease, lung disorders, a whole range of things.

Mr ROCKLIFF - Our understanding of long COVID is that it is still evolving, Dr Woodruff.

Mr O'BYRNE - Premier, you obviously have a bit of a challenge with your workforce - nurses are currently taking industrial action and all indications are it is not going to be resolved in the short term. You referred to the health recruitment taskforce earlier in your question from the member for Braddon. In last year's budget you had a line item called Health Recruitment Taskforce totalling \$15.7 million; it was \$6.7 million in 2021-22, \$2 million each in 2022-23 and 2023-24 and \$5 million in 2024-25. Looking through the Budget this year, there is no item that I can see. There is a couple of matters where you might refer to staffing ambulance workers but there is no line item about the Health Recruitment Taskforce, so have you defunded it? What has happened to it?

Mr ROCKLIFF - I am advised that it has not been defunded. It is a key deliverable of last year's budget, Mr O'Byrne, and remains underway.

Mrs ALEXANDER - Premier, as the Minister for Health, I would like to continue with that discussion about long COVID. I understand that people infected with long COVID can experience some long-term, prolonged symptoms. Could you please update the committee on what is being done currently for providing treatment for Tasmanians who may be affected by long COVID?

Mr ROCKLIFF - A significant part of our Health expenditure over the past two years has been our ongoing response to COVID-19 and the pandemic. Today I am pleased to announce that \$400 000 has been invested to establish a new statewide navigation and referral service specialising in long COVID. Evidence suggests that for the vast majority of Tasmanians who have had the Omicron variant, long COVID does not become an issue, but our knowledge of this issue continues to evolve, as I have said before. However, for those who are impacted, it is important to know that treatment for those in need will be available.

Long COVID is a syndrome that may affect a variety of body systems to varying degrees and treatment is largely supportive care. The practitioner most suitable and central to managing such syndromes is a GP, and it is still expected that most patients with long COVID will be wholly managed by their GP. This is why our most important first step was the establishment of a primary care pathway across general practice and the public hospitals. A new service has been established in response to feedback from the community and our

medical colleagues. This new service complements the existing pathway by providing a single point of referral for GPs across the state, and all patients will be managed by their GP in the first instance.

This is important for two reasons. It is critical that other illnesses are not missed. Secondly GPs can determine if their patients require referral to the new service where input may be sourced from specialists in the field including infectious diseases, respiratory medicine, psychology and neurology. Getting the health response to match the individual needs of the person is critical to managing long COVID.

The service will be available statewide and will be fully developed and launched in September this year. The timing will allow us to recruit clinicians and educate GPs on how to engage with the service and more details will become available closer to the date of announcement. Finally, we also know that fully vaccinated people suffer less from the effects of COVID-19 both in the short and the longer term, so we continue to encourage Tasmanians stay up to date with their vaccinations.

Ms DOW - How will you retain and recruit health staff and our healthcare workforce across Tasmania, namely nurses and midwives, when there is nothing in the Budget aimed at improving recruitment and retention across the healthcare workforce in Tasmania?

Mr ROCKLIFF - The outcome of the taskforce was a change of delegation for improving market allowance. I also refer to our Health Workforce 2040 that was released last year, you might recall. It is a long-term strategy aiming to shape our health workforce that meets the needs of Tasmanians now and into the future, look after those who dedicate their to looking after others and provide opportunities to support our health professionals to follow their career ambitions.

Health Workforce 2040 incorporates input from consumers and health professionals and includes the most up-to-date datasets available at the time of release. Implementation of the strategy's action items has commenced. There has been funding \$15.7 million over four years that was provided in the 2021-22 Tasmanian Budget to support the implementation of Health Workforce 2040. This funding will support workforce development, upskilling, more efficient recruitment, leadership and management training, capacity building and the development of new and innovative health workforce models to better meet the community needs of the future.

Funding has been provided to the responsible business units across the Department of Health for specific projects that will progress actions outlined in the strategy. The health workforce planning unit will report annually on the implementation of the strategy. The first annual report will be prepared for the training education and workforce subcommittee of the department's clinical executive in September this year.

Ms DOW - Other states are offering up to \$13 000 to nurses and midwives to take up a contract of employment in their state. The Budget you have presented to Tasmanians has no funding like this included in it. Do you plan to offer any incentive payments and do you think there would be benefit in that if other states are looking to do so? Are you concerned that our healthcare workers will be going to others states to work rather than stay in Tasmania and it will be an impediment to recruitment in our state?

Mr ROCKLIFF - I am mindful of what other states are doing and I believe there was an announcement today in New South Wales, if my memory serves me correctly, of a \$3000 upfront payment. We are always mindful of how we can recruit and retain our very valuable workforce and we are ready, willing and able to negotiate in good faith with our workforce.

Ms DOW - But you're not providing any incentives to attract more healthcare workers to Tasmania.

Mr ROCKLIFF - We are providing health workforce initiatives which were funded in 2021-22 budget and I have mentioned a number of those. They include establishing and administering a nursing and midwifery training and development fund with three streams - a scholarship program, refresher program and leadership program and the office of chief nurse and midwife is responsible for that; a statewide rostering and resource innovation unit to reduce administrative burden from nursing duties, including a nursing director, administration support, data analyst and project officer together with regional recruitment support officers; developing a culture of health leadership, which I am very keen on and have announced and spoken about previously; establishing a health leadership program for existing and emerging clinical leaders across the state and across professions; and establishing and managing the future health leaders forum.

Ms DOW - Premier, what's the total number of full-time equivalent and head count of nurses and midwives employed across Health in the last two years, please?

Mr ROCKLIFF - Did you say nurses and midwives?

Ms DOW - Yes, excluding casuals.

Mr ROCKLIFF - We can provide you with information of paid FTEs, Ms Dow, as I understand it, so I am happy to see if we can get that information for you. When it comes to nursing recruitment, we continue to increase the number of nurses and midwives employed across the state. From 30 June 2018 to 31 March 2022 there has been an increase of approximately 640 FTE nursing staff across the department, which is about 15.9 per cent of the total nursing workforce. Over the last nine months nursing staff has increased by 206 FTE and that is largely due to the ongoing support required as part of the department's COVID-19 response and increasing demand for services, particularly at the Launceston General Hospital and Royal Hobart Hospital.

The recruitment of nurses on a permanent basis, particularly at the grades 3 and 4 level, continues to be a key workforce strategy and priority for the department in minimising fixed-term employment. The department actively supports and progresses applications for a promotion without advertising for the current staff who have qualified as registered nurses.

I've got some total nursing FTE figures for you. In 2018-19, there were 4025.51 FTE. This includes nursing FTE outside hospital groups, for example Ambulance and Public Health, ECC and TasVax as well. In 2019-20, there were 4229.08 FTE, and in 2020-21, there were 4460.47 FTE. As at 31 March this year I am advised there are 4666.5 FTE. I can break that down by hospital if you'd like as well.

Ms DOW - That would be good.

Mr ROCKLIFF - As of 31 March 2021-22, the North West Regional Hospital had 346.24 FTE, the previous year was 333.02 FTE; the Launceston General Hospital had 1128.02 FTE, the previous year was 1095.66 FTE; the Royal Hobart Hospital had 1697.71 FTE, the previous year was 1602.49 FTE; the Mersey Hospital had 219.11 FTE, the previous year was 206.34 FTE. If I go to the total FTE of all the hospitals and start at 2018-19, we had 2926.83 FTE; in 2019-20 it was 3058.3 FTE; in 2020-21 it was 3237.51 FTE; and as of 31 March 2022, 3 391.08 FTE, I am advised.

Dr WOODRUFF - Minister, this might be a question for the Director of Public Health. Vaccines provide a form of protection against serious illness and death and, to a very small degree, against infection. What do you estimate the current population level is of vaccine effectiveness against severe illness and COVID-19 infection, separately for each of the vaccine age groups? We have the nought-to-four, who do not get a vaccine, we have five to 11, 12 to 16, 16 to 50, over 50. What is the estimated effectiveness of vaccine coverage for those age groups?

Mr ROCKLIFF - COVID-19 vaccines?

Dr WOODRUFF - COVID-19 vaccines.

Mr ROCKLIFF - Dr Veitch, if I could invite you to the table, as indicated by Dr Woodruff. Mark Veitch is the Director of Public Health. Dr Veitch, did you hear the question?

Dr VEITCH - Yes, I did. It is going to be difficult to calculate all of those figures in Tasmania simply because the number of severe outcomes such as hospitalisation and deaths becomes very small once you get into the younger age groups in our population. We can look at the vaccine efficacy from having had two or more doses of vaccine versus having had no vaccine. If we look at the data, you will see the portion of cases who have had no doses of vaccine who have died is about three times that of the people who have had two or more doses of the vaccine. That calculates out to a vaccine efficacy against the outcome of death across the whole population of around 90 per cent, which is consistent with the findings in other jurisdictions worldwide. That does not break it down by age or particular vaccine but it is encouraging evidence that supports the information elsewhere about the protective effect of vaccine against severe outcomes.

Dr WOODRUFF - What I am trying to understand is, over the winter period, what our dropping vaccine efficacy will be across different population groupings. Maybe you don't have enough numbers, as you say, to provide that information by those age groups. But what is the department's understanding about where different populations are up to in their vaccine take-up? There has been a number of months since people have had booster shots, some many months depending on where people were when ATAGI approved boosters to be available for different groups. We are having waning efficacy of vaccines. Where is your estimate that we are up to in terms of the effectiveness of vaccines? And where do you think we will be by the end of winter?

Dr VEITCH - The honest answer is that I don't think I know and I don't think that is currently knowable. I think it is best not to speculate. What I would observe is that, along with having had vaccine provided to a large proportion of the population, probably half or more has experienced COVID-19 infection in the last three or four months. We have a situation with a high level of vaccination boosting, the winter doses to the most vulnerable and also recent experience with actually having had, for most people, fortunately, a modified form of COVID-19 illness as a result of being vaccinated. There is a large number of things there contributing to population immunity.

The other thing I would note is that, at the moment, nationally and in Tasmania, we are seeing a drop-off in cases down back towards under 100 per 100 000 per day in most jurisdictions. That, I think, probably reflects some modest level of herd immunity that is making transmission a little bit less prominent at the moment.

We have to be wary of the possibility of another wave of infection later in the year, as you observe. But I think we do have good surveillance mechanisms in place, both for COVID-19 generally and for variants of COVID-19, that will give us a warning of variants of public health significance.

Dr WOODRUFF - Could you tell me, by each vaccination group, what proportion of Tasmanians are not fully vaccinated at the moment according to the ATAGI definition of full protection?

Dr VEITCH - We will need to find -

Mr ROCKLIFF - Age group?

Dr WOODRUFF - Yes. Zero to four, no one is protected because there is no vaccine. Five to 11, 12 to 16, 17 to 50 and over 50.

Mr ROCKLIFF - I've got the clinical severity and deaths reported. COVID-19 cases by age group in terms of - that is a surveillance report though. Vaccination coverage.

Dr WOODRUFF - I'm asking for the 'not covered' proportion.

Mr ROCKLIFF - Right. I have some percentages, so we can probably reverse the percentages, if that makes any sense. The five to 11 age group, population of which is 45 033, 63.99 per cent have had their first dose and 22 599 people have had their second dose, which is 50.18 per cent of the population. This is 1 May again.

Dr WOODRUFF - That is 48.2 per cent who are not fully protected?

Mr ROCKLIFF - Yes. For 12- to 15-year-olds, total population of 26 300, we have 87.18 per cent having their first dose and 83.04 per cent with their second dose. In the 16 and above -

Dr WOODRUFF - That is 17 per cent not fully protected.

PUBLIC

Mr ROCKLIFF - Yes, 16.96 per cent. In the 16 and above, the population is 444 277, percentage of vaccination is above 99 per cent and the second dose percentage is 98.84 per cent.

Dr WOODRUFF - And the booster?

Mr ROCKLIFF - I have 306 508 Tasmanians 16 and above, as at 1 May, with a booster, which is 69.63 per cent.

Dr WOODRUFF - So, that is 31 per cent in that age group not fully protected. And over 50?

Mr ROCKLIFF - If we go to today's figures, boosters for the 50 and above age group, 85.9 per cent.

Dr WOODRUFF - And the number of people, what is the actual absolute number of people in the 50 over?

Mr ROCKLIFF - I haven't got that figure but if we have got 71.36 per cent of what would be 44 000, which I've mentioned, this population figure here. But that might have changed since 1 May as well. But we can provide that number if you would like us to.

Dr WOODRUFF - Thank you, I will put that on notice.

Mr O'BYRNE - Another question on recruitment and retention. You have people unhappy with the outcome of the COVID allowance and negotiation dragged on, and effectively you starved people into submission. You have the AMNF taking industrial action with other unions potentially following suite. You have some of the lowest paid nurses in the country. You have a wage offer on the books, effectively out of this Budget, which is a real cut in wages. You have recently lost a case on appeal to the Supreme Court around an allowance matter that the AMNF ran up. I do not know why you appealed that to the Supreme Court. From what we hear, morale is quite low.

All of these things added up, sends a message to the nursing and healthcare workforce that you do not value them. How can you recruit people to a service which is in such a state when your actions have led to such an outcome?

Mr ROCKLIFF - We have continued to recruit many people across our health services, as I have said before, Mr O'Byrne. We want to create a culture across our workforce where people do feel of value and that the Tasmanian Health Service is a workforce of choice. There are many ways that we can support that objective, if I can call it that.

I have spoken about the COVID allowance already this afternoon. Mr Webster has provided more detail on that. There were lengthy negotiations, I acknowledge that, but at the end of the day, they were signed off as I understand it by the AMNF.

Mr O'BYRNE - And they both said on the public record that they were not happy with the outcome on behalf of their members.

Mr ROCKLIFF - I've just been reminded that we are the only state, other than Victoria, to have the COVID-19 allowance offered. We also have 260 transition-to-practice nurses who started their careers with the Department of Health in 2021. This represents a 21 per cent increase in comparison to the previous year, with an anticipated further increase in 2022.

Permanent employment is offered to graduate nurses who successfully complete the transition-to-practice program. Specific education programs are in place, in specialty areas, to further support the retention of transition-to-practice graduates, as an example of our ability to recruit.

Mr O'BYRNE - Sorry, is that your answer?

Mr ROCKLIFF - I gave a really good answer: I talked about COVID allowance, working with our industrial advocates and unions. I reminded you of our increase in FTE since 2018 to 31 March, increase of 640 FTE; in the last nine months the nursing staff has increased by 206, the FTE as well.

I understand where your question is coming from and I have just been reminded that our nursing agreement is not open for negotiation until 2023.

Mr O'BYRNE - I am sure that makes them happier, technically.

Mr ROCKLIFF - I thought I would provide that information for you, Mr O'Byrne. I was being helpful.

Mrs ALEXANDER - Can the minister please update on the expansion of the Statewide Trauma Service at the Royal Hobart Hospital?

Mr ROCKLIFF - I am very pleased to talk about the Statewide Trauma Service. In line with the priority of Tasmanians getting the right healthcare, in the right place at the right time, we are expanding the services at the state's major trauma centre, to provide best-practice trauma care, to the Tasmanian community.

I'm pleased to announce that for the first time all major trauma patients will be admitted under a single multi-disciplinary team of specialist trauma doctors and nurses as part of a \$2.5 million funding investment. This funding will expand and enhance the trauma service at the Royal Hobart Hospital, which will benefit trauma patients across the state. Introducing a statewide trauma bed card will enable consistent management of trauma cases, regardless of where in Tasmania the trauma case occurs.

Before the expanded trauma service begins admitting patients in January next year, we will recruit two additional trauma staff specialists, three new trauma registrars, three new trauma resident medical officers, a nurse manager (trauma), a trauma clinical data coordinator, a clinical research coordinator and a trauma fellow.

Under the new model, a medical team of trauma consultants and junior doctors will lead the trauma patients care plan from their initial injury, through to rehabilitation. Specialist trauma case management nurses will ensure that every patient receives optimal care and that

the social and mental wellbeing of patients and their family is supported. We know that the recovery process for people with complex injuries does not end when they leave hospital. The expanded trauma service will offer follow-up clinics and engage directly with GPs to support patients' ongoing recovery in the community.

Importantly, this expanded trauma service will also support community-based injury prevention programs and research to identify and remove barriers to optimal trauma care throughout Tasmania. The establishment of this service is a major step forward to develop a world-class statewide trauma service and demonstrates our commitment to patients receiving the right care, in the right place at the right time.

Ms DOW - Premier, the AMNF have called for a crisis taskforce to be established in the state to address the challenges with the nursing and midwifery workforce. Will you commit to doing this?

Mr ROCKLIFF - We have sat down with the AMNF through the health recruitment taskforce work. I've met very regularly with AMNF and other key stakeholders across our health system including the AMA, HACSU, our GPs and other key stakeholders when it comes to COVID-19. I have always been willing to sit down with, and talk with, the AMNF and other stakeholders.

Ms DOW - But, will you specifically commit to that taskforce, Premier? It's at crisis point.

Mr ROCKLIFF - We already have a mechanism for consultation when it comes to these matters and it's only a matter of approaching us, about certain matters.

Ms DOW - They wrote to you in their budget submission, calling for this initiative.

Mr ROCKLIFF - As I have advised, we have met with them - I'm including the health executive, as I understand it, and with the AMNF many times. We'll continue to do so. We met weekly there through some of the height of the more recent Omicron variant, for example; weekly with stakeholders leading up to the borders opening, following fortnightly, and then three weekly.

We'll always continue to work with key stakeholders when it comes to the pandemic, but with any concerns that they may have. At one of my first meetings with the Australian Medical Association, they put on the table the absolute dire need for a digital workforce strategy. We have delivered that.

Ms DOW - Not yet. You've announced it; you haven't delivered it.

Mr ROCKLIFF - We've delivered on the commitment. We have started delivering, some \$15 million last year. This year, we have \$150 million across the forward Estimates. When we sat down with the AMA, if my memory serves me correctly, they estimated the total cost of that project would be around \$400 million. We expect over the next decade, delivering the digital health strategy will be some \$475 million.

Through working and sitting down with key stakeholders, we talk about these things, we understand and assess the need for health system improvements; and that would be a very good example of it. I am sure there are many others, in fact.

Ms DOW - Premier, what is the current total number of funded nursing and midwifery vacancies for each hospital ward and unit across the state?

Mr ROCKLIFF - Vacancies. I will see if we can access that information for you, Ms Dow. Just bear with me -

Ms DOW - I can put it on notice, if that's going to waste time.

Mr ROCKLIFF - Ms Morgan-Wicks will provide that information for you.

Ms MORGAN-WICKS - A purpose of implementing our new human resources information management system (HRIS) - it's one of the key priorities -so that we do have then to hand a daily figure in terms of vacancies. I don't have a vacancy total as at today. Vacancies, however, will always exist with a nursing workforce the size of ours, which is in excess of 4600 FTE. What I do have, however, is turnover rates and I note in terms of the turnover rates for all nurses and hospitals that we are currently sitting at 5.59 per cent for 2021-22 as at the end of March. That is the lowest figure in over three years, in terms of turnover. I have to say that, in terms of all hospitals or health services, a 5.59 per cent turnover rate is incredibly low. However, we are always concerned to have turnover in terms of the hospitals.

We all know the length of time it takes us to recruit good nurses and to train them up and have them performing well and efficiently on our wards. We are working very hard through the outcomes of the Health Recruitment Taskforce to look at each of the issues raised, in particular by the ANMF, but also by HACSU, CPSU and the AMA to try and reduce the time for recruitment, knowing the pressure that is on. In terms of vacant shifts, however, across our hospitals - and often you'll see reporting on vacant shifts - every single day, we are filling those vacant shifts with locum nurses or agency nurse deployments.

We certainly do not try to operate hospitals by leaving vacant shifts. We are attempting to fill them. The best fill is a permanent FTE or a permanent part-time FTE; we then have to go to agency or locum nurses. If we are unable to successfully fill vacant lines, we will look at closure of beds to ensure a safe operating environment is always maintained.

Ms DOW - Through you, minister, to Ms Morgan-Wicks. I have talked about vacancies; what are the current levels around the state for each ward, unit and hospital for overtime, double shifts, and agency staff use?

Mr ROCKLIFF - Just for nursing, Ms Dow? If I am reading this table correctly, overall as of 31 March 2022 we had overtime FTE as a percentage of paid FTE at 2.95 per cent increase. Last year was 2.63 per cent and that is for all awards - allied health professionals, the Ambulance Award, Health and Human Services Award band 1 to 9, Health and Human Services Award band 1 to 5; nurses; radiation therapists; salaried medical practitioners and visiting medical practitioners.

If we look at nurses, Ms Dow, in terms of the percentage overtime FTE as a percentage of paid FTE, there has been an increase of 3.48 per cent; last year, 2.9 per cent. In terms of Ambulance Tasmania, in 2020-21 with 7.99 per cent and overtime FTE as a percentage of paid FTE is 7.79 per cent - there is a slight decrease there.

We have some hospital statistics as well. This is nurse overtime FTE. Again, at the end of March, for 2020-21, the overtime as a percentage of paid FTE was 5.29 per cent. At March this year, it is 5.94 per cent, so an increase. The Mersey Community Hospital was 2.24 per cent in 2020-21; now 3.6 per cent, so an increase. Northwest Regional Hospital in 2020-21, was 4.25 per cent; now 4.43 per cent as of March this year, so another increase. Royal Hobart Hospital, was 2.04 per cent in 2021; and at the end of March 2022, was 2.07 per cent, so that is relatively stable. I repeat those figures for all nurses; I have broken them down there by hospital. We had 2.92 per cent overtime as a percentage of paid FTE in 2020-21; but I repeat the figure of 3.46 per cent as of March this year.

Dr WOODRUFF - Can you summarise for us, please, what you understand to be the main long-term health potential health complications of COVID-19 infection? I am talking here, not about the post-viral syndrome, but particularly the research into impacts on the cardiovascular system, the brain, immune system, et cetera.

Could we have a statement from the Director of Public Health about what we understand about this?

Mr ROCKLIFF - Well, I think we are still learning a lot about this matter, Dr Woodruff. I am not sure if Dr Veitch has had these discussions at a national level at AHPPC.

Dr WOODRUFF - Or just from his own research.

Mr ROCKLIFF - Or Dr Veitch's own research, potentially. I invite Dr Veitch to answer the question if he can.

Dr VEITCH - I would love to have time to do my own research, I have to say. A number of national bodies are scanning the research, providing input to AHPPC, and to clinical advisory groups at a national level. They are throwing their net widely, so they are not just focussing on the, as yet fairly ill-defined, condition of long COVID.

Dr WOODRUFF - To be quite clear, I am not talking about the post-viral long COVID, I am talking about the more prolonged multiple organ damage, which is, I think, a different matter.

Dr VEITCH - I think you are right, Dr Woodruff. There is evidence of reduced lung function months out after COVID; there is evidence of changes to microvasculature that can have effects both in the cardiovascular system and potentially neurologically; there is active work going on to try and find immunological markers that may contribute to ongoing infections that could be precipitated by COVID-19. I think it is probably best that I don't try to profess to have a greater knowledge than that broad scan of the information, but that information will be able to feed into national guidance and inform the specialists who will be engaged as part of the Tasmanian response.

Dr WOODRUFF - Thank you, minister. There is some very strong suggestive evidence that COVID-19 infection, even in people who don't have express symptoms, can have long-term heart problems. For example, some research published in the *British Medical Journal* is quite clear in raising concerns about that. There is also very strong evidence of other cardiovascular outcomes so there is a lot of evidence now that there are issues we need to be very cautionary about. Can you tell me whether you have modelled any of the potential probable burdens of disease for some of these issues in Tasmania? I am thinking of cardiovascular disease, the increased risk of diabetes that suggested and other issues relating to brain function and dementia.

Mr ROCKLIFF - I am not aware of any modelling specific to your question. I am not sure if Dr Veitch is aware of it happening within the nation or not.

Dr VEITCH - I am not aware of that but I know there is intense interest in the long-term outcomes of COVID-19, so I would be surprised if once the risk of those outcomes was better defined such as the numerical model could be applied to it, that people didn't do that sort of work. It would also be very important to look at the other contributing factors, particularly in the Tasmanian context, to those common cardio-respiratory outcomes we are all concerned about. I think it has to fit into a more comprehensive model as well as take into account any consequences of COVID-19.

Dr WOODRUFF - Do you accept that there is the potential for serious long-term health complications, potentially disabling or causing early death from COVID-19?

Mr ROCKLIFF - I would like see the evidence that supports that, Dr Woodruff.

Dr WOODRUFF - I tabled some of it in parliament once.

Mr ROCKLIFF - You have said you have suggestive evidence there but there is evidence and data that you can present. I am sure there is lots of research in respect to this matter worldwide and it will be of intense interest to not only Dr Veitch and others in Public Health but also within the health system when we go about the clinical design of our services we provide as well into the future.

Dr WOODRUFF - A total 167 000 Tasmanians have been infected with COVID-19 since you reopened the borders. Aren't you concerned to be as careful as you can, given what the evidence is telling us about the potential long-term serious disabling impacts of COVID-19 infection? Why aren't you asking people to wear masks? Why is there no public education campaign about what effective mask-wearing looks like?

Mr ROCKLIFF - We are not discouraging people from -

Dr WOODRUFF - You're not encouraging them.

Mr ROCKLIFF - But we are not discouraging people from wearing them.

Dr WOODRUFF - You're doing nothing.

Mr ROCKLIFF - I have been reminded of the health pathway which I think I mentioned already today. Would you like to speak about that, Mr Webster?

Mr WEBSTER - The pathway for people who develop the longer-term symptoms, not just long COVID-19 but longer symptoms of it, is to make sure that we have the connections that flow through our primary care system, because GPs are the most likely to pick them up in the first place, through to our specialist services within the Tasmanian Health Service. We have worked on that pathway with primary care and that has been endorsed. In fact, we are the first state to complete that work.

As the Premier announced, the second step is then to have a navigational referral service within the Tasmanian Health Service, recognising that this is a range of symptoms. As you said, it's anything from dementia to cardiovascular and you can't have one clinic that can service all, so we're setting up a room navigation referral service that can navigate the patient from the GP to the particular service they need and we will have that in place by September. It is an evolving area. I understand that the Doherty Institute are doing national research into this which will inform the AHPPC's response and all our governments' response, but we're so small in numbers that at this stage there is no local research.

Mr O'BYRNE - Premier, just a follow-on of that previous question around overtime shifts. You quote percentages and you know percentages are percentages - 2 per cent is the same as 8 per cent if you don't know the base of the figures. How many overtime shifts are covered and what does that equate to in terms of dollars?

Mr ROCKLIFF - Dollars?

Mr O'BYRNE - Yes, how much does it cost? You can talk about percentages, but what does that actually mean in terms of numbers of shifts and dollars?

Mr ROCKLIFF - If you look at double shifts as a percentage, the total worked for the period January to March this year at the Royal Hobart Hospital, so for nursing in hospital south, the total figure is 2.21 per cent.

Mr O'BYRNE - What does that equate to?

Mr ROCKLIFF - That equates to 1036 double shifts in 46 885 shifts.

Mr O'BYRNE - That's at the Royal?

Mr ROCKLIFF - Yes.

Mr O'BYRNE - Have you got a dollar figure on that?

Mr ROCKLIFF - I'd have to get the dollar figure for you, Mr O'Byrne.

CHAIR - Can you put that in writing, Mr O'Byrne? Mrs Alexander.

Mrs ALEXANDER - The Tasmanian Government has expressed a commitment to strengthening the clinical advice and consumer feedback in health planning. Can the minister

please update the committee on the establishment of the Tasmanian Health Senate, which is a key initiative of Our Healthcare Future reforms?

Mr ROCKLIFF - Yes, thank you, Mrs Alexander. I am pleased to announce the establishment of a new Tasmanian Health Senate which will bring together health expertise to provide advice on issues of statewide and strategic importance to the Tasmanian health system. Founding the new Tasmanian Health Senate is one of the key initiatives under the Our Healthcare Future reforms to strengthen the role of clinicians in health planning and to deliver on our commitment to a stronger consumer voice. Today I table the exposure drafts of the terms of reference, here they are. I table those for public release, bringing us one step closer to establishing the health senate.

Health senates in Australia and across the world have been found to deliver benefits to healthcare reform by strengthening the clinical and consumer voice in health planning. Once established, the Tasmanian Health Senate will provide clinical leadership and advice on system-wide planning and delivery of health services to meet the needs of the Tasmanian community. The health senate will bring together clinicians, carers, consumers and the community to debate on statewide and system-wide health matters which will inform quality healthcare, planning and delivery for Tasmanians.

The exposure draft is available on the Department of Health website and I invite the Tasmanian community to provide comments and feedback by 17 June 2022. I would like to acknowledge and thank everyone who has contributed to date towards the establishment of the health senate and related documents. The feedback received through this process will guide the final terms of reference and operational framework and the next step will be to seek membership nominations for the Tasmanian Health Senate through an expressions of interest process, with an inaugural meeting plan for later this year. I am pleased to launch the Tasmanian Health Senate and look forward to the future benefits of embedding clinical advice and a stronger consumer voice into our health planning. Thank you for the question.

Ms DOW - Premier, the budget papers show a \$126 million cut to health next financial year. When questioned about this last week in parliament you stated the following:

The 2021-22 estimated outcome by health included approximately \$250 million to cover COVID-19 related expenses. As we are now transitioning out of COVID-19, the 2022-23 Budget COVID-19 related expenditure will instead come out of the Treasurer's reserve, which is there to meet unforeseen costs. To compare apples with apples you therefore need to take the COVID-19 funding out of the 2021-22 estimated outcome. When you do that, the 2022-23 Budget actually represents an increase of some \$123 million on health. There is a simple explanation for you.

How much of the Treasurer's reserve in 2021-22 was allocated to COVID-19 health expenditure?

Mr ROCKLIFF - Thank you, I have been following you reading that out and it is exactly as I had said. That advice is advice from Treasury, as I understand it. The question you had was?

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Ms DOW - How much of the Treasurer's reserve in 2021-22 was allocated to COVID-19 health expenditure?

Mr ROCKLIFF - That would be a question for the Treasurer. If you could ask that question, I believe the Treasurer is up tomorrow.

Ms DOW - Are you concerned, though, Premier, that there is not enough money in the Treasurer's reserve for your response to COVID-19, given that large amount of expenditure last year, the public health response?

Mr ROCKLIFF - Not really.

Ms MORGAN-WICKS - I can't comment on the capacity of the Treasurer's reserve, being a matter for Treasury and the Treasurer. But certainly, in terms of health expenditure, last year we did have significant expenditure in the Tasmanian vaccination emergency operations centre, for example. We do continue that, though not to the height of last year's given where we are at in terms of vaccination and the rollout to GPs and pharmacies. They are now also doing that in conjunction with influenza shots, so two for one.

Certainly, in terms of the funding itself, we have never had a concern from a health perspective in terms of receiving funding that is required for the COVID-19 response.

Ms DOW - I want to ask Dr Veitch if, in his capacity as Director of Public Health, he has any concerns about the monies allocated to the Treasurer's reserve and whether that would be adequate to cover the health response now that is not coming out of the health budget?

Mr ROCKLIFF - Dr Veitch is not able to comment on the Treasurer's reserve, Ms Dow.

Ms DOW - Premier, why is there only \$50 million included in the Budget next year for increase to hospital demand? In my previous question I refer to you referring to the transition out of COVID-19. Arguably, we are right in the midst of the pandemic in our health system as we see wards close and services changed around the state, and large numbers of staff absent. Why is there only such a small amount allocated? And why does that not continue in the out years, because we know demand is not going to decrease?

Mr ROCKLIFF - We had an allocation with respect to our beds across the forward Estimates. We brought forward the opening of the beds earlier to account for the borders opening and the COVID-19 response. There is \$50 million next year to maintain those bed numbers, so that is the reason for the \$50 million. That is what is required to keep those beds open.

Ms DOW - Why then, in last year's Budget, was there \$115 million allocated in the out years of 2024-25 with this same line item? Why is that no longer there?

Mr ROCKLIFF - We have received permanent funding in the out years for the hospital beds.

Ms DOW - How much does that funding equate to over those years?

Mr ROCKLIFF - Our CFO may well be able to explain some of this. Craig Jeffery, the chief financial officer. While Craig gets settled, we have invested heavily to increase the number of beds available to hospitals to manage the health demand. The most recent national statistics released by the Australian Institute of Health and Welfare, in August last year, show that Tasmania had 1472 available hospital beds in 2019-20. This is 2.75 beds for every 1000 people, well above the national average of 2.47 beds. In fact, it was the highest of any state, with only the Northern Territory having a higher number. The most recent data from the department's 2022 bed census, which is yet to be published by the AIHW, shows Tasmania's bed numbers have further increased to 1656 beds in April 2022. This is an increase of 296 beds, or a 21.8 per cent increase since April 2018.

I have mentioned with matters before but when it comes to the investment to meet health demand, including the progressive rollout of 152 new beds announced last year, this includes beds that will be reflected in future bed numbers reported through the Australian Institute of Health and Welfare, such as the new trauma and acute surgical unit and ward 3A at the Royal Hobart Hospital and the newly opened ward 3D at the Launceston General Hospital.

Do you have an answer to Ms Dow's question, Mr Jeffery?

Mr JEFFERY - I will do my best. In the 2021 Budget there was funding bed and demands major hospitals \$25 million in 2020-21, \$25 million in 2021-22. Then in last year's Budget there was bed and demand funding \$29 million for 2022-23, \$29 million for 2023-24, \$115 million for 2024-25, which I think was the \$115 million you mentioned, Ms Dow. That \$115 million is permanently funded ongoing.

The Government, I think, in the 2018 election, had a commitment to open a number of beds that increased over a number of years. Yes, 250 beds. The minister made a decision as part of the fight against COVID-19 to bring forward the opening of a number of beds, which I think he has just mentioned. And the \$50 million that is in this year's Budget is funding the bringing forward of those beds.

Ms DOW - Just to be clear then, how many additional beds will that be each year and how much money each year?

Mr ROCKLIFF - TASU has 24 beds.

Ms MORGAN-WICKS - We have provided the increases in beds as calculated by the bed census numbers through IHW. We brought forward an additional 152 bed openings as part of COVID-19 to meet the 15 December date. We are progressively working through those. From recall, we have about six left of the 152 that we are still working through at the Royal.

In terms of bed numbers on any given day, as we all know, do fluctuate and vary according to the safe staffing principles that are applied to them. In terms of trying to predict numbers of beds, we could probably provide an answer but it depends on openings of infrastructure that is rolling out over the next few years.

For example, we expected at this point in time to have the Peacock mental health beds open, but due to the unfortunate fire incident that occurred on Christmas Eve we don't have those beds open but we're hoping to have them open at the end of the year. It is a complicated infrastructure picture to show where the new beds are coming on and how they are going to be allocated. We also include in those bed numbers Hospital in the Home, for example, in our attempts to expand the Hospital in the Home service.

Dr WOODRUFF - Returning to the long-term health impacts of COVID infection, there is a lot of research. I want to point to some research published in the *British Medical Journal* earlier this year that reported on some research in nature medicine that found that even one year after infection with SARS-COV-2 people were at a higher risk for a range of cardiovascular disorders. For example, there was a 72 per cent increased risk of heart failure, a 63 per cent increased risk of heart attack and a 52 per cent increased risk of stroke, compared with controls. That was an enormous study of 154 000 cases of COVID-infected people. They are big numbers and the researchers found that this was independent of the types of groups, so younger people, older people, African-Americans, non-African-Americans, people with obesity, people without, all had an increased risk. Their conclusion was that if you have been infected with COVID, there is a substantial increased risk of cardiovascular disease, and the most important thing to do is to prevent infection in the first place.

Minister, despite findings in multiple studies, I think we know enough. The early evidence is very strong that there are serious potential risks across a number of health disorders. Why are you continuing to stay with a public health response which just looks at avoiding hospitalisations? Why aren't you looking at what we need to do to avoid people becoming infected in the first place?

Mr ROCKLIFF - Out of interest, did the study talk about the variant in terms of Delta versus Omicron, in that 154 000?

Dr WOODRUFF - That was research done on the Delta variant, but other research I have is looking at the Omicron variant. There's no good news from any of the variants we have information about.

Mr ROCKLIFF - The number-one priority of the Government has been to keep Tasmania safe. We have done that all the way through. We based our 15 December opening on the Delta variant, which is far more severe. Omicron hit our shores, which was highly transmissible, but it would appear less severe than Delta.

Dr WOODRUFF - In the acute phase of the disease, yes, but this is about the long term. It is not unknown, it is actually increasingly known and increasingly concerning.

Mr ROCKLIFF - Well, there is a lot more research to be done and we will be more than open and willing to avail ourselves of that research and the impacts on our health system and the health provision we need to provide our population. You mentioned cardiovascular disease in more vulnerable communities and I am mindful of the fact that in my neck of the woods, the north-west coast, we have a high rate of cardiovascular disease, so I am interested in what you're saying, but I think you're also saying that our response hasn't been the response

you would have had if you were health minister or director of public health or whatever you'd like to be.

I believe we've managed the pandemic from the start to now - and of course we're transitioning - well and we'll continue to manage it well through sensible lifting of restrictions. We've often waited to see the impacts of lifting restrictions on the mainland. We chose to open our borders with a vaccination rate of 90 per cent, not 80 per cent as other states have done, to ensure that our best line of defence was really up in that 90 per cent level.

I accept your criticism that you would have done it perhaps another way but I believe we've managed well through the best national advice and indeed local Public Health advice which has been guided by the very best of expertise. I would have to say that if you were in another state and you were questioning the minister for health of the day or the premier of the day on why there was a separation between the public health advice and response and the government response, then you might have some argument, but we have always maintained we are very closely guided by Public Health officials in this matter.

Dr WOODRUFF - Minister, you continue to talk about a period which is long past now, which is the period where we had almost no infections in Tasmania. It is a different time, 167 000 Tasmanians have been infected with a virus that the research is increasingly telling us has the potential for serious long-term health complications. For Tasmania from the hospital budget point of view, if we are talking about increases of that scale for cardiovascular disease, for example, that's a big concern for a hospital budget looking ahead into the future.

Thinking about what can be done for people's lives, it's pretty clear and there's some evidence that was published only this week in *Nature Epidemiology* that vaccination against SARS-CoV-2 only lowers the risk of infection with the virus by about 15 per cent. In other words, vaccination is not your pathway to avoiding becoming infected; in fact 85 per cent of people according to this 13 million people cohort study are not protected against infection. The only thing that can protect people is good ventilation and wearing masks indoors, so why haven't you changed your advice to make it crystal clear to Tasmanians what effective mask-wearing looks like and are setting a standard of requiring people to wear masks indoors?

Mr ROCKLIFF - Again, we have had a very sensible lifting of restrictions. I might also say that in high-risk settings we are still required to wear masks and that will no doubt continue for the foreseeable future. We still have masks worn in high schools, for example. I get many emails a day from people who are less than satisfied with our policy that continues with respect to wearing masks in high schools. I've put a lot of social media posts up and irrespective of the subject matter of the day there is considerable commentary on mask-wearing generally, in that sense -

Dr WOODRUFF - It's partly because we've dropped the ball in terms of leadership. We're not modelling it. When you went to the footy, a photo of you in the newspaper. You went to the footy, the next day, I am sorry to hear, that you were infected with COVID-19 and were sick. The press release that was handed out by your department said zero about mask wearing. Nothing in this press release that was reported here mentioned the recommendation that people wear masks.

Why? Obviously, to protect you from the embarrassment of the fact that you were sitting in close proximity with people, and got infected. It's just a numbers game. There is nothing perfect here. We cannot possibly hope to prevent all infections. No-one is suggesting that. But reducing all the possibilities that we can, will mean less people infected, and less people at risk of long-term complications and all the other things that go with that. Why aren't we encouraging people to wear N95 masks? Why aren't we mandating people do that in indoor closed spaces?

Mr ROCKLIFF - I could refer you to Dr Veitch; however, I repeat that we have lifted restrictions sensibly, in line with advice, and we still have masks in high-risk settings, such as a high school. I hope that would be a signal to you that we do take this seriously. But also, we are not saying to people they shouldn't wear masks. People can wear masks in whatever setting they like.

Dr WOODRUFF - Modelling helps. We all need to model good behaviour and Premiers, Ministers for Health especially need to model good behaviour. That is what they are doing in the US, everyone wears masks in the US.

Mr ROCKLIFF - That's no massive endorsement of a COVID-19 pandemic response. It has improved since President Biden.

Dr WOODRUFF - Its improved since Biden administration. It had a long way to catch up from.

Mr O'BYRNE - Just rounding out my questions around the labour force stuff. Obviously, using agency staff does potentially mask a few other structural issues inside the health service. Are you able to inform the committee of how many positions are filled by agency staff as an effective FTE, if you have got that? How many shifts and how much is being spent on agency staff across the service?

Mr ROCKLIFF - We will get that for you and we will probably be able to find it, Mr O'Byrne. From memory, some these questions were asked in Legislative Council last year.

Mr O'BYRNE - I just want to clarify the figure you gave before - 1000 shifts. Was that for the March quarter at the Royal?

Mr ROCKLIFF - It was 1 January to 31 March 2022.

Mr O'BYRNE - the March quarter for the Royal.

Mr ROCKLIFF - I have some dollar figures. There has been an increase in the usage and associated costs of agency nurses from March 2021 through to March 2022, from \$10.9 million to \$17.6 million.

Mr O'BYRNE - \$17.6 million.

Mr ROCKLIFF - \$10.9 million to \$17.6 million. The increase is reflective of the demands for assistance in COVID-19 services and the expansion of services in our hospitals.

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I've got some figures in terms of nursing locum costs by service: for the full financial year 2020-21 for our THS hospitals in the South - \$3 054 994; and as at 31st March - \$3 961 310.

Mr O'BYRNE - Sorry, can you repeat that?

Mr ROCKLIFF - \$3 961 310

Mr O'BYRNE - That's for the March quarter this year?

Mr ROCKLIFF - That's up to 31 March this year. The first figure I gave you was the whole financial year from 1 July to the end of June. The last figure I gave you would have been 1 July to 31 March.

If I go now to THS hospitals North and North-West. The previous financial year - \$10 393 360; as of 31 March this financial year - \$10 032 989.

If we're looking at, do you want me to keep going?

Mr O'BYRNE - I can put it on notice, that might be easier for the committee. It would also be nice to get my head across, how many actual shifts and positions. If you permanently filling people from agencies, that's pretty expensive, isn't it?

Mr ROCKLIFF - Ms Morgan-Wicks would like to make a comment regarding my figures.

Ms MORGAN-WICKS - In terms of our high agency usage for nurses. Certainly, it has been particularly impacted in the first quarter of this calendar year - January to March 2022 - by COVID-19 infection, furloughing and the close contact rules as they then applied to our hospitals. We did have attempts in terms of exemptions being offered, daily RAT tests et cetera. to keep staff on. However, a significant proportion of our workforce are also carers, so for COVID-19 cases in their families, and being unavailable, we would use an Agency nurse to try and fill those shifts and keep beds open.

In terms of our vacancy rates, our preference is always, across our hospitals, to attempt to recruit nurses into those positions on a permanent basis. I will also note, that we are in a global pandemic and we have suffered some significant impacts of the pandemic on our ability to recruit, not just in nursing but across all of our health professions.

Ms DOW - Do you have knowledge of the number of nurses that are currently not working, or registered to work, as a registered nurse across Tasmania. Do you have an understanding of how many there are actively in the community, that could possibly be working in the system, and what are you doing to attract people back to nursing?

Mr ROCKLIFF - Registered nurses, but not working? I'm not sure if we have those figures. I'm advised that we have had registers of nurses that we have been able to call upon to come into support our vaccination clinics, as an example. We would have a register of nurses, or the number of nurses registered.

Ms DOW - I want to take you back to our discussion about beds. I am a little perturbed to understand that in that official count of beds and increases in beds across the state you include the hospital at home program. I thought that it would have been inpatient beds, not community-based beds.

Mr ROCKLIFF - The hospital in the home program can provide an acute care setting. It is an acute care setting, as I understand it; an inpatient setting, and there is 12.

Ms DOW - The commitment your Government made in 2018 was to increase bed numbers at the Royal to 250. Excluding COVID-19 and the increases that you have made in relation to COVID-19 increased demand, which we spoke about before, what is the net number of new beds at the Royal Hobart Hospital that have been delivered to date, and are you on track to reach the 250 that you promised by 2024?

Mr ROCKLIFF - In 2018, we did make a commitment to open 298 beds by 2024, comprising 250 beds at Royal Hobart Hospital; 40 beds at the Launceston General Hospital; and eight beds at the North-West Regional Hospital as an acute medical unit. As of 30 June 2022, there will be 231 additional beds available in our health system, comprising 190 beds in the public hospital system. That includes 131 beds at the Royal Hobart Hospital - with 85 medical beds at the Royal Hobart Hospital, including new hospital in the home programs, more surgical beds, more adolescent beds and expanded ENU. Forty-five beds at Launceston General Hospital, including the new ward 3D, that's 28 beds; more adolescent beds on ward 4K; new surgical beds; and 14 medical beds at the Northwest Regional Hospital.

In effect, the committed beds for the north and north-west have been fully delivered ahead of schedule, with ward 3D originally committed to not come online until the 2023-24 year. For the Royal Hobart Hospital, 131 new beds is actually a significant achievement because we delivered that during the pandemic, and noting the first beds are an election commitment we committed to commence rolling out from 2020-21 - less than two years ago from today.

In addition, we have put in place a further 41 new beds through public sector partnership agreements. Each and every one helps free up a bed in our public hospital system and helps people get timelier care.

Ms DOW - What is the financial cost per annum of bed block?

Mr ROCKLIFF - Per annum of bed block?

Ms DOW - Related to bed block.

Mr ROCKLIFF - We can talk about our access and flow program, of course.

Ms DOW - I have a couple of others that are related, so I will give those to you as well. What is the financial cost per annum of ambulance ramping in Tasmania? What is the total time ambulances have been ramped for the past 12 months?

Mr ROCKLIFF - There is a few questions there. You asked me a question of total bed block costs.

Ms DOW - The total cost to the health system related to bed block each year, or this last financial year.

Mr ROCKLIFF - While we find that information, the work we are doing with respect to the statewide access and flow program is developing a statewide and system-wide framework for integrating, delivering and monitoring work aimed at improving patient care and flow through the Tasmanian health system. Again, Our Healthcare Future sets the strategic direction for the future of Tasmania's health system. The statewide access and patient flow program supports the delivery of that future state and aligns with the Department of Health's strategic priorities in 2021-23.

The program recognises access and patient flow is a system-wide challenge. It is taking a holistic approach to developing solutions, with four program aims that reflect the multi-dimensional nature of access and patient flow:

- Improve the patient's experience during their treatment.
- Improve the staff's experience in providing treatment.
- Improve the clinical patient outcomes achieved.
- Increase the efficiency with which we provide treatment.

The program will undertake statewide projects in consultation with local service areas. Local facilities will continue to implement continuous-improvement access and flow activities outside the program with oversight from the program.

It is focusing on the following streams of work:

Access and entry flow: emergency department access and flow; introduce people, process and technology changes and embed usage of clinical pathways to streamline patient flow through emergency departments; planned admissions - develop and implement a protocol to enable direct admissions, which will reduce ED presentations. We announced earlier today the inter-hospital transfer policy.

Paramedic and ED clinical patient care protocols and develop and implement a protocol governing hospital clinician and paramedic responsibility for care during the period between triage and handover.

Private hospital clinical collaboration to develop clinical protocols to support access to on-demand emergency and inpatient capacity within private hospitals.

Hospital avoidance strategies: design strategies that lead to reduced demand for ED services, including but not limited to promoting Tasmanian health pathways within the Tasmanian Health Service.

We have an acute hospital inpatient flow, introducing people, process and technology changes to streamline inpatient flow through inpatient wards. Sub-acute and district hospital inpatient flow: introduce people, process and technology changes to streamline patient flow through inpatient wards and streamline patient transition from acute hospitals.

We have Hospital in the Home, interfacility transfers and we have a transition and discharge flow as well -

Ms DOW - Premier, could I just interrupt you just at that point. I have a question that is specific to that. It is around those Tasmanians that are waiting in hospital for an aged care package or their NDIS package. What process are you putting in place or how are you ensuring that they are able to get those as quickly as possible? And are you looking at employing more social workers across the medical wards to assist with this process or enhance discharge planning across those medical wards across the state?

Mr ROCKLIFF - Social workers. Do you mean health navigators?

Ms DOW - Yes. I am talking about the Tasmanians who are waiting in hospital for their package to be discharged back into the community, and support in the community.

And you will get me the costs I asked for around bed block and ramping, won't you?

Mr ROCKLIFF - Yes. We have some figures there for you. I am not sure if they are exactly what you were after. I'll put that on notice. Dale, do you want to say something regarding Ms Dow's question about aged care?

Mr WEBSTER - The issue of aged care packages that have been approved and NDIS packages approved where the services are not yet available creates a large impact across our hospital system, as well as our mental health system. What we have done about that is our executive director, Allied Health, works closely with both the Aged Care Commission and the NDIA in terms of liaison direct with higher levels to advocate for individual clients to move across. The issue is less of the social work within the hospital being able to help them do it, as to the commission or NDIA being able to put the package together and fund it in a way that creates the residential spot at the other end. So, it does not need additional resources at our end; it needs a speeding up of the process at the Commonwealth end, if you like, to overcome this.

This is an issue that is Australia-wide. Through the ministers for health committee, this has been directly raised with former relevant ministers and I am sure it will be on the agenda for future meetings.

Ms DOW - Just to clarify those figures, are you going to come back to me with those figures or would you like me to put them on notice? The cost of bed block, ambulance ramping?

Mr ROCKLIFF - We don't have the cost of bed block but we monitor the numbers and hours ramped for major hospitals. Numbers ramped July 2021 to March 2022, 14 399. Hours ramped; 20 363.

Ms WHITE - You can't provide the financial cost of bed block?

Mr ROCKLIFF - We don't calculate that as a dollar figure. We calculate the number of hours ramped. Paramedics are paid as part of our employee expenses.

Dr WOODRUFF - That is another question and I have the call.

About ambulances, you have added some more money in this Budget towards some additional full-time paramedics, which is welcome. But it is completely incommensurate with the scale of the need. I want to provide you with some information that you may or may not be aware of about the state of the standard Friday and Saturday nights that paramedics in many places in Tasmania are facing.

On 13 May, the state operational shift report from the communications centre shows that there were five stations that had no staff: Huonville, Dodges Ferry, Bridgewater, Kingston, and New Norfolk were all unstaffed. So, there was nothing south of Hobart.

Mr ROCKLIFF - What was that date, Dr Woodruff?

Dr WOODRUFF - That was 13 May, a Friday night shift. South of Hobart, there were no ambulances at all available. On the Saturday night, the day after, 14 May, there were no ambulances staffed at Huonville again, Dodges Ferry again and then at Glenorchy.

On another date, 7 April, there was no crew for Sorell again, Dodges Ferry, Bridgewater and also New Norfolk. Another date I have, 12 April, shows that there were vacant shifts in Hobart, Claremont and New Norfolk. This is just a sample but I am hearing that Friday and Saturday night in particular are very difficult to staff. There are multiple systemic reasons for this, there is no one reason but there has been a perfect storm of long-term weariness, the impact of COVID-19 and fundamentally and very importantly, the casualisation Ambulance Tasmania has taken to paramedic graduates, so that once they finish their three-month contract, it seems they now no longer go onto full-time permanent contracts. They are being moved around to plug gaps in the rosters and this is having a devastating effect on their own mental health and on team morale with people who are working in the hardest situations every day.

Will you make a commitment to end the casualisation of paramedic graduates, in particular, who are leaving Tasmania because they can't save to buy a house? They have no security and they are working without any sense of commitment from Ambulance Tasmania to them as an employee.

Mr ROCKLIFF - I would like to say that in 2019-20 we had 567.4 FTE at Ambulance Tasmania. As of 31 March 2022, we have 652.15 FTE employees. Through the 2021-22 financial year Ambulance Tasmania has also recruited key leadership roles to support the ongoing transformation of the organisation. The director of operations commenced in January 2022 to lead the operations directorate for the organisation. We have our commitment for 48 new paramedics, most of which have been filled, and 11 more with respect to Sorell and Huon.

In early 2021 Ambulance Tasmania introduced a graduate program for paramedics employed on a casual basis. Early monitoring of the program highlighted some concerns, particularly in relation to working hours and clinical support. This program has been discontinued and graduates of the program have been placed on 12 months full-time fixed-term contracts as per the conditions of other graduates in the organisation.

Dr WOODRUFF - Why are they still on 12-month contracts? Why aren't they permanent contracts? Why don't they become full-time employees?

Mr ROCKLIFF - I will invite the chief executive of Ambulance Tasmania, Mr Joe Acker, to the table to perhaps go through some of the more operational nature of your questions, Dr Woodruff.

Dr WOODRUFF - Mr Acker, why aren't graduate paramedics being put onto permanent full-time contracts instead of 12-month contracts?

Mr ACKER - Through you , Premier, we hire graduate paramedics to fill vacancies and we put them onto fixed-term contracts to get their what we call graduate year, which is their induction year. We can only move them into permanent positions when we have funded positions to move them to, so we put them into fixed-term positions as they are, the vacancies, and then if during their first year or after we can put them into permanent positions, we do so.

Dr WOODRUFF - Okay, thank you, you've answered my question. I have heard the reason young paramedic graduates are leaving Tasmania is because they have no confidence in a system that doesn't have confidence in employing them on a permanent basis and they are being moved around to fill gaps. Minister, I just point you to the Mingara [TBC] review, which was very clear about the numbers of new paramedics that needed to be employed, which was 224. You talked about employing new paramedics and you mentioned 11 in the south. That is a drop in the ocean compared to what the independent Mingara review recommended is needed. In case you haven't seen it, your counterparts in New South Wales today have announced some extraordinary commitments to ambulances, the sort of stuff we should have had in Tasmania. If we don't have that, do you recognise that we are at grave risk of many paramedics leaving Tasmania permanently or leaving paramedicine altogether because of the stress they are under?

Mr ROCKLIFF - I would like to highlight, however, that we have recruited 48 paramedic positions that we committed to as a government at the last election. This includes an additional 24 paramedics for the urban communities, with 12 based in Hobart and 12 based in Launceston; 24 paramedics have been recruited to support regional stations across the state, including Sheffield, Strahan, Campbelltown, Ouse, Swansea, Miena, Alonnah and Bridport, and this commitment has also seen the upgrade of the New Norfolk station to a paramedic-only station in recognition of the growth of the community. Bridport, as I mentioned the other day, began operations as a single branch station on 31 May 2022 with one paramedic on duty during the day and on call at night, supported by a volunteer ambulance officer.

The election commitment of 24 paramedics for rural communities was to be staggered over two years and these positions were brought forward to support the state border reopening

and the response to COVID-19. The secondary triage model is currently being resourced to a sustainable 24-hour, 7 days a week operation, supporting more people in the community who call triple zero and are assessed as not requiring an emergency ambulance response to be referred to telehealth and other alternative referral pathways. Recruitment is underway for nine community paramedic positions across the state, with a new training program commencing in June this year, and these community paramedics will be able to support more Tasmanians to receive care in the community and reduce pressure on our hospital emergency departments for patients with non-life-threatening medical care needs.

The recent commitment to convert Sorell and Huonville stations to career paramedic-only ambulance stations from July 2022 will see a further 11 paramedics recruited to these locations and we are working very closely with volunteers from both stations to identify new models of service delivery. A 10-year master plan is being commissioned by Ambulance Tasmania to assess the future needs of communities in Tasmania and to determine, using evidence, where best to expand and deploy new ambulance services around the state. The report from the consultant Operational Research in Health [TBC] is expected to be complete in November 2022.

From the RoGS data I'm looking at, in terms of dollars spent per person in the population for ambulances, if we go across, Tasmania is the highest of that, and if we look at New South Wales at 136.68, we are at 201.7 per person, so any announcement in New South Wales would most certainly be welcomed based on those figures, but we are 201.7.

Dr WOODRUFF - How many new paramedics have graduated in the past 12 months and how many of them have been given permanent full-time positions?

Mr ROCKLIFF - Well we can comment perhaps, and I'll refer to Mr Acker on the graduates we've taken on in Ambulance Tasmania, perhaps, Joe do you have that information, I'm not sure -

Dr WOODRUFF - I thought graduates had to have 18 months for training. I thought there was an 18 month period for on-the-job training required after they graduated.

Mr ACKER - Through you Premier, Dr Woodruff, paramedics are unlike other health professions in APRA, so when paramedics graduate they are essentially 'fit-for-service.' Our internal process is that they have to accomplish a number of competencies in 12 months after we've hired them and if they need more support we provide that ongoing, but it is to ensure that they're safety and clinically competent, but there is no requirement under APRA like there is for other health professions.

Dr WOODRUFF - 12 months, not 18 months, you said.

Mr ACKER - 12 months is the minimum, in some cases we go to 18 months, depending on the needs of the individual.

Ms DOW - Premier, how many hours are ambulances ramped for, over the last 12 months, how many hours?

Mr ROCKLIFF - So we have the last nine months, Ms Dow, and the hours ramped - did I indicate this before - 20 363 -

Ms DOW - Can you say that again please.

Mr ROCKLIFF - 20 363 hours ramped.

Ms DOW - Thank you. Premier, is it correct that your department has calculated that ramping costs \$3 million per annum?

Mr ROCKLIFF - I'll stand to be corrected, but I'm not aware of it. We could probably work it through, of the hours ramped, and I'm not saying there's not a report somewhere with potentially that figure on it, but you could probably calculate that figure through the hours ramped and 20 363 by the cost of the per-hour - Dale, sorry.

Mr WEBSTER - Through you Premier, you could calculate it that way, however, you need to work out what was the call on those ambulances? Would they have been stationary anyway and those sorts of things. It's an incredibly complex calculation to do. As the deputy secretary for that area I'm not aware that we've actually calculated it in the way that you're asking and as the Secretary said, it's part of access and flow. It's incredibly difficult to calculate bed-block ramping as an element when you've got all the moving parts. It really is about the level of activity within your hospital, is how we measure it, rather than bed-block or ramping. So, is our activity up, is our activity down, those sorts of things.

Ms DOW - Okay. Premier, when you said the cost of bed-block is not costed, so bed-block is not costed each year and the impact of that on the Budget. Are you sure that you don't have a figure for that in front of you? It feels like you are being quite reluctant to give that figure.

Mr ROCKLIFF - No reluctance, Ms Dow, if the figure was here I would give it to you. Ms Morgan-Wicks would you like to speak to that?

Ms MORGAN-WICKS - Through the minister, I am not aware through extensive preparations for Budget Estimates that there is a dollar figure calculated that is, for example, monitored at health executive. What we do monitor, as the deputy secretary just mentioned, are the key KPIs in relation to ramping, because we are quite concerned obviously as to the number of hours ramped at each of our hospitals, the impact that then has on patients in our emergency departments and the flow through to inpatient wards and discharge. That is why we have a statewide access and patient flow program that we have commenced and that significant resources both dollars and people have been working through to try to reduce that, particularly given the impact of the global pandemic. We do not monitor, for example at our health executive which meets on a weekly basis, a dollar figure associated with ramping. We obviously closely monitor our employment expenses because ramping is made up of people that are attending to patients during that time that they are delayed on the ramp.

Ms DOW - Premier, how many Tasmanians, this year, have died due to bed-block across each of our major hospitals?

Mr ROCKLIFF - I think we were asked this question last year by Dr Seidel.

Mr LAWLER - Yes indeed, we have been asked this question a couple of times. I refer, in part, to the answer that the secretary has just given in terms of measuring the impact of 'access-block'. 'Bed-block' and 'access-block' are often used interchangeably and the fact that it is not a financial impact, it is an operational impact. There is well-recognised data that has been published both in Australia and also internationally where countries face similar problems to this with respect to increasing block and that is partly through increasing demand on our acute sector, challenges of primary care and also the deferred care burden that we have seen through COVID-19.

We do not measure mortality associated with access-block. That is not something that is easily achievable. The definitions are not as clear as we would like. There has been, as I mentioned, research done around the increased incidence of adverse events, increased incidence of medication errors and increasing patient mortality that is seen in high areas of access block but it is not done on individual patient basis. The measurement of such mortality is very difficult. We do collect adverse events through our safety reporting and learning system, there are a number of ways in which those events can be categorised, sometimes it categorises infrastructure, sometimes it categorises demand, sometimes it is categorised as medication error. It is not possible to bring them all together and say this x-number of deaths or x-number of adverse events are purely related to access-block.

Ms DOW - Can the same be said about ambulance ramping?

Mr LAWLER - Again, it is not a figure that is possible to pull out. We can identify adverse events that have been recorded and associated with ramping, we do that as a routine matter of course and not only do we look at those through the lens of the capacity of the hospital to respond but we also regularly review them at Ambulance Tasmania executive committee. Sometimes they are classified as infrastructure issues and sometimes as service delivery issues.

We do look at them on a severity rating as well, there is a severity assessment code that is applied to any adverse event that looks likely in consequence and we look at all SAC1s and SAC2s. We also look for trends in the numbers across the board but also it is very important to note that it is difficult to necessarily draw a very close line between ramping and the adverse outcome because we are talking about patients frequently who are very sick and are in hospital with significant pre-existing conditions.

Mr O'BYRNE - Premier, I am sure you are aware and I know that in my electorate office we are getting increased presentation from family and parents around adolescent mental health and appropriate facilities to respond to that. The previous health minister in 2019 stated that a first dedicated adolescent mental health unit was to be built and was being built at that time. I am hearing stories of families having to relocate to Victoria to access mental health facilities. Could you talk to me about what is in the Budget for those families?

Mr ROCKLIFF - I can. I take the question in good faith. We have an hour allocated for mental health and wellbeing towards the end of the day. We have our child and adolescent mental health investment which you would be aware of, and -

Mr O'BYRNE - It's more in the hospital system, having a dedicated unit.

Mr ROCKLIFF - If I can possibly find that information for you; I don't have our mental health team here, Mr O'Byrne. Before I throw to Mr Webster, we have a number of initiatives on the way. You would have been aware of the Child and Adolescent Mental Health Report that we did a few years ago, which was pretty challenging reading and pointed to a lot of system change, which is happening now. Last year, the commitments we made were in line with the development of an integrated mental health service system. We announced several initiatives.

This investment included \$26 million to deliver better mental health services: \$8.5 million over two years for the mental health hospital in the home pilot in the north-west as an example of that; \$500 000 to commence the [re-write? TBC 4:26:33] of our Rethink 2020 mental health plan as well; our emergency mental health co-response team, which I have mentioned; our Police, Ambulance, and Clinician Early Response (PACER) team; \$1.9 million over three years to deliver peer workforce coordinator and youth peer worker to continue development of a Lived Experience Workforce; \$7.35 million over three years to continue and expand innovative services established through the COVID-19 pandemic, including Tasmania Lifeline, the Mental Health Council of Tasmania's Checkin website; regional coordinators and community engagers.

Were you referring to stage three of the Child and Adolescents Mental Health Service (CAMHS) reform for that in-patient facility? We are going through stage one and two of at this present time; at stage three, it was our intention then to build an in-patient facility, if my memory serves me correctly.

If we look at the \$45.2 million we have committed to implementing the first two phases of reform arising from the Child and Adolescent Mental Health Services Review, our focus is very much on early intervention, but acute facilities at our major hospitals for young people who need urgent, clinical support is very important.

We recognise and are monitoring the growing demands for acute adolescent mental health services in our state and have undertaken initial measures to increase service capacity. The measures include the availability of mental health safe beds, specifically designed for adolescents in the Royal Hobart Hospital and the LGH paediatric units. In addition, from Monday, 6 June, CAMHS at the Royal Hobart Hospital expanded its capacity to respond to child and adolescent mental health presentations and inpatient needs by extending staff hours until 10pm, seven days a week, and that is a very welcome new initiative.

Under the previous arrangements, CAMHS were available at the hospital from Monday to Friday, between 8:30am and 5pm. The expanded capacity will support the RHH Emergency Department and paediatric areas with clinical liaison and assessment in hours in which CAMHS were not previously available. It is anticipated that the extended service will contribute to reduced wait times in the emergency department for young people in the evenings and on weekends, and potentially reduce the length of admission.

The Royal Hobart Hospital CAMHS team will also, where appropriate, provide a seven day follow-up to Royal Hobart Hospital child and adolescent and mental health patients after discharge. This expansion is an important step in building a more responsive and accessible

service. The Tasmanian Government remains committed to the development of an adolescent inpatient mental health unit and day program planned for phase 3 of the CAMHS reform.

Mr O'BYRNE - Just to clarify the commitment - I have a press release in front of me from then Minister for Health, Michael Ferguson saying that it was being built in May of 2019?

Mr ROCKLIFF - At the Royal Hobart Hospital?

Mr WEBSTER - At both the Royal Hobart Hospital and the Launceston General Hospital new pediatric wings, we have built dedicated child and adolescent mental health beds. They can be switched; they are a swing unit between the two types of beds. That is why we have extended the service into the Royal, to make sure we're covering those beds across those 7 days. The CAMHS beds are being built; they're part of the paediatric unit, rather than a standalone unit.

Phase 3 of the reforms sees us moving that into a CAMHS precinct type of approach. It's also important that the mental health precinct in the North-West, and the mental health precinct in Launceston - which are both funded in the Budget for building - will include planning to make sure that we have sufficient ability to change the nature of the units. If I take Northside as an example, we've got 20 beds; our flexibility in terms of swinging is 2. Basically, it's 18 or 2; which means if you've got 8 adults you can't put adolescents in - unless there is less than 2. What we would do, going forward, is make greater flexibility of our unit, so it's still 20 beds, but it might be 5 times 4, which then allows you to actually swing one to an adolescent unit if there is need in the community at that time.

There are a number of steps to creating CAMHS beds over the next 5 to 10 years. The first step was to attach them to the paediatric units; the second step is the flexibility we get from new precincts; and the third step is a dedicated unit that is part of phase 3.

I should also say that it's not just tied to infrastructure. A very deliberate recommendation of the CAMHS review is that we build the community sector services, before we build the inpatient unit. It is essential that we don't have the inpatient unit as our core delivery method for this cohort of patients. The community should be your core delivery unit.

Mrs ALEXANDER - Can the Minister for Health please update the committee on the implementation of Our Healthcare Future reforms.

Mr ROCKLIFF - As part of our ongoing implementation of Our Healthcare Future reforms, I've released Our Health Care Future - Advancing Tasmania's Healthcare exposure draft for public release. I can table that now.

It sets out a shared vision for the future of health services in Tasmania, to support a collaborative health system that delivers consumer-centred and evidence-based care. The release of the exposure draft represents an important milestone in our commitment to develop a long-term plan to ensure Tasmanian's are supported by a world class, integrated health system.

To deliver on our vision we will focus on several key strategic policy areas for future health reform. These include: increasing community care accessibility; strengthening preventative health; partnering with consumers and clinicians; building our health workforce; strengthening Tasmania's pandemic response; and delivering health infrastructure for the future. We've already invested heavily to support these priority areas, and will continue to build on our investment in coming years. This Budget is committing \$11.2 million to health over the next four years. The exposure draft has been developed with guidance from the Expert Advisory Panel, and in consultation with key stakeholders and organisations.

I thank everyone who has contributed to this important process so far, particularly, individuals and organisations that provided written submissions. Today's release is supported by a companion document, *Drivers of Tasmania's Future Population Health Needs*, which provides further information on key trends and will influence health planning into the future.

The exposure draft is available on the department's website. I invite the Tasmanian community to provide comments and feedback by 4 July 2022. By implementing reforms, we will advance the health and wellbeing of all Tasmanians through a strong and sustainable Tasmanian health system. I commend the document to the committee. Thank you for the question, Mrs Alexander.

Ms DOW - Premier, I take you back to my previous questions about the cost of ambulance ramping. I wanted to ask you if you were aware of Ambulance Tasmania's monthly performance reports and whether, in fact, you are briefed about those?

Mr ROCKLIFF - We have regular meetings with our chief executive and discuss a number of matters pertaining to operations of Ambulance Tasmania.

Ms DOW - That being the case, you should be aware that the imputed cost to Ambulance Tasmania of ramping was \$283 109 in April 2022. The total staff cost for Ambulance Tasmania crews ramped at hospital in the 12 months to April was \$2.98 million. Were you aware of that, Premier, when I asked you that question?

Mr ROCKLIFF - I will invite Mr Acker to speak.

Ms MORGAN-WICKS - My understanding is that it is a monthly operational report that is produced within Ambulance Tasmania to inform the Ambulance Tasmania executive, which Joe as chief executive sits on.

Mr ACKER - The monthly report you are referring to doesn't articulate the cost of ramping to Ambulance Tasmania, the Tasmanian health system or the Department of Health. But it does give a general quantitative determination of how many hours the paramedics are spending on the ramp at the hospitals. That was the direction I asked our data people to put together with the expectation that at some point in time we could articulate how much care is being provided by paramedics within the hospital.

When I was a chief executive in Canada, we did that as a way to determine how much time paramedics were spending in lieu of nursing care, for example, and what the cost of that was to the ambulance service. This isn't an additional cost. This is a cost that has already been spent by the ambulance service in terms of staffing paramedics on our system on a

regular basis. We don't have a capability at this point in time to determine the costs of overtime or other things associated with transfer of care delays.

Ms DOW - Is that information going to be provided to the Premier to inform him, as the responsible minister, about what those estimated costs are? Or at what point in time will that then become knowledge to the Premier and the department?

Mr ACKER - These were identified for trending purposes for executive to determine. I have had some conversations with our chief financial officer about, for example, transfer of costs from the THS to Ambulance Tasmania. The outcome of that, of course, is that we are all one health system so it wouldn't actually provide any benefit to transfer costs from the THS to Ambulance Tasmania to cover for our own internal staff that are being used for managing offload delay.

Ms DOW - Premier, now that you know about the cost of ramping at our hospitals, how are you going to better support Ambulance Tasmania? I note that their chief executive's here and he has previously made public statements about the need for more ambulances, of which none were provided for in the Budget.

Mr ROCKLIFF - Well, apart from Sorell.

Ms DOW - Not paramedics. Ambulances. They are different. Vehicles versus people.

Mr ROCKLIFF - Ambulances are also in the Budget. I think we have committed some \$9 million for 30 new vehicles.

Ms DOW - How many of those been delivered to date, though?

Mr ACKER - We have ordered 30 ambulances in this financial year; 24 are delivered and in service; and six will be in service before the end of the month, as planned, on target.

Ms O'CONNOR - On 24 April the ABC reported that Ambulance Tasmania last year concluded a resilience scan of employees to collect confidential anonymous feedback. The scan was conducted by Frontline Mind and participants were asked to share experiences from their work with Ambulance Tasmania. The ABC reported that a Frontline Mind employee emailed Mr Acker with the line:

After a bit of trickery, I have managed to format the full data set as attached and hopefully this works for the purposes required. I think it is important to note that this could be taken out context quite easily so I advise some discretion as to who has access.

Minister, how was the full data set of a confidential and anonymous survey able to be fully reconstituted after 'a bit of trickery' by Frontline Mind?

Mr ROCKLIFF - First, we have done the resilience scan for Ambulance Tasmania and it is an important step in assessing the health and wellbeing of our workforce. I commend the leadership of the chief executive in terms of listening to our employees. I must say there was some challenging data presented. I think shortly we are undertaking another resilience scan

to see if there has been improvement and how further we can support our workforce. Mr Acker, would you like to respond to Ms O'Connor.

Ms O'CONNOR - That was not the question.

Mr ROCKLIFF - I am talking more broadly about the resilience scan.

Ms O'CONNOR - I am talking about how data which contained private information was able to be fully reconstituted after a bit of trickery.

Mr ACKER - The quote that was used by the ABC came from an RTI request. 'A bit of trickery', as Frontline Mind referred to it, was specific to how they were able to convert what is an online database, which is a survey database, into a PDF document so that it could be reviewed by the executive committee. The Frontline Mind survey, the resilience scan, specifically said that the information was to be anonymous and we were not collecting any identifiable information. There are no fields to collect any identifiable information and the confidentiality was assured by maintaining the results within the executive team.

It was also made clear at the time of the survey that the executives would have access to all of the data. The only way that we can solve the issues and the challenges at Ambulance Tasmania is to know what our people think. There was full expectation that what they put in the survey the executive would read. The only people who have had access to the data results are the executives.

Ms O'CONNOR - The public response by Ambulance Tasmania stated that Ambulance Tasmania cannot control where the people include identifying information in the responses they choose to share in the survey and that a confidentiality framework and an agreed Chatham House rule extended to, 'the executive team who are looking at the patterns identified'. How many people saw the full data set and are now operating under this Chatham House rule? And why was it necessary to de-identify the information prior to being shared with Ambulance Tasmania instead of simply redacting identifying information from the responses and sharing the thematics of the content identified?

Mr ACKER - When we worked with Frontline Mind they were very specific that they would not themselves do the thematic work or do any redacting. The way that this resilience scan is delivered - and it is delivered across many other organisations internationally - is that all of the richness needs to be presented so that the feelings of the staff can be clearly articulated. The information was not redacted; it was provided as entered. There were some cases where people's names were used, individuals who specifically put in there, 'My name is so-and-so and here is how I feel'. Those were the names that came out into the survey. There was very few of those. It was only the executive team that had access to the results and it has not been shared beyond that.

Ms O'CONNOR - Can I ask what Ambulance Tasmania and the Government's response has been to the results of the survey? First of all, can I ask if there is an understanding that for some Ambulance Tasmania staff it has been a confronting and intrusive reality that their personal information could identify them? Secondly, what has been Ambulance Tasmania's response and therefore the Government's to the views of Ambulance Tasmania staff that were identified in the survey?

Mr ACKER - Through you, Premier. When the ABC news came out we responded immediately to our staff in an all-staff email and we follow that up with a Teams meeting with all of our staff and we do every month. The message that we shared with them is that your information is confidential and assured, but also a reminder that they did not have to provide identifiable information. It was not requested and only those who provided that did so under their own situation. The survey was completed by 323 staff members. The survey, as I said, has had a significant impact on the organisation in terms of allowing us to understand how our staff feel about the organisation and has given us a road map to do things better.

Mr ROCKLIFF - Can I say, further to your question, Ms O'Connor, in response to the resilience scan, Ambulance Tasmania leadership committed to three immediate actions to commence the journey of cultural improvement for Ambulance Tasmania, including -

1. A program of monthly all-staff forums conducted via Microsoft Teams, to communicate with staff and volunteers and share information from the Ambulance Tasmania executive team.
2. Face-to-face workshops in each region to identify ways the organisation can take ownership and make a positive change in culture, processes and leadership, and
3. Filling vacancies and stabilising the senior leadership team by the end of 2021.

In October 2021 Frontline Mind facilitated four workshops, I'm advised, across the state to provide an opportunity for employees and volunteers to identify issues at Ambulance Tasmania and to seek suggestions from staff as to how these can be addressed. A further 11 consultation sessions were held across the state, facilitated by the chief executive. Employees and volunteers were provided with many opportunities to contribute to the consultation process including online via Microsoft Teams, at face-to-face workshops and sessions and via email to a general inbox. All feedback has been considered by the senior leadership team with actions now being finalised and prioritised to inform Ambulance Tasmania's cultural improvement action plan and a follow-up resilience scan, as I believe I mentioned before, will be conducted shortly after the plan is released in June 2022 to enable organisational cultural improvement to be measured.

Ms DOW - Premier and minister, I find it concerning that you wouldn't have access to this data. Today you've said that it's over 20 000 hours that ambulances have been ramped across Tasmania over the last year. Mr Acker himself is concerned about that, he said that in his media statement, in addition to wanting additional trucks he also wants to see an end to ramping. He's actively collecting data to support the case around that and the impact that's having. We're seeing the impact that it's having not only on the mental health and wellbeing of our paramedics and our volunteers but also on the terrible tragedies where people aren't getting ambulances in the times they need them, and Tasmanians have sadly died waiting for an ambulance this last year. Why don't you know this information? What really are you doing about this and how will you use this information that Mr Acker is collecting to better inform what's happening across the ambulance service and to save Tasmanians' lives?

Mr ROCKLIFF - Thank you, Ms Dow. There is an enormous amount of work that we are doing, investing in Ambulance Tasmania; investing in paramedics; investing in new

vehicles; we're upgrading ambulance stations, as I've mentioned before, and good examples of that are of course Sorell and Huonville; we are investing in statewide access and flow programs; there's been an increase in demand, we accept that. An enormous amount of work has been done to improve the service. We announced our direct transfer policy, of course we announced today, there is a huge amount of, not only investments, but also a system improvement as well.

Our secondary triage program which we commenced I think in February 2021; our PACER response commenced earlier this year and that's seen huge benefits in terms of the 400-plus people who have been seen, and I think that on the statistics, around 74 per cent of people have been cared for in the community that have -

Ms DOW - Are you briefed though, Premier, on this data that we have been speaking about today, are you briefed on that regularly? About the cost to Ambulance Tasmania?

Mr ROCKLIFF - I have outlined some figures for you today -

Ms DOW - So you are briefed regularly?

Mr ROCKLIFF - I have outlined some figures for you on the hours ramped, and I have not seen those figures for some time, but -

Ms DOW - Will you make a commitment to be briefed on data from here?

Mr ROCKLIFF - Of course I will be briefed and discuss policy with respect to improvements we can make in Ambulance Tasmania, we have invested enormous amounts in a very high pressure, increasing demand, system improvements. Absolutely.

Ms MORGAN-WICKS - Reporting in relation to dollars spent on the ramp will not actually inform you as to where further paramedics are required to improve clinical service in particular areas. What is critical is the clinical service planning that is under way which looks at the demand across all of the regions, where ambulance stations need to be located in Tasmania, taking into account the types and the staffing for each ambulance station that we already have, and that information needs to be brought together into a 10- and 20-year plan which is both infrastructure and resources to actually implement that.

So, in terms of a dollar figure which can be calculated off a KPI that is actually monitored in terms of the hours on the ramp, or the number of vehicles ramped, is not going to inform the health executive, for example, of where future infrastructure needs to be built or resources need to be put in play. The chief executive, Joe Acker, informed us of concerns in relation to Huonville and Sorell off the basis of clinical service planning and information, which is why a budget submission was made in relation to those stations as an example.

Ms DOW - Just further with that 10-20-year plan, can you confirm that work is being done now? Why hasn't that been done in the past?

Mr ROCKLIFF - It is under way, because we have committed to a review of service demand and I'm advised this has commenced. Mr Acker can probably speak to that a little bit more if he would like to.

Mr ACKER - We have engaged ORH out of the UK to conduct the 10-year master plan. The reason it has not happened yet is that we had to do a five-year retrospective look at our data. So we had ORH in and that work has already been completed, which was to look at our computer-aided dispatch data and ensure that we had accurate information going forward, so now, they will use that data to do our 10-year go-forward plan, so that is under way and we expect a report by November of this year.

Ms DOW - Will that be publicly released that report or plan?

Mr ROCKLIFF - I would expect so, you know my commitment to releasing reports and the like.

Ms DOW - Further question, Premier, I guess this is probably a question to Mr Acker but I will direct it through you in the first instance. Back in 2019 there was a Senate inquiry undertaken around the mental health and wellbeing of our first responders, and there were 14 recommendations to that. Whilst I acknowledge a number of those are federal responsibilities, there are some things that the State Government could do right now around those recommendations, and I want to understand, from you, what progress has been made against each of those recommendations.

I understand that paramedics have written to you about their thoughts on each of those areas. May we have an understanding of when those things will be implemented, since it has just been ongoing since 2019, we have the issues that Ms O'Connor spoke about before and Ms Woodruff, as well, around the poor mental health and wellbeing outcomes for our paramedics across the state.

Mr ROCKLIFF - Well firstly, I believe we have at least nation-leading PTSD legislation we enacted, which is very important. Since May 2019, Ambulance Tasmania has worked in partnership with DPFEM being the support unit to enable staff access to wellbeing support officers, access to MyPulse, online physical and mental health assessments and allied health coaching support and/or referral to a psychologist as required. The wellbeing support program has been critically acclaimed within Tasmania and across Australia as a leading provider of best-practice programs and services for emergency services employees and volunteers.

In the 2021-22 Budget we committed to additional funding to the wellbeing support program, which increased staff numbers from five in April 2019 to 23 today. This includes nine fulltime wellbeing support officers who provide 24/7 support to police, fire and state emergency services, and ambulance staff and volunteers. In August 2017-18, the Ambulance Tasmania [inaudible 4:56:06] a statewide peer support program with peer support officers were trained by and clinically supervised by psychologists. The peer support officers provide support to their colleagues and volunteers for any issue, providing 24/7 coverage.

A new Ambulance Tasmania wellbeing program officer commenced in May 2022 who will coordinate wellbeing initiatives and develop a mental health and wellbeing strategy. Ambulance Tasmania staff have access to the Department of Health employee assistance program, which is supported by four service providers. Following exposure to traumatic

events identified, Ambulance Tasmania staff are referred to the critical incident stress management program administered by DPFEM. The CISM program is available 24/7.

We can go on to talk about the resilience scan and the emergency services wellbeing leaders' group as well. Anything further to add, Mr Webster?

Mr WEBSTER - Specifically in relation to the Senate inquiry, in relation to recommendation 5, which was about training in mental health awareness, that is now part of Ambulance Tasmania induction training. It is also part of the ongoing peer support program, as the Premier mentioned. And, of course, we are part of the general MyPulse program which is for all first responders, not just Ambulance Tasmania. In terms of recommendation 5, which sits with the state, that work has been done.

Recommendation 6, which is the training for staff, again, it is part of the MyPulse program but, secondly, the appointment of the health and wellbeing officer just recently, with that particular focus, is important to note.

Recommendation 7, all of our volunteers are provided with access to all the mental health supports of employees. Recommendation 12 - have access to early intervention mental health services, that has been fully implemented through the MyPulse and Critical Incident Stress Management as well as the peer support program. Recommendation 14, which is the ongoing support for first responders employed, again, some of those mechanisms are in place. We are still refining them. It is tracking employees after they have left an organisation. We make sure that those programs are known to them but we cannot guarantee that they are made available given they have left our employ.

Ms DOW - Do you have a good uptake to those programs currently?

Mr WEBSTER - To the programs that I have just described, yes, there is a good uptake. The peer support program is particularly well accessed. The MyPulse program, which includes the online health screening and things like that, is reasonably well taken up. The mental awareness training, as I said, is part of induction so it is 100 per cent uptake at that level.

Ms O'CONNOR - New ambulance stations have been desperately needed in a number of regions for years. The data probably shows this but our understanding is that the eastern shore has had a significant uptick in calls, but it is basically only serviced through Mornington and Sorell. Tasmania's largest subdivision is proposed at Droughty Point with more than 2000 dwellings planned. I understand a station site has been mooted for the Police Academy at Rokeby. Is this true? And will you be funding a new ambulance station to service the eastern shore? Will it be on this site or somewhere else?

Mr ROCKLIFF - Mr Acker, would you like to answer Ms O'Connor's question regarding the eastern shore?

Mr ACKER - As we have discussed previously, we are engaging all our [inaudible] to use evidence and data to determine where we build stations. That is the most accurate way to identify where the trends are going, not only in terms of our call volumes but also in the demographics of the population of Tasmania.

At this point in time, beyond the current established or agreed-upon stations, we have not identified that list in terms of priorities. We do not have any stations on the list in terms of priorities beyond what we are currently building.

Ms O'CONNOR - So, there is no planning for an extra eastern shore ambulance station despite the fact that the population is increasing and the number of calls from that area are increasing?

Mr ACKER - That is the purpose of the ORH review, to look at our call volumes, the locations using GPS coordinates, the volumes, the severity of the patients, the scale of the paramedics required and also the demographics. That data will now inform our masterplan in terms of where we are going to build stations going forward and where we are going to staff ambulances and what level of service we will provide to the paramedics on the ambulance.

Ms O'CONNOR - Thank you. For people who are asking these questions, and these come from constituents, what is the time frame on that body of work and when is it likely to lead to the delivery of new ambulance stations?

Mr ACKER - We intend to have the report back from ORH by November this year. Then we will come forward with budget submissions with our top-priority stations in terms of where they have identified we need to invest in infrastructure around the state. That will then depend on funding from the Government in terms of stations, ambulances and staffing for those stations.

Ms O'CONNOR - Final question on this line of questioning: is this likely to lead to the closure, and I was listening to what Ms Morgan-Wicks said earlier, to the closure of some ambulance stations and the opening of new ambulance stations? Are we likely to see a rationalisation in some areas?

Mr ACKER - We have asked for the scope of the whole system, identifying where we need resources and how resources are being utilised right now. That may result in us making an assessment on where vehicles are used. For example, in some places we have volunteer-only services. They may be upgraded or they may be downgraded in terms of places where we have put ambulance stations very close to those communities.

Ms O'CONNOR - So there could be closures of ambulance stations?

Ms MORGAN-WICKS - I did not suggest there was going to be any rationalisation of ambulance stations, but what I -

Ms O'CONNOR - I am sorry if I have misspoken you. I understood that there was a looking at the whole system and where you needed more resources and where you might need less.

Ms MORGAN-WICKS - Yes, and certainly we look at every station. But it is usually around an upgrade of that station so, for example, if we move from a single branch to a double branch station, or from a volunteer to a manned station. The work being undertaken

will consider existing stations to determine whether they are sufficient in terms of resourcing or whether they need an upgrade. It is not to suggest that we would close particular stations.

CHAIR - As it is past 5 p.m. we will now take a short break.

The Committee suspended from 5.04 p.m. to 5.15 p.m.

CHAIR - I call Ms Dow.

Ms DOW - Premier, I understand that Ambulance Tasmania is working closely and positively with the Volunteer Ambulance Association of Tasmania and that they have a constructive memorandum of understanding that they're working together on. Can you could update us on that work; but also on the work that is being done around the transition from volunteer-based stations to permanent paramedics, if you can provide anything further on that?

Mr ROCKLIFF - I had the pleasure of attending a Volunteer Ambulance annual dinner towards the end of last year, I think it was; or this year. It was a very good night in one of the surf clubs across the north-west coast and presented five life memberships as well, which was fantastic.

I acknowledge the hard-working staff we have at Ambulance Tasmania. We've recruited 243 full-time equivalents in terms of additional FTE at Ambulance Tasmania since 2014. I spoke about our increased investments - 48 paramedics in 2021. While our paid paramedics are extremely important, we also know that the volunteer ambulance officers are the lifeblood of so many regional services. Currently, we have 450 volunteer ambulance officers, I'm advised, assisting Ambulance Tasmania with recruitment ongoing, and we sincerely thank them for their support.

Ambulance Tasmania is in the process of recruiting to the position of Manager, Community First Responders and Volunteers, and this position will place increased strategic focus on volunteers and service delivery. There are also three permanent volunteer coordinators across the state whose role is focused on recruitment and retention of volunteers. Our volunteer ambulance officers are provided with the same supports as our full-time paramedics, in terms of access to the health and wellbeing program for emergency service personnel, through the Department of Police, Fire and Emergency Management. We are also working with the Volunteer Ambulance Officers Association of Tasmania on attraction, retention, training and support for volunteer ambulance officers and have committed \$50 000 to support this work. Again, I thank our dedicated volunteer ambulance officers. We launched a video, wasn't it, Joe, at that function I was referring to, about recruitment of volunteers -

Mr ACKER - Yes.

Mr ROCKLIFF - which was excellent. We have committed additional paramedics between Sorell and Huonville stations, based on the advice received from Ambulance Tasmania in recognition of the increased demand we're seeing in these locations. I spoke about that at both those events, where we announced the Budget commitment. The stations are currently supported by a dedicated team of volunteer ambulance officers who will work

alongside our paramedics. These changes are an upgrade of clinical care that's intended to improve patient outcomes, reduce paramedic fatigue and improve paramedic safety. I'm advised that these significant investments were of a positive impact to ambulance availability and reduce response times. I'm advised that the Director of Operations has already met with volunteers from both Huonville and Sorell to hear their feedback about these changes. Consultation with volunteers will continue, and it is my intention that our dedicated volunteer ambulance officers at Sorell and Huonville will continue to have an important ongoing role within their local communities.

Is there anything further we can add, Mr Acker, to that?

Mr ACKER - Specific to Sorell and Huonville, we've had six meetings with the volunteer ambulance officers to date, including our director of operations meeting with the whole board on 21 May up in Campbelltown. There is a further meeting this week with the volunteers in Sorell, to discuss various options for their continued volunteering in the communities.

This is relatively new for us, where we're converting voluntary supported stations to career stations, and where it's not immediately available to move the volunteers to other surrounding communities. So, we're looking for innovative and creative solutions to enable these really committed volunteers to continue to provide services.

We're looking at a range of different options, also investigating some first responder capabilities and first responder options as well. We hope to trial those in Huonville and Sorell, and then continue to consider application across the state.

Ms DOW - I understand there has been a vacant position on the north-west coast for some period of time. We have had concerns raised with us about the need to fill that position, and we understand that it has been managed across, as a northern role at the moment. Could you provide an update on that situation and when you would expect that position to be filled? I understand it's very important, particularly for supporting new paramedics and intern paramedics, and there a large number of those, I understand, currently on the north-west coast.

Mr ROCKLIFF - Thank you, Ms Dow, I will go straight to Mr Acker.

Mr ACKER - We have an Assistant Director of Operations in the north-west, based out of Burnie. It's not a vacancy; there is a substantive person holding that position. That person is on a personal leave of absence, at this particular time. We still don't know the end of that leave of absence, when that individual will be returning to the organisation.

This is a senior level position, reporting to the Director of Operations. We are temporarily solving that strategic gap, by having the Assistant Director of Operations North, Alistair Shephard, covering both regions.

It's substantially the same call volume and number of staff as we have in the south, where we also only have one Assistant Director of Operations. The other thing that we're doing to do to support staff in that community that is our Director of Operations, who is based

in Hobart, spends one week a month up in Burnie. That has been going on for several months now, where he has an opportunity to catch up with leaders there.

This position is a strategic position, this is overseeing the leadership in the region, it's not directly involved in supporting front line staff or volunteers. Indirectly of course, through the leadership; but this is a very strategic position. We've covered that gap until the individual returns by having the Director of Operations there as well as the Assistant Director of Operations covering both areas.

Ms DOW - To Mr Acker again, just curious about this data that you've been collecting that the Premier hasn't been made aware of. Is that data improving, or is it getting worse over time?

Mr ACKER - The data that you refer to is not new data, the Premier is briefed on the data. The data is simply a calculation. As I explained, we're using that data as a trend in terms of providing a proxy. It's simply, \$55 per hour x two paramedics, x 20 000 hours of ramping. It's a very simple calculation that we use for trending purposes, as a proxy for what, perhaps, nursing services could be used in hospitals. The Premier is well briefed on ramping data on a regular basis, he and I have regular conversations; as well as a secretary. This is not new data, in particular, it is just one of many metrics that we review at our executive team.

Ms DOW - Through the Premier, is it trending up or down?

Mr ACKER - Currently it is down from last month, actually March was our highest month ever. It's down slightly, but it is still a significant number of hours.

Ms DOW - My final question to Mr Acker was that we've spoken before about the comments you had on the record about wanting to see more resources in the Budget. I wondered if you were happy about what was in it, or whether you think there could be more investment across Ambulance Tasmania?

Mr ACKER - Through you, Premier, I was very satisfied that, as the Premier said, he listened to my concerns regarding the challenges we were having in Sorell and Huonville, and very quickly came to the rescue by providing us with 11 full time equivalents to solve that problem.

I think that will have a big impact on the community, it will have a big impact on paramedics and a big impact on patients. Of course, as a Chief Executive of an ambulance service with many hard-working individuals that are continuing to work harder than ever, I welcome all future investment into more paramedics, more support staff, more educators, more call takers, more despatchers so that we can continue to provide high quality care for the people of Tasmania.

Ms DOW - One last question, Mr Chair, please. Through you, Premier, you have said that you wanted to see action on ramping or more done around ramping. So, what particular initiatives do you want to see and were they covered in the Budget?

Mr ACKER - Through the Premier, my calls for action and support were to do a number of things and I believe that the Secretary has demonstrated some significant leadership in helping us with this.

First is to identify that the transfer-of-care delays that happen in the hospital are not an ambulance problem or an emergency department problem, they're a whole-of-health-system problem and a whole-health-system challenge and, we have to work from the top down to solve those challenges.

The Secretary has taken this on, specifically by directing our chief medical officer to provide some leadership for the whole of Health so we can solve some problems. It started today by the release of our intra-facility transfer policy that is something new that we haven't had in decades that will allow paramedics to go directly to wards for admitted patients that will reduce the bottleneck in the emergency department.

We are working very closely with my colleagues, the chief executives of the THS on better escalation processes that are statewide, so that when we have transfer-of-care delays that are significant to the ambulance systems, that I can escalate those with the THS and we can develop plans together to reduce some of that bottleneck.

We have a number of initiatives in the pipeline that I believe will make a significant difference. This is an ultra-complex problem. We are not the only system that is dealing with this. Every system in Australia and in fact worldwide is dealing with these challenges. The AMA Report Card recently said that Tasmania is actually doing quite well relative to the rest of the country, in fact better than most of my colleagues in other states but we still have a lot of work to do. I am reassured that my calls for help and my colleagues' calls for help have been heard by the Government and our department and we are doing everything we can to solve the problems.

Mr ROCKLIFF - Just on that data, when results are adjusted to correspond with how other jurisdictions measure performance, Tasmania recorded one of the highest proportions of patients transferred from ambulance to hospital within 30 minutes, that being 79.6 per cent in 30 minutes. If you compare: South Australia is 54.1 per cent in 30 minutes and New South Wales is 4.8 per cent in 30 minutes and notwithstanding the success of our PACER team as well, a very good initiative in which we have got over 73 per cent of people that are seen by PACER that remain in the community in terms of their care. They are therefore avoiding the emergency department which you would appreciate would not be the best place for a person with a serious mental health problem.

Ms CHAIR - Ms O'Connor, finally.

Ms O'CONNOR - Thank you, finally. Minister, I am glad to hear that the data on ramping and wait-times is available. Are you able to provide the committee with the current figures for this financial year for the percentage of patients arriving by ambulance at each of the state's major hospitals who are transferred within 15 minutes of arrival and within 30 minutes of arrival?

Mr ROCKLIFF - So, if you look at statewide and at major hospitals, in 2021 we had 68 per cent -

Ms O'CONNOR - At which hospital?

Mr ROCKLIFF - This is statewide - transferred within 15 minutes. For July 2021-March 2022 it was 64.4 per cent. In 2020-21, transferred within 30 minutes, 75.5 per cent. And July 2021-March 2022, 71.9 per cent. And then if you want to go hospital by hospital -

Ms O'CONNOR - Yes, thanks.

Mr ROCKLIFF - 2020-21 transferred within 15 minutes, 66.2 per cent

Ms O'CONNOR - Which hospital is that?

Mr ROCKLIFF - Sorry, my apologies. The Royal Hobart Hospital.

Ms O'CONNOR - 66, yes.

Mr ROCKLIFF - July 21 to March 22, 58 per cent within 30 minutes; 2020-21, 74.3 per cent; and then in the nine months, July 21 to March 22, 66.6 per cent.

I will now go to the LGH: 2020-2, within 15 minutes, 61.4 per cent; July 21 to March 22, 60.2 per cent transferred within 30 minutes; 2020-21, 68.1 per cent; and July 21 to March 2022, 66.2 per cent.

In the North West Regional Hospital: 2020-21, transferred within 15 minutes, 77.5 per cent; July 21 to March 22, 78.7 per cent; transferred within 30 minutes, 85.7 per cent; and July 21 to March 22, 86.9 per cent. That was the North West Regional Hospital.

I have Mersey Hospital as well, Ms O'Connor. For 2020-21, 85.1 per cent transferred within 15 minutes; July 21 to March 22, 86 per cent; transferred within 30 minutes, 89.8 per cent and July 21 to March 22, 91.3 per cent.

Ms O'CONNOR - Thank you. So, once a patient is subject to offload delay of more than 30 minutes, what is the average amount of time they have to wait? We would like the data for the Royal, the LGH and the North West Regional Hospital, if possible.

Mr ROCKLIFF - All right. We don't have it with us today, Ms O'Connor, but if you'd like to put that question on notice, we'll see if we can find that data and if it is reported somewhere internally and if we are able to access that for you.

Ms O'CONNOR - Thank you. And minister, how many triple zero calls have been referred to Ambulance Tasmania's secondary triage service since it commenced in February 2021? And I am happy to put this on notice, but are you able to provide a monthly breakdown?

Mr ROCKLIFF - Okay. As at 22 May 2022, approximately 2140 patients who had rung triple zero since the commencement of secondary triage on 22 February 2021 were diverted from an emergency ambulance response. Did you want the monthly data as well?

Ms O'CONNOR - Yes.

Mr ROCKLIFF - Which I don't have in front of me.

Ms O'CONNOR - Is that okay to put on notice?

Mr ROCKLIFF - Sorry, Mr Acker, would you like to comment on that?

Mr ACKER - Secondary triage doesn't get referred calls any longer. We started in the initial phases of taking specific triple zero calls and sending them to secondary triage. Now, the secondary triage clinicians have access to all the cases, so they will look at appropriate cases that they can apply their methodology to, to find referrals and apply the referrals where necessary. So, any case that is identified as a low acuity sits in a queue and the secondary triage clinicians will look at the queue and take cases out of it that can be appropriately referred. We don't specifically take triple zero calls and send them to secondary triage any longer because we [indistinct].

Ms O'CONNOR - We all have heard the terrible story of the man who called an ambulance, and waited seven hours and died alone. What is the secondary triage process? How does a tragedy like that happen? This is no reflection on Ambulance Tasmania staff. How does a tragedy like that happen, and what sort of investigation has there been into that tragic situation, and is it possible that through that secondary triage process some people are being misdiagnosed in terms of their urgency for help?

Mr ROCKLIFF - I agree with you on the tragedy, Ms O'Connor, and there is a root cause analysis which is currently underway. I am not sure if Mr Acker can provide any further information to that, at this particular point in time?

Mr ACKER - We do have serious cases that require investigations; this is one of them. Any case that results in a poor outcome from a delayed ambulance respond we take very seriously and we look into it. We set up a panel to review that case with an external expert from New South Wales who is joining the panel to identify opportunities for us to do better. The cases like this are not a result of secondary triage. Cases like this could be a result of some challenges to our primary triage and that is the initial 000 call and how it is initially triaged - whether it's high acuity requiring an ambulance immediately or whether it's lower acuity. Those are the cases where secondary triage would pick them up.

Secondary triage is essentially a very safe system in that they try to find referral pathways, and if those safe referral pathways are not available then those patients will always get an ambulance at some point in time. Secondary triage is not likely a failure in this particular case, but we are investigating our primary triage system to identify where we can make improvements.

Ms O'CONNOR - Can I ask, finally, when that investigation into that man's death will be completed, and will there be a public announcement about it or an explanation?

Mr ACKER - Importantly, our Director of Operations has done an open disclosure with that family to express our condolences and to also commit to following up with them to

let them know what happened in that particular case. As I mentioned, we do have an external person that is joining our panel and we hope that commences very shortly. Once the results of that are available to be released we will be using those internally, of course, in terms of making our process improvements.

Ms O'CONNOR - You would foresee it will be available this year?

Mr ACKER - Yes, I would hope that it is done this year. We have a number of cases to investigate and a very small team investigating cases. This is a priority case for us, because there is a system issue that we need to understand better. We will work this as quickly as possible - recognising that doing a complete and thorough process is incredibly important.

Mr ROCKLIFF - Ms O'Connor, there is the coroner's review as well and we can't interfere in that process.

Mr WEBSTER - Yes, it is subject to a coroner's inquiry so that may delay the release of our information because we do need to provide it to the coroner, and the coroner may not wish us to release it. A note of caution on that. Your first question around secondary triage and would it delay a case - in fact, there is a percentage of cases that go to secondary triage that result in an upgrading in acuity level and results in a 000 ambulance responding, so it actually works the other way.

Ms DOW - Premier, can you provide detail of the number of times taxis have been used to transport patients to hospital, instead of ambulances, in the last 12 months, broken down by month, region and level of urgency?

Mr ROCKLIFF - I will see if I can access some information for you. When it comes to secondary triage, taxi use data and information, Ambulance Tasmania started using contracted taxi services for appropriately assessed secondary triaged patients in mid-October last year. In the nine months there have been 285 incidents, 1 June 2022 with an average of nine cases per week. Taxi usage has been highest on Saturdays, with higher use in the afternoon. The highest usage by hour of days between 11 a.m. and 9 p.m.; and there is a relatively even distribution by age group and gender.

Use of taxis can only be authorised by secondary triage clinicians as part of a low acuity referral pathway, and this is specifically for patients who have been clinically assessed as not needing an emergency ambulance response and have no other way of attending their general practice appointment, pharmacy or hospital department.

A 35-year old male with foot swelling calling on a Friday night, a 22-year old female with a laceration wound calling on a Saturday afternoon are a couple of examples.

Ms DOW - A further question about infrastructure. I wanted to understand whether there has been any community consultation about the new Glenorchy Ambulance Station site that has been selected.

Mr ROCKLIFF - On Timsbury Road. Perhaps I could ask Shane Gregory to come to the table. Shane is Deputy Secretary Infrastructure.

Mr GREGORY - With regard to consultation, there are two components. There was consultation with the land owner that we are acquiring the land from through an acquisition process. We first made contact with the land owner, talked to them about what we were interested in doing; they indicated that they were amenable to a sale, so we proceeded with that process. Now that the land has been acquired by the department, we are starting consultation with adjoining property owners and the broader community. That's a process we have started just recently.

Ms DOW - When will that be concluded?

Mr GREGORY - I would need to check the exact date, but we have written out to a number people seeking their input.

Ms DOW - Chair, I wanted to take you now, Premier, to the LGH where, incidentally, we have the worst bed block in the country. I want to talk to you about the signature policy that you had at the last state election about health, and that was the LGH Master Plan. We have been asking for some time now why there is no money in the Budget for that plan, and there is still only a very small amount - I think it is about \$50 million - that is in there across the forward Estimates. When will this project be fully funded by your Government?

Mr ROCKLIFF - Major refurbishments and expansions to the hospital's facilities are complex, and careful staging is required to create more space for development while taking care to minimise disruption to ongoing service delivery. Clearly, all of this takes careful planning and it takes time. There has been a lot of community consultation with respect to this matter, including the development of the master plan.

We know that we have some space constraints at the LGH main campus as well. That's why we have developed a carefully staged plan to decant and demolish existing buildings on the Anna Burn site before we can build the new Mental Health Services precinct. It is only once this building is operational and Mental Health Inpatient Services move to the new mental health precinct that we can demolish the existing Northside building and begin construction of the new tower. To do this safely and to minimise disruption to services, it simply cannot be done any quicker than planned.

Our health infrastructure team has developed a sensible and achievable plan for redeveloping the LGH precinct to ensure that we are able to meet the health needs of Tasmanians living in the north of the state for generations to come. Over time, I have spoken about this redevelopment. We have made a commitment to deliver \$580 million over ten years, and that is what we will do.

As part of our ten-year redevelopment program, the 2022-23 Budget contains \$50 million for stage two projects, and \$38 million to complete stage one projects. Further funding will be allocated in future Budgets to this important project. As we move to future stages of the redevelopment, subsequent Budgets will continue to show funding over successive years.

Stage two of the LGH redevelopment has begun, and the mental health precinct will be a key development. We have committed \$80 million beyond the forward Estimates. The 2021-22 Budget includes funding to start the planning and groundworks for the new mental

health precinct, which is due for completion in 2027. As I have mentioned, it is a major redevelopment over 10 years, at \$580 million. A lot of work is being done to develop the master plan, including a lot of consultation.

Mr GREGORY - The staging is critically important. At the moment, we have a number of projects underway at the LGH. We have got the fitting out of levels 3 and 5 of the women's and children's services tower, we have got the fitting out of 39 Franklin St. That is a critical piece of work to allow us to decant staff and services out of the Anne O'Byrne Building. We are in the process of setting up for the relocation of a supply warehouse off the Anne O'Byrne site. That will then allow us to do all the preparatory work, so there will be demolition of buildings, an early works package to establish all of the necessary power infrastructure and so on, to allow us to move into the construction phase of the new mental health precinct in the second quarter of 2023-24.

Ms DOW - Premier, when can Tasmanians expect to see a collocated private hospital in Launceston? I know you have signed an MOU. There are a number of services not currently being provided in the north of the state and many are suffering, particularly around pain management clinics, which is part of that proposal. When can we expect to see these services being provided in the north of the state as part of that collocation project?

Mr ROCKLIFF - I think pain management is part of the clinical services planning exercise. The memorandum of understanding between the Crown and Cavalry was finalised on 29 April. The signing of the land sale contract for 52 Franklin St, Launceston and a project delivery agreement.

Collocating the Cavalry Private Hospital with the LGH will maximise the benefits of the LGH master plan by supporting delivery of improved health services and to help attract and retain health professionals, including specialists, into the Launceston community.

The new collocated hospital will deliver additional and complementary services to support and reduce pressure on the LGH, with 128 in-patient beds, 10 operating theatres, as well as specialised facilities for oncology and two cardiac catheterisation labs. Services will include general medical and surgical services, as well as gastro-intestinal, cardiology, oncology, mental health, gynaecology, radiology, pathology, palliative care and an extended-hours GP clinic.

Cavalry will now work towards meeting a number of conditions to land transfer, including obtaining all the required planning and other approvals that will enable it to deliver the private hospital facility under the commitments it has given to the state, and confirmation that it has available financing to complete the design and construction of the project.

Another condition to land transfer is the development of a detailed services agreement between the Department of Health and Cavalry to address issues such as patient transfer protocols, private hospital admission hours and purchase of services.

Key elements of the service agreement include extended admission hours, which have already been agreed in the PDA. The Government agreed in 2021 to fund the construction of the link bridge between the two facilities to enable patient transfer, at an estimated cost of \$3 million. This cost will be offset against the proceeds from the sale of the land to Cavalry. All

other construction costs are being met by Cavalry. Subject to obtaining planning approvals and normal construction time-line risks, Cavalry is expecting to have the facility operational in early 2026, I am advised.

Mrs ALEXANDER - May I please ask the Premier and minister to update the committee on the state's neonatal survival rates? I understand there have been some positive changes in that respect.

Mr ROCKLIFF - Thank you. It is a very important question. When we think of hospital care, it's hard to imagine a more vulnerable group of patients than babies who are born prematurely. That's why I was very pleased to receive an update on the outstanding success of the Royal Hobart Hospital's Neonatal Intensive Care Unit in dramatically increasing the survival rate for premature babies. Since 2003, when survival rates did not meet the national average, the Royal Hobart Hospital NICU has progressively improved neonatal survival rates so that survival rates at every gestational age are now on par with, or ahead of the national average. These improvements in neonatal survival rates have been achieved through investment in state-of-the-art facilities, contemporary equipment and a strong culture of innovation.

In fact, under the leadership of Professor Peter Dargaville, the Royal Hobart Hospital NICU has developed the Hobart Method as a less invasive method to deliver lipid protein solution to treat respiratory distress in pre-term infants. This avoids the need to insert a breathing tube, which reduces the risk of lung damage and pneumonia. I am pleased to report that the Hobart Method is now widely used both in Australia and internationally.

Dr WOODRUFF - Great.

Mr ROCKLIFF - It is great. Another impressive innovation has been achieved by our own Prof Dargaville in collaboration with Dr Tim Gale from the University of Tasmania's engineering department. Together they have invented a more sensitive system to continuously control oxygen concentration for pre-term infants. Clinical studies at the Royal Hobart Hospital NICU have confirmed that the new technology can improve pre-term babies' oxygen levels by 25 per cent compared with standard measures.

Following the medical advances pioneered here in Hobart, the focus is now on creating devices that will be financially accessible in low and middle-income countries.

Although I appreciate it can be a challenging time for families with a premature baby in the NICU, and I can relate to that, having a little daughter born in 2008 very small indeed, under three pounds from memory, these innovations coupled with the dedication of our amazing NICU staff mean that Tasmanian families can be confident that if their baby is born early, they will have access to world-class care and facilities. I harp back to 2008. The staff were absolutely and utterly wonderful during what was a very distressing time.

Dr WOODRUFF - Minister, an important measure of the performance of an emergency department is how many patients are being seen on time. You'd know when a patient arrives at the emergency department they're allocated a triage category based on their condition. Each category has a clinically recommended time frame for treatment and a national benchmark target for how many patients it's acceptable to see outside that time

frame. Those targets are important for all the categories but they're particularly critical for category one patients, who are in an immediately life-threatening condition.

The standard set for category one is common sense. Every single patient in this category has to be seen and treated immediately when they arrive at an emergency department. Was that the case in Tasmania during the year of 2020-21 across each of our emergency departments?

Mr ROCKLIFF - We'll see if we can provide that information for you.

More broadly, in recent years our emergency departments in Tasmanian hospitals have experienced increasing pressure due to rising numbers of people presenting for care, especially at the Royal Hobart Hospital. This is caused by several issues, including a growing and ageing population, relatively high rates of chronic disease and an increase in patients with mental health issues presenting to EDs, although hopefully our PACER initiative will support some improvement in that.

In the first 10 months of 2021-22, there were 144 783 emergency department presentations statewide. If this trend continues, it is expected that in the full 2021-22 financial year, we'll have a similar total number of presentations to last year. We have seen substantial staffing increases in our emergency departments with a statewide increase in staffing of 4.4 per cent which amounts to some 24 full-time-equivalent staff over the first nine months of 2021-22 and this follows sizeable increases in recent years. In regard to COVID-19 of course extensive planning and investment in hospital capacity along with the extraordinary dedication and resilience of our health staff to ensure that our health system was able to manage the surge-cases in Tasmania.

Also, I want to acknowledge the COVID@home program, which help us to -

Dr WOODRUFF - Sorry, minister, I am a bit confused, because I am not talking about any of this. I am asking about Category 1 patients, and was every single Category 1 patient seen on time at an emergency department in 2020-21? Straight question, I can put it on notice if you want.

Mr ROCKLIFF - We can provide that information for you, and while we are just finding information I just want to talk about our initiative to prepare our health system to manage demand. I mentioned this deliberate establishment of statewide access and flow program, strengthening collaboration with private hospitals, convening a health recruitment taskforce to review and improve recruitment across the public health system.

And so, if I look at the percentage of patients in each triage category - patients seen on time - on our dashboard we have Category 1 at 100 per cent for the last 13 months.

Dr WOODRUFF - Okay, is that right? The annual report for the department says that of triaged Category 1 patients - 98 per cent were seen immediately at the Mersey, 98.3 per cent of the time at the North West Regional, 99.6 per cent of the time at LGH and 99.9 per cent of the time at the Royal. Can you give me the total number of patients who presented with a life threatening condition who were not seen and treated immediately? And why is there a discrepancy between the annual report and the dashboard figures?

Mr ROCKLIFF - These figures are April 2021-April 2022. Category 1 - 100 per cent, I'm reading from that now.

Dr WOODRUFF - I asked for 2020-21, the whole year, not just one month.

Mr ROCKLIFF - We have the last quarter of 2021, so I'll get that information for you. We are now publishing this on a monthly basis.

Dr WOODRUFF - So, is there rounding happening on the dashboard? Because you can't round a number like being seen immediately because they are real people, real life-threatening situations.

Ms MORGAN-WICKS - We are just referring to the public health dashboard which is published monthly which has the last 13 months. So, it is reporting at the moment from April 2021 through to April 2022. We just happen to drop off the last month as and when we report each new month. We will certainly have that data. I don't understand that it is a matter of rounding. I misinterpreted when you said for the year 2020-2021 and I went to latest one. So, my apologies.

Dr WOODRUFF - Thank you. I will put that in writing.

Minister, back to the secondary triage. Do have an estimate of how many of Ambulance Tasmania's secondary triage calls resulted in a diversion to an alternative service provider? And how does that figure compare to the target for the first year of the services operation?

Mr ROCKLIFF - I think I have that information at hand before. I believe that I did answer that. Excuse me if I hadn't but the scope of patients referred to secondary triage has expanded since its commencement and will expand further over time as more alternate medical care pathways are established.

Secondary triage commenced on 22 February 2021 and at May 2022 approximately 2140 Triple Zero calls had been successfully diverted from an emergency ambulance response. Of these cases approximately 50 per cent were in the south and 25 per cent in both the north and the north west. The implementation of the secondary triage service formed part of the 2018 commitment of the Government to invest \$125 million into ambulance services.

The Police, Ambulance and Clinician Early Response, otherwise known as PACER team, is dispatched to mental health patients in the community experiencing crisis following by a secondary triage clinician. This service delivery model has assisted many mental health patients to get the care they need without needing to present to an ED and I have spoken on that a number of times. Is that the information you wanted around secondary triage?

Dr WOODRUFF - Yes, how many diversions have there been? Mr Webster said last year in Budget Estimates that the secondary triage goal was to reduce ambulance callout by 35 per cent or in real terms, by 16 000 calls a year. We are only halfway through the second year of this service, and understanding that, when do you expect that we will be able to reach

those levels of 35 per cent diversion, 16 000 calls, and do you have intermediate targets? How are we tracking, basically?

Mr ROCKLIFF - Thanks, Dr Woodruff.

Mr WEBSTER - In referring to the 35 per cent, I was referring to the Victorian outcome and the full suite and so we are building our suite and it will take some years to do that. Switching on mental health was an important step this year. So far, we have had 2140 Triple Zero calls diverted in the 15 months. PACER is now running at about 30 diversions per day. Importantly, the majority of those, around 74 per cent actually, result in avoiding hospital for that cohort, which is the operative way to do that. So, as we build the cards, as Ambulance Tasmania call them, which are actually the conditions that are written up into cards, we would hope that eventually we will get close to what Victoria are doing. They have been going now for a number of years and have a very aggressive approach to it. We need to build our confidence in the system and also build the number of cards, as I said.

The major step for 2022 was building in PACER and as the Premier announced recently, the extension of that to the north west as a mental health emergency response - it won't exactly be PACER - is a really important step because that will grow our capacity and we grow that over the period as well. I would hate to put a time line on it, it's just that, use it as much as we can and build our confidence in it.

Ms DOW - Premier, can you confirm that the present Director of Monitoring, Reporting and Analysis of the Department of Health was directly appointed to the position, replacing the acting incumbent, and can you explain the nature of the direct appointment and why the usual recruitment process wasn't used to fill this role?

Mr ROCKLIFF - Thank you for your question. I will refer to Ms Morgan-Wicks.

Ms MORGAN-WICKS - Through the minister, the current incumbent in that position is acting in the position. The previous person in the position had also acted and was at their limit of six months and the current incumbent has been appointed for a six-month acting position, at which time the position will be advertised.

Ms DOW - When does that become vacant?

Ms MORGAN-WICKS - Through the Premier, my understanding is in the next few weeks.

Ms DOW - Premier, I want to just take you to the west coast and to a commitment that was made three years ago at the federal election of \$1 million for the West Coast Hospital and that money was meant to be provided for an addition of five aged-care beds for the west coast community.

Mr ROCKLIFF - That's still better.

Ms DOW - Yes, but it was to be delivered by the state Government. The money was given to the state Government to deliver, is my understanding.

Why weren't those five beds delivered? When will they be? What has that million dollars been allocated to? Consequent to that commitment made by the federal Liberals at the last election, I think around three additional beds, can you just provide some clarity? Give some confidence to the people of the west coast that this commitment will be delivered on and that the eight people that are waiting on the list down there to go into the lower wing will get access to an aged care bed on the west coast sometime soon.

Mr ROCKLIFF - Thank you for the question. I acknowledge the importance of being able to allow people to remain in their local communities, if possible, where many have lived all their lives.

Yes, the federal government provided funding of \$100 million to allow for upgrades to the West Coast District Hospital in Queenstown. Since that time, the Department of Health has undertaken a site review and consulted with the local service managers and the General Manager of the West Coast Council to plan how to allocate this funding.

The site review considered the intent of a community health and hospitals program initiative, the available budget, as well as current and future needs and challenges. An emergency department redesign and building of an additional aged care capacity were identified as priorities. The agreed scope of works, as accepted by the Australian Government in September 2021, comprises building one additional aged care room by infilling an existing courtyard to the north of the facility.

This will increase the available aged care rooms at the West Coast District Hospital from 16 to 17. Redesigning the emergency department to install a flexible-use, negative pressure room, addressing infection control and COVID-19 preparedness issues, and improving privacy issues associated with the current facility configuration, enhanced telehealth facilities to improve access to allied health and specialist services for all patients presenting to the hospital are also included in the scope.

Through this planning, initial design work has been completed for additional capacity, called for by the West Coast Council. We welcomed the announcement during the recent election campaign. Of course, our government will now support the council lobbying the new federal government to match the funding offered by the coalition to support the west coast community with additional aged care beds.

Ms DOW - Premier, can you clarify: in the money that you committed to during the state election in 2021, there was an amount that was allocated to the West Coast District Hospital. Is that simply the same money or additional money? I thought that was for the same things that you have just updated the committee on. Was that simply a reannouncement of the federal commitment?

Ms MORGAN-WICKS - Through the minister, we include within our budget chapter, as Treasury does also in terms of state revenue, Commonwealth contributions to revenue, and we would include capital projects that we are undertaking in relation to those Commonwealth contributions.

Mr ROCKLIFF - Are you thinking about the funding for equipment in our regional hospitals? Is that where you getting that from?

Ms DOW - It may have been. I am just trying to understand. There has been a lot of money committed but there does not seem to be a lot delivered to the people of the west coast over the last three years.

Mr ROCKLIFF - Have you anything further to add on this, Shane? I know it is really a big issue for the council. I have spoken to the mayor about this on a few occasions. I've had correspondence as well.

Mr GREGORY - Through the minister, the design work is essentially complete for the works of the West Coast District Hospital. We are expecting to go to tender in July, and then subject to a successful tender process and obtaining a builder, the plan is to start construction in September and have that work completed in March of 2023.

Ms DOW - So, there is no further update then on when the additional aged care beds will be provided?

Mr ROCKLIFF - No.

Ms DOW - There will be one, with the closed in courtyard but they originally wanted five which is what the federal commitment was, minister.

Mr ROCKLIFF - At that time, \$1 million.

Ms DOW - For five aged care beds.

Mr ROCKLIFF - I am happy to work with the new health minister. I have reached out already to Mark Butler to congratulate him on his announcement. I invited him to Tasmania. He could do a round of some of the health facilities that need -

Ms DOW - Did you take Scott Morrison to Queenstown to show him the aged care beds that he committed to fund?

Mr ROCKLIFF - I wasn't with Mr Morrison on the west coast.

Ms DOW - No, that is interesting, isn't it?

Mr ROCKLIFF - Do you think?

Ms DOW - Lucky you, you dodged one.

Mr ROCKLIFF - I will have a lot of interaction with the new health minister. I would welcome his commitment for the west coast.

Dr WOODRUFF - Minister, I have a story from a Franklin constituent whose name is Stephen. He has given me permission to use his name and discuss his case. He was put on the public waiting list for ear, nose and throat elective surgery as a category two and he was told that he would be seen within three months. After three years and three booked and

cancelled surgery dates, he finally received surgery over 1000 days after being put on the waiting list.

After two follow-up appointments at around 12 weeks, post-op, it was discovered that the biopsies and samples taken during the surgery had never been tested so no cause was available for Stephen's clinician to understand why he now had returned and had very severe symptoms. Stephen is back on the waiting list for surgery as a category three case and has been told to expect a 12-month wait. His breathing issues are now so severe, he has difficulty with simple tasks around the home and is scared every day.

Minister, the category one to be seen in 30 days' time average overdue days are 93.4; category two to be seen in 90 days are 235 days. Can you explain what work is being done to address those particular category one and two wait times and the significant blow-outs and multiple cancellations that people like Stephen have been experiencing?

Mr ROCKLIFF - Thank you, Dr Woodruff. You have raised a matter regarding your constituent. I am very mindful that the data that is presented has a face behind each and every bit of it. I am pleased to see some improvement in key elective surgery performance indicators, albeit there is more to go.

We performed more elective surgeries in the first 10 months of 2021-22 than we did in the first 10 months of 2020-21, a rise of some 11 per cent. There was more elective surgery performed over these 10 months than the first 10 months of any financial year since current record keeping began in and around 2010-11, I am advised.

Over the past year, we have seen elective surgery waitlists reduced and, most importantly, there are now fewer people on the waitlist who are waiting longer than clinically recommended. At the end of April 2021-22, the elective surgery waiting list had reduced by more than 2650 patients from the peak we saw in January 2021.

Dr WOODRUFF - Minister, may I clarify? Are you talking about category one and two patients which is what I am talking about?

Mr ROCKLIFF - I am talking about all.

Dr WOODRUFF - So you are including category three.

Mr ROCKLIFF - It includes the category one and category two following the impact of the first wave of the COVID-19 pandemic, a reduction of some 22 per cent. This is also a reduction of 1739 patients compared to the end of 2019-20 or 15.3 per cent.

I am also pleased to let you know that over the 10-month period from the start of July 2021 to the end of April 2022, the number of over-boundary patients on the waitlist decreased from 5616 to 4790, a reduction of 826 patients or 14.7 per cent, compared to the end of 2019-20. This is a reduction of some 1449 patients, or 23.2 per cent. I recognise that while those figures are encouraging, there is a lot more work we need to do. Those still waiting outside the clinically recommended time frames, I hope it provides a little bit of comfort that it is moving in the right direction. But I appreciate that more needs to be done well and truly, particularly when it comes to people having their elective surgery within the clinically

recommended time frames. I am very pleased that the work we have been doing with clinicians has resulted in a four-year elective surgery plan. We are expecting that to continue, given the investment we have committed to in the forward Estimates.

I have some figures regarding the number of over-boundary patients per hospital. The Royal Hobart Hospital, in 2020-21, was 2669 and it is now 2098. That is to April, those figures. The LGH, 1998 in 2020-21 and in April it is 1911. The North West Regional Hospital, 737, April 2022, 695. The Mersey Community Hospital for the financial year of 2020-21, 212, and so far this year, in April 2022, the Mersey Community Hospital had 86. Those figures add up to 5616 and 4790 respectively between 2020-21 as about [TBC] and that is for April. It is heading in the right direction, albeit there is more to go.

I do have an updated waiting list number for this week, which is down. If you look at June 2021, we were 11 153 and we are now at -

Dr WOODRUFF - What waiting list are you talking about?

Mr ROCKLIFF - That is the elective surgery wait list. In June 2021, 11 153. It is now 9 414. I think that is where that 1739 figure came from that I just mentioned.

Dr WOODRUFF - The latest health dash board update shows 1157 category 1 patients in the elective surgery waiting list. Category 1, for people who are listening, are for urgent patients. We have gone back over the records of the health dashboard and previous data sources and we cannot find a time that the category 1 waiting list has been higher. How do you explain this? It is obvious that things are not improving for elective surgeries for people in the urgent category, as we understand it.

Mr ROCKLIFF - Are you talking about category 1?

Dr WOODRUFF - Category 1, urgent patients.

Mr ROCKLIFF - On the dashboard 1157, category 1. March 2022, 1145.

Dr WOODRUFF - As far we can see that is the highest it has ever been.

Mr ROCKLIFF - The figures that I have seen in front of me is April 2021, which is 1000. Yes, it is the highest in 13 months.

Dr WOODRUFF - Something that clinicians warned early on when the Government announced the plan for elective surgeries was that the Government would inevitably focus attention on the cheaper and easier surgeries and not on the patients who need the help the most. This health dashboard seems to identify that while the number of urgent patients waiting has gone up, the number of non-urgent patients has dropped by 25 per cent, or around 1500 patients. We looked at all the previous health dashboards, it's not just the last year, and the elective surgery category 1 is the highest it has ever been. So, isn't it the case that the elective surgery plan is more about a political solution than a genuine solution for the people who are in the highest risk category?

Mr ROCKLIFF - No, that is not true.

Dr WOODRUFF - What is the evidence that it is improving for people who are most urgent?

Mr ROCKLIFF - The evidence is there in terms of all the categories.

Dr WOODRUFF - That's the point my question makes.

Mr ROCKLIFF - I have to disagree with you, with respect to that matter. This has been a clinician-led, patient-focused plan.

Dr WOODRUFF - When are the category 1's going to go down?

Mr ROCKLIFF - I accept we need to have further improvements but the average overdue days waiting statewide for category 1 is coming down. In April 2021, with 107 average days waiting; it is now down to 86 days. The lowest it has been is 83 and the highest it has been is 108, back in May 2021.

Dr WOODRUFF - It is still the highest it has ever been, so we're not having cut-through on the urgent category.

Mr ROCKLIFF - I am advised that clinicians determine the surgeries that are allocated.

Ms MORGAN-WICKS - Yes. The four-year elective surgery plan was developed and approved by the statewide surgical and perioperative committee, which is currently chaired by Marcus Skinner of the Royal Hobart Hospital. In terms of the allocation of surgeries, that is a matter for the surgical and perioperative departments within the hospitals.

Mr ROCKLIFF - I've met with them twice -

Dr WOODRUFF - They are always going to give recommendations within the frame of what they're capable of recommending, aren't they?

Ms MORGAN-WICKS - In terms of the very hardworking teams that are working in the surgical departments, they are making very complex decisions on a daily basis as to patients that are available for surgeries.

Dr WOODRUFF - I am sure they are.

Ms MORGAN-WICKS - I do need to speak in favour of them.

Dr WOODRUFF - I wasn't speaking against them. I was only making the point that they can only work within the framework of what they've got. It's a resourcing, capacity and flow issue which is out of their hands.

Ms MORGAN-WICKS - They have received a significant injection under the four-year plan to uplift resources and to open more operating theatres, in which additional surgery is being undertaken this year.

Mr ROCKLIFF - That is all aimed at getting that waiting list down to that sustainable level of which I believe I have said publicly before, according to clinicians and the clinical statewide surgical and perioperative committee, is that's around 5400 at that sustainable level.

Mrs ALEXANDER - There has been a significant announcement in the Budget about digital health. This is a welcome initiative which I know that a number of peak patient and health consumers have been advocating for a while across Australia. It is a significant announcement. Could you please update and outline for the committee what else is being achieved through digital technology across our health system?

Mr ROCKLIFF - I have the Health ICT end of year financial year highlight report 2021-22, which I will also table for the committee's benefit. We recognise the role that digital technologies can play in improving patient care, providing better data on patient management and reducing a reliance on manual processes. We recognise that, and that's why we're implementing our 10-year Digital Health Strategy. The Department of Health has a dedicated team of ICT professionals who already provide great innovations for our health system. They are also responsible for managing our digital environment. This includes supporting 50 000 wi-fi devices across the state; security cameras; electronic door locks; infusion pumps; x-ray machines; medication dispensing; and even fridges. Our team also sustains critical patient administration systems and millions of patient records, which include some 72.6 million radiology images.

Today, I will table the Health ICT Highlights Report, produced by our Chief Information Officer, Mr Warren Prentice. The report details the key deliverables our health ICT team have been working on over the last 12 months, and I would like to share a few of those with you now, including: provision of remote monitoring to 3400 seasonal workers; repatriated Australians in Antarctic expeditioners through the Hotel Quarantine Program; the rollout of the COVID@home Program, which has been utilised by over 16 000 patients providing support over 43 800 patient care days; implementation of a contemporary surgical instrument-tracking system at the Royal Hobart Hospital; development and publishing of anew website for the Department of Health which focusses on the needs of Tasmanians as well as making health information more relevant and less confusing.

I do thank our hard-working team at Health ICT for the work they have done over the last 12 months. The report tabled today demonstrates the important work that our Health ICT team undertakes in supporting a digitally-connected health system. We look forward to that investment of the \$150 million rolling through over four years. Thank you very much for the question, appreciate that.

Ms DOW - Chair, I want to go back to elective surgery and ask a question, Premier, about the public-private partnerships that are in this Budget and are a continuation from last year. There is less money in this year's Budget than last year's Budget. Just an explanation for that and why that's not continuing in the out-years, or is there a view to do that, and has it been successful to date?

Mr ROCKLIFF - Yes, it has been successful. I recognise it did decrease, yes -

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Ms MORGAN-WICKS - In terms of the announcement of the private-public partnership funding, which I believe was \$20 million in last year's Budget. The Budget was handed down later in the year last year, and in terms of timing of negotiating those partnerships with the private hospitals. That money has been carried over into this financial year. We made a determination to keep those contracts going and that we didn't require the full \$20 million in relation to that. So, we had sufficient funding being rolled over into this next financial year.

Ms DOW - So, is there a view to continue that again in the future, or you'll wait to the outcome of this coming financial year?

Ms MORGAN-WICKS - We've got some significant agreements that are actually being funded by that amount of money. In particular, we've got some 41 private hospital beds that have been brought into play to assist with demand. That's quite significant in terms of an agreement. I don't think we've probably reached that level of private beds being made available for our public sector in past years. Certainly, I haven't been informed that we have. From our perspective of access and flow, if we can get a transfer of a patient from a public sector bed into private - if it's a particular type of acuity, we need to match it to the particular hospital involved - that then allows patients to be admitted up through from our emergency department. It is assisting in terms of that flow.

Ms DOW - I had a couple of instances with constituents that have been charged transport costs after having been admitted to private hospitals, because of these overflow issues. One of them you kindly reimbursed; but the other one I still haven't received a response from you about and it was a large amount of money that this person then had to pay to be transferred from Hobart to the North West Coast. Ordinarily, they had been treated as a public patient, which is what they had thought would have been intended for their care. That changed quite suddenly given the circumstances and they had the procedure undertaken at a private hospital. They would never have had to pay that exorbitant amount of money. It was something that has been brought to my attention on a number of instances now, and I wonder what sort of processes are put in place to make sure that people aren't charged huge amounts of money for something that they had no control over.

Mr ROCKLIFF - Thanks for your advocacy. I am pleased that one of these matters has worked through positively. We will follow up on the other one.

Ms MORGAN-WICKS - If there is an unintended consequence of that transitional transfer to a private bed, I am very happy to look at that case. I am not aware of it, but happy to have that referred to me.

Ms DOW - I have written to you about that, and I know that the person's family has as well.

Mr ROCKLIFF - I am advised that our office has been in contact with the particular family and is following up that now.

Ms DOW - I want to go back to our COVID-19 response, and on page 98, in relation to the National Health Reform Agreement, some of the shared costs that we have had with the Federal Government with that 50/50 basis, on the costs associated with COVID-19. That

agreement will be coming to an end on 30 September and I wanted to ask, Premier, if you were concerned about that and what impact that will have on the state Budget, given that we will then have to fund those initiatives in full out of the state Budget. Which brings me back to my question about the Treasurer's reserve and whether that will be adequate in the response to COVID-19.

Mr ROCKLIFF - All states and territories are experiencing increased demand and challenges with managing patient flow exacerbated by COVID-19. I suggest the current hospital funding model needs to be looked at more clearly. I have spoken to Premiers across Australia and Chief Ministers and we are united in the view of that advocacy for a more equitable funding arrangement, including 50/50 split of costs. It is my view that engaging with the states and territories to renegotiate the National Health Reform Agreement should be a critical first priority for the newly formed Australian Government, following the federal election. Of course, I will continue to support collaborative and continuing discussions between the Australian Government and my state and territory colleagues about the current funding model, with the aim to identify funding improvements that can support future health system capacity. I look forward to those discussions taking place in terms of COVID-19 specifically and it will be actively discussed at the first ministers ministerial meeting.

Ms DOW - Can you provide me with a breakdown of the number of people that have been provided care through the COVID@home Program, but for influenza rather than COVID-19?

Mr ROCKLIFF - I think I had 15 000 for COVID-19 more broadly covered at home. COVID@home Plus starts this week; is that correct, Dale?

Mr WEBSTER - One or two may have come across because of referrals et cetera. But, the actual switch on of the referral pathway with GPs and the EDs commences later this week to coincide with what we see as the usual start of the winter flu season in Tasmania. At this stage there isn't a direct referral pathway for influenza, but that will commence later this week.

Ms DOW - Further to that question I wanted to ask you, Premier, about the community case management facility that was stood up at Wellers Inn earlier this year, and I want to understand, I think \$375 000 was what they were paid in contractual arrangements. Has that money been repaid to the Government, or what is the current arrangement with that, considering that it didn't ever eventuate?

Mr ROCKLIFF - Sure. I know that Mr Gregory has been involved with these discussions. Shane, do you want to detail some of those discussions for Ms Dow, where all that is at?

Mr GREGORY - Through the minister, we did make a commitment to the facility, so the money we paid was three months' rent in advance to move into the facility and make that ready for a community case management facility. It was during that process we realised there was a reasonable amount of work to do to implement it the way we needed to, in terms of a community case management facility and it was going to take longer than we had anticipated and wanted to take. So, we made the decision to move on to another site. Now, there is no

expectation that money would be repaid because we committed to a period of time for the site.

Ms DOW - Through you, Premier, was that the only outstanding cost? Was there additional cost to the Health department around security and those types of things that were in place on the site?

Mr GREGORY - Through the minister, with any facility we stand up as a community case management facility there are other costs. There are operational costs which we were expected to pay. So, yes, there has been.

Dr WOODRUFF - Minister, at the state election last year your Government promised to tackle the elective surgery crisis and a media release said on 5 April: 'We will prioritise 2021-22 providing an extra 8300 surgeries statewide.' The extra would be on the back of the department's annual report figures for 2020-21 which were 18 192 patients. So, we'd be expecting in the order of 26 500. The dashboard so far this year shows that only 16 300 patients have had elective surgeries and even if we have heroically high numbers in May and June for the rest of the year, that is still going to leave us something like 6000 surgeries short of your election promises. Aren't these figures even more evidence that the election promise was never achievable and it was just a dishonest attempt to get voters to think you had something to offer in health that you could deliver?

Mr ROCKLIFF - The commitment at the election, if I understand it, was 22 800.

Dr WOODRUFF - The words were, 'An extra 8300.'

Mr ROCKLIFF - And then the extra 8000 was over four years, is my advice.

Dr WOODRUFF - That is not what your media release said. That is not what you told Tasmanians. That is not what the average person would have understood that to mean.

Mr ROCKLIFF - Yes, but the four-year elective surgery plan and the extra funding took that up to 30 000. An extra 30 000 over four years.

Dr WOODRUFF - Tricky language designed to deceive people.

Mr ROCKLIFF - No, that's not true, Dr Woodruff. We can get that information for you. So, the clinicians said they could achieve a target of 22 800 admissions for the first year of the four-year plan, however, hospitals are tracking to deliver an 11 per cent uplift in the first year to around 20 300 admissions. The statewide surgical and pre-operative services committee considers the required total of admissions over the four years to be achievable, around 20 400 admissions per year over the remaining three years.

Dr WOODRUFF - So, you standby that commitment?

Ms MORGAN-WICKS - Through the minister, I cannot speak to the actual commitment because I do not have that in front of me but I can only speak to the four-year plan which the surgical and peri-operative committee have developed and approved. The first-year target was for 22 800 surgeries to be performed. It was always a stretched target

noting that the budget was handed down later in the year. They have worked very hard particularly in the first quarter of this year with additional COVID-19 load on the system to deliver and we predict over 20 300.

They are going to continue in that ramped-up state, so it has taken them time to ramp up in both resources and in infrastructure in opening an additional theatre capacity, and in the service plan we have said 20 300 - I believe that is correct - per year over the next three years to deliver the four-year plan and to try to bring the waiting list number down to 5400 which was a target that the clinicians actually have chosen as the lowest amount that they'd had in terms of elective surgery waiting list back in 2016. They actually knew the patients on the list, they could actively manage them from week to week and make sure they were bringing on the right patients for scheduled surgery.

Dr WOODRUFF - Minister, I am going to move on, basically it was disingenuous to people the way it was framed in the election, there is no doubt about it. But I want to talk about the controlled-access scheme for medicinal cannabis which still requires patients to have cannabis products prescribed through a specialist pathway through the Tasmanian Health Services. The cost of medical cannabis products through the federal scheme can vary from \$50 to hundreds of dollars per week depending on the product and the dosage that is required.

Can you please tell me how many patients have applied for medical cannabis through the Tasmanian controlled-access scheme in the period since the rules were updated on 1 July and how many patients were successful in obtaining subsidised medical cannabis prescriptions under the controlled-access scheme in the same period?

Mr ROCKLIFF - Okay, did you say 'apply'?

Dr WOODRUFF - How many applied and how many were successful, since July 2021?

Mr ROCKLIFF - The 1 July 2021 legislative amendments were made to align Tasmania with the national approach of providing access to unproven cannabis products. Tasmanian general practitioners are now able to prescribe unproven cannabis products following the same process for all other scheduled medicines in Australia. Tasmanian community pharmacies are now able to dispense unproven cannabis products as well. Like all other jurisdictions, Tasmanian GPs are required to seek approval from the Therapeutic Goods Administration under either the special access scheme or authorised prescriber scheme to prescribe unproven medical cannabis products. The Commonwealth implemented a streamlined online portal which allows for applications from prescribers to be considered by both the Commonwealth and state or territory jurisdiction within the 48 hours of all information being received.

Between 1 July 2021 and 30 April 2022, the TGA approved 83 applications from Tasmanian medical practitioners to prescribe Schedule 4 cannabis products to a specific patient. Between 1 July and 30 April 2022 TGA and the delegate of the Secretary of Health have approved 333 applications from Tasmanian medical practitioners to prescribe Schedule 8 cannabis products to a specific patient. That is the most information that I have.

Dr WOODRUFF - They are different things. One is Schedule 4 and one is Schedule 8t.

Mr ROCKLIFF - That is right; that is what I said.

Dr WOODRUFF - I wanted to know how many people have applied. You said 334 were successful - 83 applications, 334 were successful?

Mr ROCKLIFF - No, I didn't say that.

Dr WOODRUFF - Sorry, I have got confused. It is getting late.

Mr ROCKLIFF - I was talking about Schedule 4 and Schedule 8. A total of 83 for Schedule 4 and 333 for Schedule 8.

Dr WOODRUFF - Were applied for?

Mr ROCKLIFF - Applications.

Dr WOODRUFF - How many were successful?

Mr WEBSTER - They were approved but that is not the Controlled Access Scheme; that is the TGA approval process, which was the change that we made, which has seen the massive increase in the number of approvals.

The Controlled Access Scheme is now a subsidy scheme for those who cannot afford it through the method of purchasing it themselves; they can apply through their specialist for approval through the Tasmanian Health Service.

I only have the total figure since November 2017 which is 32 patients have applied for 19 approvals. We have one application that is currently still deferred so not yet completed.

Dr WOODRUFF - Through you, minister, could I take that question on notice to get the figures for the same period since 30 July?

Mr WEBSTER - Through the minister, my recollection is that that would be two patients in this financial year.

Dr WOODRUFF - Okay. The TGA shows that there have been 258 000 prescriptions for medical cannabis issued under the SASB scheme since it started in 2017 but only 564 of those were for Tasmania. That doesn't stack up with the proportion of numbers. It is an extremely low uptake of medical cannabis in Tasmania largely, some say, because of prohibitive costs and the increasing cost of living issues.

Minister, will you consider expanding access to the Controlled Access Scheme for all cannabis to make it more equitable for Tasmanians?

Mr WEBSTER - Through you, Premier, the two things in comparing those figures is the Tasmanian figure is only this financial year because our scheme didn't start until four

years after the other states and territories. Secondly, the figure we have given you is the number of patients approved whereas the figure you quoted from the TGA is the number of scripts. A patient might have multiple scripts.

Dr WOODRUFF - My question still stands to the minister. Will you consider expanding access to the Controlled Access Scheme so that Tasmanians can have a fair access to that really important drug?

Mr ROCKLIFF - I will seek advice on that matter.

Mr WEBSTER - The Controlled Access Scheme is a subsidised scheme and there is a pathway there and it is well-established through GPs, et cetera. There is a pathway to get it in a subsidised way. At the moment, that is not under review.

CHAIR - Dr Woodruff, would you put your question in writing please.

Dr WOODRUFF - Yes, thank you.

Ms ALEXANDER - Can the minister please update the committee on the work the Department of Health's infrastructure services division has undertaken this year, considering that there has been a significant funding to future health infrastructure by the Government?

Mr ROCKLIFF - I will table the infrastructure services group interviewed highlights 2021-22. You have a very good report there and we have another highlight here. You have a lot to read.

When it comes to improving the health and wellbeing of Tasmanians, it is essential that our health facilities are well designed and have the right equipment to support the delivery of quality health care services as evidenced by our plan to invest \$1.5 billion into contemporary health infrastructure over the next decade. We are expanding and upgrading our health facilities across the state to ensure we are well placed to meet the future service needs.

Today I am pleased to table the Department of Health's End of Year Highlights 2021–22 document, which outlines progress made by the infrastructure services group: to implement a new best practice asset management policy and strategic asset management plan which provides strategic guidance to ensure effective management of our significant health infrastructure portfolio; undertake medical equipment audits to classify equipment by replacement priority level with a focus on supporting increased surgical activity under the statewide elective four-year plan; manage leases, licence agreements and property purchases for the department and develop an office accommodation plan; and develop master plans to define the future vision for our major hospitals in line with clinical service planning to ensure our facilities meet future health service needs.

The highlights document also outlines the department's progress to deliver its significant capital works program for 2021–22 including: the Royal Hobart Hospital; redevelopment Stage 2 projects; the Launceston General Hospital Redevelopment Stage 1 projects; construction of new facilities at both the North West Regional Hospital and Mersey Community Hospital; and Ambulance Tasmania capital projects. One of the department's key strategic priorities is to build the infrastructure Tasmania needs for our health future and the

End of Year Highlights document demonstrates the department's significant progress towards this. I thank Ms Alexander for the question.

Ms DOW - Chair, just going back, through you, Premier, to Mr Gregory to my previous question about Wellers Inn and the community case management facility. If you could provide the committee with a breakdown of those additional costs for that facility incurred by the state Government, that would be most appreciated.

The other point that I want to raise, was there an expression of interest process undertaken for that site, given that it was a Liberal Party member who was successful in securing contractual arrangements on their hotel? Is there anything further you can provide around that?

Mr ROCKLIFF - We wanted to provide the accommodation. There are various reasons why Wellers Inn was chosen, including the proximity to the North West Regional Hospital, for example.

Mr GREGORY - Through the minister, during the response to COVID-19, it was necessary to stand up in a number of facilities very rapidly. The Premier at the time provided a treasurer's instruction that allowed us to move quickly to undertake procurement processes. Typically, we were identifying sites that fitted within parameters - proximity to healthcare facilities; proximity to demand. And we were really moving at a very rapid pace.

It was a case of identifying what were suitable properties and contacting the owners - were they interested? Wellers Inn wasn't the first site that we sought to open up. We went back to a number of facilities that we had used early on as a COVID-positive accommodation options very early in 2020 when the pandemic was on the rise. Those facilities declined to be involved, essentially because we had borders opening and they were interested in the tourism market. They weren't our first choice. We went to a number of properties that we'd already had dealings with. When they declined we went back and did more of a desktop to identify what were suitable locations.

Ms DOW - Through you Premier, there was no formal expressions of interest process; it was really just whom you could approach in the end to secure the facility?

Mr ROCKLIFF - Mr Gregory has outlined the process there.

Ms MORGAN-WICKS - Yes, thorough the minister, in terms of Mr Gregory's explanation as to the rapid pace at which we had to go to secure particular accommodation facilities, we were concerned as to the potential for Fountainside and the Coach House in Launceston to become full and a resource required in the north-west.

There are not many suitable accommodation facilities that met our criteria in the north west as you would be well aware of the numbers that were involved. We approached all. We do not have time to run a formal procurement process. We were operating under COVID-19 emergency procurement guidelines, which allowed us to go directly to market to try and source. We did not take into account or even know people's political membership in terms of ownership of accommodation facilities.

Ms DOW - The 10 budget line items that are in Budget Paper No 2, page 99, that it states are being funded from the department's existing resources, how are they going to be funded? Is that by reducing services or positions across those areas? An example of that would be the community mental health and wellbeing safety and quality positions. How are they going to be rolled out under existing funding?

Mr JEFFERY - Obviously, budget funding is an allocation of scarce resources, so there is a fixed budget allocation and decisions have to be made. There is no single answer for all 10 of those initiatives that you have identified, Ms Dow. Some of them will be internally funded from savings in initiatives next year that are funded, where there are vacancies or savings. Some will be funded through reprioritisation. It will depend. And some will be funded from existing central funds that have been carried forward from past savings.

Ms DOW - One example of that is the Tasmanian Child Health and Parenting Services capability. The budget papers state that that would be extended and funded from existing resources. I note in your previous answer you made reference to the fact that if there were vacancies across services, that funding would be used to implement these new services. So, is the budget line related to child health and parenting service capability going to be funded through the vacant child health nursing positions in the north west that can't be filled?

Mr WEBSTER - The short answer is no. The effort will be made to recruit to those positions. We do have a turnover with the child health and parenting service. We are also trying to drive changes to our digital infrastructure and improve how we do things, and we are confident that this money is within our envelope. But it won't be funded by reducing staffing in the north west to increase staffing elsewhere. This program is a statewide program and will result in additional jobs in the north west as well.

It is really about finding efficiencies through our digital strategy. We have put a lot of effort into moving towards a better record system within CHAPS over the last few years. We are putting effort into improving other processes as well. The other thing is that CHAPS is based around a very flexible workforce. We have 70 sites for 114 staff in CHAPS. So, it is about efficiencies around travel and things like that, with using telehealth and those sorts of things, which has made us confident to be able to achieve this.

Ms DOW - Do you mean using more telehealth for assessments?

Mr WEBSTER - That is right. Using telehealth, particularly for remote area assessments but general assessments, particularly for lower risk categories. Obviously, if someone is identified as higher risk, we want that personal contact. But for lower-risk categories, telehealth is actually proven to be quite a good tool throughout COVID-19.

Ms DOW - I would have thought that at such a critical time in an infant and a new parent's life, it would be really important to have face-to-face services, particularly for clinical assessment, rather than telehealth.

Mr WEBSTER - That's right. It is always critical that the first assessment is actually face-to-face and we did continue that through COVID-19. Once we have done that

assessment, we've got a risk profile. Then we can make decisions about whether it is appropriate to do telehealth.

Ms DOW - Just one follow-up, Chair. Premier, you didn't answer my previous question about the community case management facility and what the additional costs were to the Government to stand up that facility. Mr Gregory looked as though he was going to answer that but then he didn't actually provide a figure.

Mr ROCKLIFF - My apologies, I thought he put that on notice.

Ms DOW - Did he say put it on notice? I missed that. I can put it on notice.

Mr ROCKLIFF - Yes, happy to take that on notice.

CHAIR - All right, if you could put that in writing, please, Ms Dow. Dr Woodruff.

Dr WOODRUFF - Thank you. Minister, another topic raised by us in relation to men who have sex with men is their capacity to participate as blood donors, given that we have a very serious need for blood. There are many men who have sex with men who would like the Government to lobby on their behalf to remove some now out-of-date blanket and discriminatory restrictions that have been placed on men who have sex with men.

This is a national issue that comes on the back of a decision made by the Therapeutic Goods Administration to lift a ban on blood donations from people who lived in the UK between 1980 and 1996. You might remember that that was as a result of mad cow disease. There was this blanket prohibition of people who had lived in the United Kingdom in that period because of the fear of transmission of mad cow disease.

The recent strong medical research is that there is a non-significant risk to the blood supply between the individual risk assessments that should be done for each person when they go to give blood and the discriminatory, blanket, time-based restrictions that are placed on men who have sex with men. Other countries, the United Kingdom, Netherlands, Israel, France, Greece and Canada, have all now removed or are about to remove these old-fashioned time-based deferrals for men who have sex with men. They have committed to do that by the end of this year.

Will you speak to our Tasmanian representative to the jurisdictional blood committee and ask them to make a strong representation to the committee to accept the current medical evidence of the low risk to our blood supply if individual risk assessments are used for everybody who gives blood? I am sorry that is such a complicated question at this time of the night.

Mr LAWLER - It is quite a complicated question.

Dr WOODRUFF - I can repeat any bit of it if you need.

Mr LAWLER - I am aware that the Tasmanian representative to the jurisdictional blood committee has met with a number of key stakeholders in this area and this issue has been discussed a number of times. I think the challenge is the fact that there are a number of

key decision-makers in the space. The TGA, as you've referenced, is one of those in terms of the referral period and the other also is Life Blood and Australian Red Cross Organisation that accepts those donations as well.

Dr WOODRUFF - My understanding is that their stacked by a particular religious position which has a strong historical antipathy to allowing access to the blood supply to men who have sex with men. Not matched by the medical evidence.

Mr LAWLER - I can't speak to that underlying ideology. I can understand the scientific argument and I can also understand the decisions made previously by the TGA around risk management. I am very happy to have that discussion with the jurisdictional blood committee representative who works within my area.

Dr WOODRUFF - Minister, this an old issue; and I'm hoping that now that you are Premier and Minister for Health that you will find an opportunity to reconsider the position that we've taken as a state of having pill testing as a harm reduction measure. You would be very familiar with the comments that we've made over years about the evidence of the efficacy of pill testing as a harm reduction education measure.

The recommendation from the New South Wales Coroner, on the back of six deaths between 2017 and 2019, that pill testing should be introduced at all music festivals and the great work that was done in the ACT at numerous festivals there, getting the evidence of the effectiveness and showing that young people when presented with the reality of what is in the things they are thinking of putting in their mouth, overwhelmingly choose to bin them. That is about saving lives first and foremost. This needs to be essentially a commitment to reopen this conversation. All of the will and all of the capacity is there in Australia for Tasmania to be able to provide these services at festivals. It would just be a great thing if we could just move on and include it as part of our harm reduction suite of measures for drugs.

Mr ROCKLIFF - While this is probably more appropriate in the Mental Health and Wellbeing side of the portfolio, I am happy to answer it.

Pill testing is an issue on which opinion is divided right across the community. We have been clear in the past that is not something that the Tasmanian Government supports at this time. What we can all agree on, of course, is that we want our festival goers to be safe and that is what the Government will focus on for future festivals. I remember the lead up to a 2019-20 summer; a lot of work was done and I was in discussion with Ms Morgan-Wicks and others about supporting our young people at these festivals. Pill testing is one measure that can be used to improve the safety of patrons at festivals.

As I have alluded to, the department committed to improving health outcomes at music festivals and has previously provided support to Tasmanian festivals to minimise harm from consumption of drugs and alcohol. The Government, through the Department of Health, worked successfully with the festival organisers behind The Falls and Party in the Paddock in the past, with the establishment of a safe space tent and increased resources. This is something that we may consider doing again in the future, if that is appropriate. Of course, no Tasmanian Government support to music festivals was required in 2020, as all major Tasmanian festivals were cancelled due to COVID-19.

Key areas of the Department of Health have been diverted to the COVID-19 response and no work has progressed in the department in the absence of major festivals being held in Tasmania. I note however that the Party in the Apocalypse was held in Launceston in December 2021 and Hobart in March 2022. While at this stage we don't currently support pill testing, we will continue to consider evidence on the matter as it emerges. Implement any pill testing in Tasmania would require complex policy and legislative framework development. There is a bit of work to go through yet. I know it's been a long-standing policy and commitment of the Tasmanian Greens and you've been speaking about it for some time.

Dr WOODRUFF - It's very disappointing, on behalf of all of the wider community organisations that are pushing for this. It's all RACGP, emergency medicine, AMA, ANMF, Ambulance Union, and the Public Health Association.

There are so many who support this now, and it really is a case of moving on past the ideology. It was never popular to take the sorts of approaches that Australia did in the AIDS epidemic. If it wasn't for injecting drug support, and the needle and syringe exchange - that was contentious at the time, but that saved lives. The coroner was really clear this can save lives. There is nothing like the personal, one-on-one information to a person actually looking at what's in the thing they are about to put in their mouth. All the pamphlets and conversations will go nowhere.

Mr ROCKLIFF - I am happy to look at the evidence in support, and base our decisions on that. I was President of an organisation called Youth and Family Focus in the late 1990s early 2000s. Needle availability programs were quite contentious at that particular time. I remember arguing publicly at that time, in favour of needle availability programs, because the organisation I was Chair of, provided that service. It was very contentious, and very contentious with local business providers. We stuck to our guns, because clearly evidence was there to support that.

Dr WOODRUFF - I will keep providing the evidence, and sending it to you.

Ms DOW - Are increases in adverse events driving the increase in liabilities for medical in the Tasmanian Risk Management Fund?

Mr ROCKLIFF - My advice is the Treasury manage that Fund.

Ms DOW - I'll ask it in Treasury. My next question is, how many vacancies are there across the hospital pharmacies - full-time equivalent and by head count?

CHAIR - As the time has expired, could we please move across from Health to Mental Health and change staff over.

CHAIR - Thank you, Premier. Can I invite you to introduce the persons at the table, their names and positions for the benefit of Hansard and to give a brief opening statement please?

PUBLIC

Mr ROCKLIFF - Thank you. To my right, there is Ms Morgan-Wicks, the Secretary and State Health Commander; to my left is Dale Webster, Deputy Secretary Community, Mental Health and Wellbeing, Commander Health Emergency Co-ordination Centre.

I have an opening statement which includes the importance of building positive mental health and wellbeing as being very important to me and I will continually personally champion, which is why I have retained the Mental Health and Wellbeing portfolio.

Highlights of this year's Budget include an additional \$20.5 million ongoing funding from 2022-23 to continue implementation of the recommendations of the Roy Fagan Centre and the Older Persons Mental Health Services Review. Additional funding of \$1.5 million is also committed over three years for the implementation and the evaluation of Rethink 2020-25 and Tasmania's new over-arching mental health plan with the focus on suicide prevention.

Our \$45.2 million child and adolescent mental health services reforms continue to be progressing and the emergency mental health co-response model, PACER, commenced in January this year as a pilot in southern Tasmanian. Given its success, additional ongoing funding has been provided with a pilot in the north-west to commence in early 2023 as the next phase in evolving PACER to a statewide model.

Additional funding is provided in 2022-23 and 2023-24 years to support the reform agenda for the alcohol and other drug sector in Tasmania. This funding builds on existing Government investment into alcohol and other drug services to enable harm-reduction. Funding is also provided over four years to support the whole-of-Government Tasmanian drug strategy 2021-27 and the Drug Education Network. The department will also commit funding over two years from the existing resources for a grants program to allow organisations in the alcohol and other drug sector to employ peer workers, enhancing delivery through the sharing of lived-experience.

Since coming to Government, we have invested some \$370 million in mental health and alcohol and other drug services and we will continue to allocate record levels of funding across public mental health and alcohol and other drug services and the broader sector which is something I am very proud of and will continue to champion.

Ms DOW - Premier, at the outset of this committee hearing this afternoon, we spoke about the changes to acute mental health services in the north and north-west of the state. There still really has not been an explanation as to why that wasn't communicated publicly to people - why there was a memo issued by the department but really only upon questioning from the media, is my understanding, as to why there was any further explanation provided by the department about these changes and what that would mean for Tasmanians.

If you could clarify that for me again as to why that was the case. The other thing I want to ask is about transport for people, particularly from the north-west. How are people expected to transport themselves to the south if they are in acute mental health distress? What arrangements are in place to ensure that people can safely access the services in the south and will they be transported by a non-emergency transport or will it be via ambulance? If it is via ambulance, will the system cope with this additional demand?

Mr ROCKLIFF - Thank you for the question. Although Northside and the Spencer Clinic are not currently accepting new admissions, both facilities continue to operate and provide high quality care to patients in the north and north-west in need of mental health care can continue to attend these facilities where they will be triaged, assessed and treated on site as required. Patients requiring admission for acute mental illness will be transferred to the mental health inpatient unit of the Royal Hobart Hospital, while patients requiring general hospital admission can be admitted through the North West Regional Hospital and the Launceston General Hospital emergency departments, respectively.

These measures are in line with the Department of Health's established protocols implemented at times of surge demand or operation pressure. Hospitals within our statewide health system work together in an integrated way to support other parts of the system experiencing pressure which may involve patients being transferred between regions. Outbreak management teams at both Northside and Spencer Clinic will continue to monitor their respective outbreaks and re-open to new admissions when safe and appropriate to do so in the near future.

I do want to reassure members and the Tasmanian community that clients requiring clinical intervention will continue to receive service. With COVID-19 still circulating within the community we will continue to see outbreaks in health settings and we will respond appropriately to keep patients and staff safe. Ms Morgan-Wicks did detail this matter comprehensively this afternoon. I know you have a question about transport, so we cover that potentially, Mr Webster?

Mr WEBSTER - I think it is important to reinforce the reason we did not advertise it in a major way is that we did not want people to assume they could not go to the North West Regional or the Launceston General Hospital for mental health treatment. It is inherent in your question about how do they get to the south. That would come into people's minds if we actually put out there that we have only got the south open. It was important. It was a real decision not to say 'do not come to us,' because we will still admit them, it is just that we cannot admit them to the units that are closed because of COVID-19.

The second part of it is that the level of transport will depend on the acuity of the admission. In general, most of them will be by ambulance but we are doing triage on site and, in fact, over the weekend for instance - the unit was closed late last week so it has been closed over the three days of the weekend - there wasn't a need to transfer to the south because we were able to divert or triage onsite and find alternative treatment methods for those particular patients. It is a difficult cohort who might avoid treatment if they think that they have to travel all the way to Hobart to get it. It was not a case of not going to tell anyone, we made a positive decision that it was clinically appropriate just to encourage people to continue to come in the normal way.

Ms MORGAN-WICKS - Certainly we do not issue a media release every time there is an outbreak in a particular part of a facility, so we do take an operational decision to determine whether it is required to be publicly known or not, depending on our ability to provide that health service in another way, for example, at that same location. There have been hundreds of small outbreaks that have occurred across various organisations, not just within Health, since January. Those have been very actively managed on a daily basis by our Public Health Emergency Operation Centre. When it happens within a health service we

have dedicated protocols that come into play to manage the outbreak and we will make operational decisions which we immediately share with all of our staff.

I know the minute, sometimes, we put up an article on our own communication tool, Reach, it can obviously also go out externally but we have to make those judgements. Certainly, Mr Webster detailed the concern that patients may not turn up for care should they make a particular decision.

Ms DOW - The fact is though that information was then disseminated to people but not through a controlled way which by the sounds of it, that is what was intended, Mr Webster, that you wanted that to be controlled and not to cause distress to people. My concern is the way that information was disseminated to people and obviously picked up by the likes of the Vigilante News was not helpful in conveying what options were available to people. It is pretty serious when you cannot be admitted to the acute mental health unit and the closest one is four hours away, which is the case for the community where I live.

I would have thought it would be very important for people to be provided a good level of detail and information about that and what supports we put in place to support them to access that care when they need it. Whilst I understand the sensitivity about not wanting to cause undue alarm or people access services when they need them, it is disappointing the way that information was communicated and the fact it was lacking in what support services and what options would be available to our community. It is probably more of a statement than a question. I am putting it on the record about breaking in why I am asking these questions and why that is serious.

Ms MORGAN-WICKS - Through the minister, I am happy to take that statement as a criticism. We learn each and every occasion in terms of outbreak. We do want people to continue to turn up to the acute mental health services in the north-west and the north, as they should be, so that we can assess and triage them and then provide adequate transport, but we do need to make sure that we risk balance against their admission to a unit that has an outbreak of COVID.

Ms DOW - Premier, can you please update the committee about when the north-west mental health in the home program will commence? I had feedback that that was meant to commence last year in the north-west as part of the CAMS review and increasing access to mental health services across the north-west coast?

Mr ROCKLIFF - The extension of the Mental Health Hospital in the Home investment represents a commitment to improving the mental health of Tasmanians through the provision of greater community-based supports. The Mental Health Hospital in the Home service has been operational in southern Tasmania since March 2019.

The majority of admissions are received from the acute in-patient mental health unit at the Royal Hobart Hospital. Referrals can also be received from the emergency department at the Royal and mental community teams. Mental Health Hospital in the Home has improved access to acute care for presentations to the Royal Hobart Hospital requiring admission, the positive impact on patient flow for people presenting to the hospital.

The service is now co-located and aligned with the new acute care stream team, within mental health services. From 2023-24, we will provide \$8.5 million over two years for a mental health hospital in the home pilot in the north-west - so next year. This pilot, based on the existing southern service with a focus on youth aged 16 to 25 in the north-west will enable people that may have otherwise been hospitalised to receive intensive short-term support in their own home. The Tasmanian Mental Health Reform Program has covered the early stage of progressing this particular initiative.

Dr WOODRUFF - There are, undoubtedly, some very welcome investments in the Budget into mental health, there is no doubt about that.

I want to raise the issue of workforce development, which the sector is concerned about in understanding what the Budget means. The Budget came out after the bilateral, National Mental Health and Suicide Prevention Agreement was issued; it was issued a fortnight beforehand, so I assume that those commitments in the bilateral agreement are reflected in the Budget. The issue of workforce development is getting to be very critical - the shortage of qualified mental health workers in Tasmania. The rising costs of employing people and living costs are just compounding the issue.

My question is: given that lack of investment from your Budget in workforce development and given the bilateral agreements commitments to what is going to be delivered in terms of largely community-based mental health initiatives, how are they going to be administered by Tasmanians without money into workforce strategy? I understand that it is the state's responsibility to deal with the funding of workforce issues.

Mr ROCKLIFF - Thank you for the question. The next few years will see major, systemic reform that will have a significant impact on our current services and how we support our consumers into the future, including a considerable growth in clinical positions. We are very pleased to be expanding our mental health workforce and this is a critical investment to support the delivery of quality services that ensure Tasmanians have access to the treatment and care they need.

To grow the workforce, including the lived experience workforce, there will be continued and concerted recruitment and retention activity, and a focus on workforce development. These options include the introduction of a number of developmental positions, such as assistant in nursing, transition to practice nurses, and entry-level positions for allied health professionals and the service consistently advertised as vacant positions interstate and overseas.

The TTP program supports new graduate nurses into statewide mental health services and aims to support nurses to go on and complete their postgraduate diploma and mental health nursing via the provision of fully sponsored and supported training. The number of positions under this TTP program across the state has been increased from 24 to 49 positions in 2022, an increase of some 25 positions.

The service supports student placements for social work, psychology, medical and nursing. The Government has recently agreed to support a market retention allowance for psychiatrists. There is a dedicated staff member within the mental health reform program who works exclusively on recruitment and can provide extra assistance to interstate and

international inquiries. We are also excited to be supporting the growth of mental health peer workforce. The mental health peer workforce coordinator has recently been appointed, based at the Mental Health Council of Tasmania, which is good, and is working to implement actions under the peer workforce development strategy and establish Tasmania's youth peer worker model.

Peer workers, with their personal lived experiences, hold an important level of knowledge and understanding as they provide advice and hope to both consumers and carers on their own mental health journeys. Importantly, a key action of the Rethink 2020 Implementation Plan is to develop a joint workforce development strategy with public, private primary and community sectors, along with education and training providers. This work will commence later this year.

We are also investing some \$5 million to expand the health hub at TasTAFE's Alanvale site to include a new facility that will deliver training and workforce development for workers in the alcohol and other drug, mental health and youth sectors.

Mr WEBSTER - Through you, Premier, in addition, the state Budget funds a number of allied health education positions within the agency, which is to support the uplift in courses for physiotherapy, speech pathology and occupational therapy at UTAS, with two of those degrees starting this year and a third next year. Those positions will facilitate the placement of those students and hopefully facilitate the long-term staying of these students in our state. So, that's another initiative as well.

Dr WOODRUFF - That is all very important but I think you confirmed my question by the answer. Almost everything of what you've said was to do with the state mental health services. My question was about the community organisations and the concern in the sector, from the community organisations that will have a large footprint of responsibility in the bilateral agreement to deliver the services, is that there is no money in the Budget for that. I think you mentioned talking about it with the sector later in the year. That is good, but where is the financial commitment over the forward Estimates to workforce development training?

Mr ROCKLIFF - We have supported the Mental Health Council of Tasmania with funding to monitor and collect data on the impacts of COVID-19 in the community mental health sector workforce over the last 18 months. The Mental Health Council of Tasmania has released the COVID-19 Impacts on the Community Mental Health Workforce Report, which is informing our Government's ongoing planning and response supporting the mental health and wellbeing of Tasmanians.

The report demonstrates that the pandemic has exacerbated many pre-existing challenges facing the community mental health workforce in Tasmania. There will be a focus on addressing the key priorities. Immediate actions required from the report include: integrated workforce planning to support recruitment and retention; prevention and early intervention measures to address increased service demand; upskilling and diversifying the mental health workforce; fostering and supporting the wellbeing of staff; equipping mental health services to respond to ongoing COVID-19 impacts, and data collection and monitoring to inform an effective response. These actions have been mapped against our Government's Rethink 2020 Implementation Plan to ensure that they are a priority in our shared approach with the community sector, public health system and primary care.

Mr WEBSTER - The bilateral is not reflected in the state Budget because the timing of the bilateral was too late for the state Budget.

Dr WOODRUFF - Really? It's being negotiated.

Mr WEBSTER - That's right. But until it's signed, it's not money that can go in a state budget. That's the first thing. The second thing is that Primary Health Tasmania will be the primary funding source from the bilateral to the community sector. The CEO of Primary Health Tasmania had already met with the department with a view to setting up a joint approach to workforce development in relation to the bilateral.

Dr WOODRUFF - Thank you. The Rethink 2020-2025 Implementation Plan has a reform direction 3 and, particularly, direction 9, which is about supporting and developing our workforce. That's not related to the bilateral agreement. Where is the money in the Budget to implement that part of the Rethink 2020-2025 Implementation Plan?

Mr WEBSTER - The general workforce development work happening within statewide mental health services is not just focused on mental health services. The work that has been done by the Mental Health Council and the ATDC informs us, but also the work we're doing jointly with primary care because we acknowledge that in a state of our size we can't be competing for the same workforce. We advertise a job, take one from them, they advertise and take it, so we need a joint approach. It doesn't require an additional funding line; it requires the cooperation across the services to make sure the efforts we're making are actually benefiting the entire service.

CHAIR - Mrs Alexander.

Mrs ALEXANDER - Minister, can you please provide the committee with an update on the recent work of the Tasmanian Mental Health Reform Program?

Mr ROCKLIFF - Certainly. The reform program commenced in August 2019 with a key task of implementing the 21 recommendations from the Mental Health Integration Taskforce in government response. The core goal of the reform program is to transform the mental health system in Tasmania and to ensure that people who live with mental health issues or in suicidal distress have access to world-class systems of care, including alternative services for people to avoid acute hospitalisation where this is not necessary.

There is \$6.48 million provided in this year's Budget from 2022-23 to address priority areas in the mental health reform program. This funding includes commencement of the roll-out of the adult acute care and continuing care models of service in the north and north west. This funding is in addition to the \$8.8 million provided in the 2021-22 Budget for the full operationalisation of the acute care team in the south and the progression of the Peacock Centre facility and associated service components in the south. It is also supporting the ongoing work of the Tasmanian Mental Health Reform Program project team, which is coordinating this work and several other initiatives.

Progress on our new community mental health hubs, including 27 new mental health beds, is well advanced, with the development application being considered for the St Johns

Park site and building works progressing at the Peacock Centre site following that terrible damage sustained in a fire on Christmas Eve last year.

The highlight of the mental health reform program has been the commencement of PACER in January this year. PACER is staffed by trained mental health clinicians who work alongside police officers and paramedics, and provides a dedicated rapid specialist response to acute mental health or behavioural concerns in the community. The function of this service is to provide timely, integrated and coordinated mental health care, including mental health service-based secondary triage, mental health assessment, brief interventions and referrals. In its first 17 weeks, PACER responded to 486 clients with 75 per cent of these people able to remain in the community without the need for attendance at the Royal Hobart Hospital ED. There has been \$9 million provided in this year's Budget for the continuation of PACER in the south and we have also committed to rolling out the mental health emergency response model in the north west as well. It is a great example of innovation in an increasing demand-driven system.

Ms DOW - Mr Webster said before about the importance of not utilising each other's workforces. We look to see this additional funding come to the state through the bilateral agreement.

Have you estimated what the shortfall of mental health workers and practitioners is across Tasmania? There are already considerable problems with recruiting now, so it is going to be a significant challenge to deliver these additional services.

Mr WEBSTER - Across statewide mental health services and the community sector, at this stage we predict that there will be a need for 350 additional staff over the next two years.

Ms DOW - Premier, what's the Government factoring into service agreements for community-managed mental health services to enable organisations to manage CPI and superannuation increases as the state funds some organisations to provide outreach services. How is the Government factoring increased travel costs as well? This relates to the indexation across the Budget of 2.5 per cent.

Mr ROCKLIFF - We have a very strong relationship with the community sector across Tasmania. The sector delivers important health services and initiatives to the Tasmanian community, providing support to the vulnerable and priority population groups. The Community Sector Grants Program administers over 160 funding agreements to 100 community sector organisations across a range of program areas such as mental health, alcohol, tobacco and drugs, public health, and home and community care services. By 30 June 2022, total funding provided to those organisations will exceed \$73 million. This includes over \$10 million associated with recent election and state Budget commitments.

The 2022-23 state Budget and forward Estimates continues our strong commitment to the community sector by providing commitments to various community sector organisations totalling over \$2 million.

Mr WEBSTER - We do work with the community sector organisations on their sustainability, including impacts on them, those sorts of things. There are occasions where they will underspend and we will use that as the opportunity to look at the sustainability

model. It may be returned to us to be used in other ways or may be that we look at other cost pressures that apply. We are working very closely in this portfolio. We have grants team in both public health and the mental health, alcohol and drug directorate. We are in constant contact with our providers to make sure that they are sustainable and can deliver what we need them to do.

Ms DOW - Premier, I recently visited Curraghmore House in Devonport with residents there who have a lot of uncertainty about the future of their residence. Their funding expires on 30 June. There are a number of residents there who are unable to transfer to an NDIS package. What will happen to those people? Will they still be accommodated at this facility post-30 June or will they need to find other accommodation options? And will they be supported to do that? At the moment it is still quite tenuous for the small percentage of the people that live there.

Mr ROCKLIFF - I visited Curraghmore House as well. It is one of several residential rehabilitation facilities that formed part of Tasmania's contribution to the National Disability Insurance Scheme under the bilateral agreement between the Commonwealth and Tasmania. Anglicare Tasmania, the care provider at Curraghmore, finding navigating the NDIS application process difficult. Anglicare Tasmania has allocated additional resources, specifically to address the matter of progressing applications due to the complexity associated with the application process.

It has been identified that some of the clients based at Curraghmore are not eligible for the NDIS, as pertained to your question, due to the intricacies in the assessment process. A letter has been drafted to be sent to the Prime Minister regarding concerns with the process and assessments of residents at Curraghmore. The Tasmanian Government has been required to continue financial support of the Curraghmore facility to support clients not currently registered with the NDIS.

If Curraghmore clients continue to be ineligible for the NDIS, a longer-term solution will be investigated and implemented as a matter of priority. The Department of Communities Tasmania, through the Community and Disability Services Branch, will need to be engaged in this matter to work through options for continuity of service provisions for individuals who are either ineligible for the NDIS; or choose not to test their eligibility; or are over 65 years old and therefore outside of the age range for the NDIS.

A letter of intent has been provided to Anglicare Tasmania advising that funding will be provided to Curraghmore for the 2022-23 financial year. Funding of just over \$1.3 million will be provided for the period 1 July 2022 to 30 June 2023. The amount of funding being provided under the agreement is for full occupancy of 12 clients, and provided for a period of 12 months. The funding agreement will be administered by the Department of Health's Mental Health, Alcohol and Drug Directorate and will be developed in consultation with Anglicare and Statewide Mental Health Services.

It is the Department of Health's expectation that the funding required by Anglicare Tasmania for Curraghmore will reduce throughout the 2022-23 financial year, as clients receive NDIS support. The funding agreement will incorporate necessary clauses to enable the repayment of funds. The Department of Health requires Anglicare Tasmania to take steps

PUBLIC

to ensure that clients are not being dual-funded by both the state and the Commonwealth, and that no additional long-term clients will be provided with places at Curraghmore.

I know that is a comprehensive answer, but pleasingly -

Ms DOW - What happens past June 2023, Premier?

Mr ROCKLIFF - We will continue to work with Anglicare, as I have said. Is there anything further to add there, Dale?

Mr WEBSTER - The longer-term future of the clients is that the majority of them will move, we believe, to the NDIS. It is the navigation that we are working on, rather than our belief that they are not entitled to it.

Secondly, if they are not entitled then we will be looking for the alternatives away from Curraghmore, such as Richmond Fellowship in the north-west, and give them a placement other than at Curraghmore.

Dr WOODRUFF - Premier, despite the progress that has been made in Tasmania, LGBTIQ+ people of all ages still report stigma, prejudice, exclusion and discrimination in our communities. Those experiences impact upon their mental health and wellbeing, and that is a core issue that was identified in your Government's LGBTIQ+ Tasmanians: Telling Us The Story report. The report notes that mental health care was, without exception, one of - if not the most - prominent point of discussion, an area requiring urgent attention for LGBTIQ+ Tasmanians.

There were a number of recommendations that emphasise the need for comprehensive policy and service responses in the mental health space, including funding for a dedicated mental health service; increasing mental health care services in rural and remote areas; and increasing health care practitioners' knowledge and awareness of the LGBTIQ+ people.

Minister, will you commit to funding the implementations of the recommendations made in the report, and talk about what you are doing now, to address these key mental health issues?

Mr ROCKLIFF - We are committed to the health and wellbeing of LGBTIQ+ Tasmanians, and the Department of Health is progressing key actions to improve health outcomes for the LGBTIQ+ community.

We have an active LGBTIQ+ Reference Group, which includes membership across the department, non-government sector and community representatives. The Department of Health is progressing reforms started in 2020 in response to legislative changes to the Justice and Related Legislation (Marriage and Gender Amendments) Act 2019. A dedicated resource within Health ICT is working to modernise our business practices and systems to align with current inclusive best practice.

LGBTIQ+ learning resources for all Department of Health staff were launched in December 2021, and include an online introductory module, a webpage, discussion guide and supporting materials. The resources were developed in collaboration with the LGBTIQ+

community and they are being implemented across the department. Feedback to date is very positive.

Through the department, the Government funds a number of organisations to provide services to the LGBTIQ+ community, and advice on those matters. These include: Working It Out; the Tasmanian Council on AIDS, Hepatitis and Related Diseases Incorporated; and Women's Health Tasmania. Through the Healthy Tasmania Fund round two grants program, Working it Out has also been funded to expand their peer support project, Working It Out Together, which was set up in response to COVID-19.

In addition to national research, the department is informed by Tasmanian data. The recent Tasmanian Government LGBTIQ+ survey report, Tasmanians: Telling Us the Story, will form the basis of the Government's new LGBTIQ+ Strategy. This will support the Department of Health to identify future actions. Research such as Private Lives 3 and Writing Themselves In 4 highlight the need to support mental health and wellbeing for LGBTIQ+ Tasmanians.

In line with reform direction three - reducing stigma, and reform direction seven - responding to the needs of specific population groups of the Rethink 2020 Plan, the department is working with LGBTIQ+ Tasmanians to reduce stigma and to ensure that they can access the mental health support they need, when they need it. The department is working with the LGBTIQ+ community to understand the gaps, the needs and issues within the current mental health system specific to the LGBTIQ+ community. In the 2022-23 state Budget, the Tasmanian Government committed funding of \$375 000 over three years to employ LGBTIQ+ peer worker navigators, and the department is also partnering with the LGBTIQ+ community to determine and implement actions that will ensure Tasmania's mental health service system - at all levels - understands and provides care to meet the specific needs of the LGBTIQ+ community. Thank you very much for the question.

Dr WOODRUFF - Thank you. I have a follow-up question about that report. Thank you for those comments. The report also had heartbreaking stories of trans and non-binary people talking about feeling marginalised and unsafe. One finding said that the prejudice towards the trans community is still bad enough to force people to leave the state. Will you again reassure LGBTIQ+ Tasmanians by strongly condemning the rump position of members of the federal Liberal Party, Claire Chandler and the new Leader of the Liberal Party, Peter Dutton, who are still saying hateful and divisive things about trans people, including the proposal for the national curriculum and to go in and be divisive about trans and non-binary people? Will you hold firm on our state's position of promoting inclusion for trans and non-binary people in all areas, including in schools, sports and everywhere?

Mr ROCKLIFF - I would encourage everyone to be respectful, not divisive. I made it very clear the intention as Premier to ensure that all Tasmanians be valued, included, encouraged and supported to be best they can be. My comments regarding the bill you refer to are well known in that sense and I will not change my position on those matters.

Ms DOW - Premier, my question is in relation to the lack of medically supervised withdrawal beds in Tasmania. Would you be able to explain your Government's approach providing adequate access to medically supervised withdrawal beds noting the ongoing advocacy of the community managed sector to have withdrawal services available in the

community given there are only eight beds available in the public system to support over 100 residential rehabilitation beds operating in the community sector.

Mr ROCKLIFF - I might get Mr Webster to talk about that in more detail. We are investing some \$7.5 million to continue the reform agenda for the alcohol and other drug sector. The 2021-22 budget also provided \$450 000 over three years for the Alcohol, Tobacco and other Drugs Council to employ a fixed term project officer. This position will support the roll out of the Government's reform agenda for the alcohol and other drug sector in Tasmania and increase the capacity of the ATDC to establish a consumer representative body. An additional funding of \$900 000 over four years has been provided for drug education network to continue to meet increased demand for education in the community.

Funding of \$1.27 million has been provided to support the whole of Government Tasmanian drug strategy to prevent and reduce the health, economic and social costs and harmful effects of alcohol, tobacco and other drugs. If I go to the commitment of \$7.5 million over two years to continue the implantation of the reform agenda of the alcohol and other drugs sector in Tasmania to increase access to alcohol and drug services across the state. Implementation to date has increased specialist resources within the alcohol and drug service and the new detox of the home service will be operational in July 2022.

The ADS is a state-wide service that provides specialist support and treatment through community teams, psychosocial support and interventions, youth programs, opioid therapy program, inpatient withdrawal management unit, a consultation liaison and smoking cessation program. It provides the vital specialist services as part of the alcohol and other drugs service system, together with the funded community sector organisations. The community sector organisations are funded to provide a range of alcohol and other drug support and treatment interventions ranging from health promotion and prevention, family support psychosocial counselling and residential rehabilitation including day programs.

The Government's alcohol and drugs service showed there were 25 people on its pharmacotherapy wait list at March 2022 down from 196 at December 2020 and 127 at May 2021. It is expected the significant decrease in the number of people waiting for treatment will continue as resource issues are addressed in a model of care.

Mr WEBSTER - The inpatient unit at St John's Park is actually a state-wide unit. It has been challenged through COVID-19 because of its set up and we have had reduced beds through that period. Your question was about medical supervised withdrawal. The answer is really important in that the reforms are about better integration so that we can actually provide that more broadly across the state. One of the very first steps is the detox-in-the-home program. There will eventually be a medically supervised statewide program. That should be online in July 2022. So, we are growing the service over a period of time.

Ms DOW - Through you, Premier, is there a view to have dedicated beds in other parts of the state, like at St Johns Park, or will they only be in the home?

Mr ROCKLIFF - The inpatient withdrawal management unit is a specialist, nine-bed, inpatient unit located at St Johns Park, New Town. The service supports clients from all regions of the state.

Mr WEBSTER - Our clinical services planning, as the Secretary for Health also mentioned, also applies across mental health. That is happening throughout 2022 and that will inform what we need across the state. If there is a need to spread the beds completely across the state, that will be highlighted through that plan, but incredibly important is that better integrated model so we are providing more services at a local level rather than at statewide level.

Ms DOW - Another question is in relation to another federal commitment that was made three years ago around access to perinatal mental health services on the north-west coast and through the LGH as outreach services. My understanding is that those services have not been provided yet. I would like to understand when they will be and any other further information that you could provide would be most appreciated.

Mr ROCKLIFF - Perinatal mental health services, services for parents experiencing mental health concerns from conception to one year of age, are organisationally aligned differently across the state. In the north and north-west, perinatal services are currently located within the adult community health services, and in the south the service sits within CAMS, for example.

Consideration of the most appropriate service location of perinatal mental health services in consultation with adult community mental health services is prioritised for 2022 to determine reform options. In the southern region, the perinatal team provides a specialist service for pregnant and postnatal women with a range of mental health problems, psychiatric disorders, and the team also provides mother-infant therapy and to high-risk dads as well.

Mr WEBSTER - We have established a perinatal mental health services in the north and north-west, and we will expand them over the next period. One of the critical issues comes from not being part of CAMS in the north and north-west, so they have a different model of care. We are developing the model of care to make sure that we have a statewide service. The second area as has been highlighted is the workforce development issues, so we are going through the recruitment processes as well to establish that.

It is important that we put in a level of service almost immediately, and that has been ongoing. We are building that service with a model of care and those sorts of things. As the Premier highlighted the service is well established in the south, and we would hope to be able to replicate that across the state through this source and through the CAMS's reforms.

Mr ROCKLIFF - The main 2019 commitment by the previous government was some \$4.5 million for new perinatal infant and mental health services for mothers and babies at the North West Regional Hospital. The full \$4.5 million was received by 21 September.

This is probably a bit more information for you as well. Funding has been allocated to adult community mental health services, north, north-west, and the child health and parenting service, north, north-west; \$1.5 million was received in August 2020, and \$3 million was received in 2021. There will be a reconciliation of the funding following the end of the 2022 financial year, with any unspent funds carried forward to continue the rollout of the service in the 2023 financial year.

PUBLIC

The funding has enabled the government to recruit additional staff to deliver peri-natal and infant mental health services in the north, and north-west.

Ms DOW - When will your government provide dedicated mental health beds for our young people on the north-west coast?

Mr ROCKLIFF - Thank you for the question. For background, adult community mental health teams in the south have been transitioning to an acute care stream and continuing care stream. The ACT, the frontline service which responds to acute emerging mental health needs in the community is designed to be a safe alternative to ED presentation. The CCS - that being the continuing care stream - is designed to provide specialist case management care co-ordination for individuals assessed as requiring an extended period of mental health treatment and care.

Funding is provided in the 2022-23 budget to commence the rollout of the ACT and CCS in the north and north west, investing \$8.5 million over two years for a mental health hospital-in-the-home pilot in the north west to commence next year which will have a youth focus for 16- to 25-year olds, as I stated I believe today. Work is under way to develop an operational service model that meets the needs of the region and incorporates some of the key learnings from the evaluation of the southern mental health hospital-in-the-home service. We are pleased we are bringing new mental health services such as these to north west coast and I note we -

Ms DOW - The question was about dedicated beds though, patient beds.

Mr ROCKLIFF - Yes, I am just detailing a number of matters pertaining to increased services regarding mental health for the region. I can mention additional funding provided to Rural Alive and Well to increase their capacity; provide a phone for Life Without Barriers; to deliver the YouthARCH initiative and that was a commitment made to the Circular Head region. I have mentioned PACER earlier as well; supporting the grief support service of New Mornings in Ulverstone, a not-for-profit mental health service supporting the north west Tasmanian community with \$340 000 new funding in this year's budget. Of course, the 2022-23 Budget provides -

Dr WOODRUFF - Chair, I am just looking at the time. If I could interrupt the minister

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Mr ROCKLIFF - a total of 40 million for Stage 1 of the new mental health precinct adjacent to the North West Regional Hospital to replace the ageing Spencer Clinic, and clinical services planning has begun for both the new mental health precincts which will guide the functional requirements and model-of-care for how services will be delivered.

Dr WOODRUFF - Minister, the evidence is developing, unfortunately, that the relationship between mental health and natural disasters is a very strong one and that people's mental health, particularly young people's mental health is significantly worsened after natural disasters. Some evidence from the southern Sydney area in the Shoalhaven, which was very badly affected by the Black Summer bushfires and then by the recent flooding on top of COVID-19 isolation issues.

A youth trauma recovery organisation called Youth In Search reports a cluster of suicides in Sydney, after those experiences, identifies that more than 5000 children and youth are at risk of significant harm in the area, and identifies that one in five children in the area is affected by mental illness with more than a third of self-harm emergencies involving people aged 15 to 24.

Clearly a terrible situation. My question is, has your department undertaken or considered looking at focused mental health disaster planning, including the immediate response support and a program of planned support for the two years after an event, which is the most important point for serious self-harm and degrading mental health, and if you haven't, will you commit to looking into that?

Mr ROCKLIFF - Thank you for the question. Perhaps I will refer to Mr Webster in respect of the departmental view pertaining to Dr Woodruff's question.

Mr WEBSTER - Thank you, Premier. Yes we have started that planning, so the mental health disaster planning is under way, being led in Tasmania by the office of the chief psychiatrist. In addition, we are working across the states and territories and the Commonwealth in this space, and in fact if I was not here this afternoon there was actually a national exercise in how we would respond in a disaster, and particularly how I would respond as the mental health recovery co-ordinator in the state. So there is work at both the state and federal level in this area.

Dr WOODRUFF - That is exciting. That is the immediate acute response, but are you also looking at the two years post-response.

Mr WEBSTER - The recovery is not defined as immediate. The immediate is the response, and then beyond the response there is the recovery, so recovery is the longer term.

Dr WOODRUFF - Thank you very much. Minister, I have a question about the registration for working with vulnerable people. The ACT has a broader purview on who is captured under the requirement to register to work with vulnerable people, they regulate activities that include amongst other things mental health and addiction services. Do you agree that people using those services can be vulnerable, and, that services should be regulated under the registration to work with vulnerable people regulations, and, will you investigate that and work with the Minister for Justice in making some changes to the act?

Mr ROCKLIFF - I am advised that we are currently looking at expanding the working with vulnerable people process throughout the Health department to cover all employees. Is that correct Ms Morgan-Wicks? Would you like to provide some more information?

Ms MORGAN-WICKS - Through the minister, we are currently examining and working with Justice, who are responsible for that particular act, but the health executive had committed to having all Health employees registered to work with vulnerable people under the act, but, obviously that is a large number, and we are working through that in a staged process with Justice.

Dr WOODRUFF - And in relation to the specific groups that I mentioned, will you look at that as well?

PUBLIC

Ms MORGAN-WICKS - To the extent that they are our employees, we are happy to consider the issue generally across the health sector and provide Health-relevant advice through to the Department of Justice and through to the Attorney-General.

Dr WOODRUFF - Oh, so you wouldn't cover non-state employees under the working with vulnerable people regulations? Wouldn't that capture all people working with designated vulnerable people?

Ms MORGAN-WICKS - Through the minister, Health is not responsible for the act or the regulations, we are able to comment in relation to what we do with our own employees.

Ms DOW - Premier, can you provide clarification of where the \$9.4 million of the bilateral agreement expenditure is reflected within the 2022-23 Budget?

Mr ROCKLIFF - I've had that discussion and the bilateral was signed and so -

Ms DOW - That's not there, that's right, sorry.

Mr ROCKLIFF - In saying that, I am very excited about it, bilateral, a very good commitment, the \$9.4 million is half of our response to reforms to our child matters and health services I am advised. Do you want to expand on that, Dale?

Mr WEBSTER - Through you, Premier, just to respond to that, so, the adult Head to Health, which is the Commonwealth concept, is very similar to our integrated hubs, so under the bilateral we are funding an integrated hub to the south, and they are funding adult Head Health in the north for instance.

CHAIR - Time has expired for the Mental Health and Wellbeing portfolio. Thank you, Premier, and all your staff. Thank you to everyone in attendance.

Mr ROCKLIFF - Thank you, Chair. I thank the team.

The Committee suspended at 8.15 p.m.